

 Utah Department of Health & Human Services Licensing & Background Checks		Inspection Checklist				This tool is a preliminary inspection checklist designed to ensure consistency across all inspections. <i>(Revised Jan 2026)</i>	
Provider Name:	Second Nature Therapeutic Program	Facility ID:	(F22-93635)	Phone Number:	(435) 738-2040	Notes	
Site Name or Address:	382 W Main Duchesne, UT 84021			Email Address:	steved@second-nature.com, dominic@second-nature.com, admin@second-nature.com, alea@second-nature.com		
Approved Capacity:	60	# of Present Residents\Clients:	20				
Please review the following items prior to the inspection: (Mark with a check mark if completed and make any necessary notes)				Please review the following items during the inspection: (Mark with a check mark if completed and make any necessary notes)			
☒	Current backgrounds in DACS			☒	Any active rule variances	No variances in place.	
☒	Current staff roster collected			☒	Introduce yourself and any DHHS staff		
☒	Any license restrictions or conditions	None in place.		☒	Staff Interviews	One staff interview completed	
☒	Any needed rule variances	None needed		☒	Clients Interviews	Three client interviews completed	
Inspection Information:							
- These are initial observations and do not constitute a final inspection report. Under Review indicates evidence of potential noncompliance.							
Inspection Type:	Announced Annual	Date:	5.13.26	Time Started On-site:	10:00 AM	Time Ended On-site:	4:00 PM
Number of Items Under Review:				Name of Individual Informed of this Inspection:		Dominic Conigliaro, Kayleen Richter	
Licensors(s) Conducting this Inspection:				Rob Jones, Heather Guanajuato		OL Staff Observing Inspection:	

Pre - Inspection Checklist (Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☒	☐	☐	☐	☐	
R380-600-3(2) Each applicant and provider shall comply with any applicable administrative rule, statute, zoning, fire, safety, sanitation, building and licensing laws, regulations, ordinances, and codes of the city and county in that the facility or agency will be or is located.	☒	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☒	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☒	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☒	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☒	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☒	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☒	☐	☐	☐	☐	
Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-4(1) The licensee shall submit to the office, before program implementation, policies and procedures that include: (a) a description of what constitutes sex and gender abuse, discrimination, and harassment; (b) procedures for preventing and reporting abuse, discrimination, and harassment; and (c) procedures for teaching effective and professional communication with individuals of any sexual orientations and genders.	☒	☐	☐	☐	☐	
R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.	☒	☐	☐	☐	☐	
R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.	☒	☐	☐	☐	☐	
R501-1-4(4) A congregate care program licensee shall submit to the office any policies and procedures that describe behavior management, suicide prevention, restraint, or seclusion used in the program as described in Section 26B-2-123, before implementation.	☒	☐	☐	☐	☐	

R501-1-4(5) In addition to complying with Section 26B-2-123, a congregate care program licensee shall ensure that the congregate care behavior management policy and practices reflect the following: (a) a congregate care program licensee uses behavior management techniques that are trauma-informed and appropriate for the client's age, behavior, needs, developmental level, and past experiences and defer to the least restrictive method of behavior management available to control a situation; (b) a congregate care program licensee only uses behavior management techniques that emphasize de-escalation and promote self-control, self-esteem, and independence; (c) a congregate care program licensee identifies a behavior management curriculum that emphasizes de-escalation and is compliant with Section 26B-2-123; (d) only direct care staff familiar with the child and the child's needs conduct passive physical restraint; (e) restraint is only used if it does not cause undue physical discomfort, harm, or pain to the client; (f) interventions that use painful stimuli are prohibited as a general practice; (g) passive physical restraint is used only as an emergency, temporary means of physical containment to protect the consumer, other persons, or property from immediate harm; (h) restraint only continues as long as the client presents an immediate danger to self or others; (i) passive physical restraint is not used as a convenience to staff, a substitute for programming or associated with punishment in any way; (j) a client, non-direct care staff member, or other unauthorized individual does not use any form of restraint; (k) staff do not use physical work assignments or activities that inflict pain as behavior management techniques; and (l) staff are trained to ensure the following safe practices: (i) appropriate de-escalation techniques and alternatives to restraint or seclusion; (ii) thresholds for restraints; (iii) the physiological and psychological impact of restraint; (iv) appropriate monitoring of restraint episodes; (v) how to recognize the physical signs of distress, positional asphyxia, and obtaining medical assistance; (vi) how to intervene if another staff member fails to follow correct procedures when using a restraint; (vii) time limits for restraints; (viii) the process for obtaining clinical approval for continued restraints; (ix) the procedure for documenting and reporting restraints; (x) the procedure for processing restraints with clients; (xi) the procedure for following up with staff after a restraint; (xii) how staff address injuries and complaints; (xiii) department code of conduct; and (xiv) client rights.	☒	☐	☐	☐	☐	
R501-1-4(6) A congregate care program licensee shall ensure that congregate care seclusion policy and practices reflect the following: (a) seclusion is only used to ensure the immediate safety of the child or others and is terminated as soon as the risks have been mitigated, not to exceed four hours without clinical justification; (b) staff who are familiar to the child directly supervise the child during the seclusion; (c) staff supervising seclusion ensure that any potentially harmful items or objects are removed from the seclusion environment; (d) seclusion rooms measure a minimum of 75 square feet and have a minimum ceiling height of seven feet with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (e) seclusion rooms have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (f) seclusion rooms do not have locking capability and are not located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; (g) bedrooms are not utilized as a seclusion room and seclusion rooms may not be utilized as bedrooms; (h) seclusion episodes are documented in detail by the staff involved in initiating and supervising the seclusion episode; (i) seclusion episodes of more than two in a 24-hour period are supported by clinical review and documentation regarding client suitability for remaining in the program; and (j) client time-out is used when addressing behavioral issues only if: (i) a client in time-out is never physically prevented from leaving the time-out area; (ii) it takes place away from the area of activity or from other clients, such as in the client's bedroom; (iii) staff monitors the client while in time-out; and (iv) the reason for and duration of time-out is documented by staff on duty when it occurs.	☒	☐	☐	☐	☐	
R501-1-4(7) A congregate care program licensee shall develop and follow a suicide prevention policy that complies with Subsection 26B-2-123(5).	☒	☐	☐	☐	☐	
R501-1-4(8) A congregate care program licensee shall ensure that the program's licensed clinical professional conducts regular reviews of client restraints, seclusions, behavioral interventions, and time outs to inform processing discussions with clients and training for direct care staff.	☒	☐	☐	☐	☐	
R501-1-4(9)(a) Before a congregate care program licensee may accept a client or send a discharging client who is transported by a youth transportation company as defined in Section 26B-2-101, the licensee shall ensure that the transport company is registered with the office. (b) A congregate care program licensee shall report private placements to the office as described in Section 26B-2-124 by completing the congregate care out of state placement survey on the office website no later than the fifth business day of each month.	☒	☐	☐	☐	☐	
Program Administrative and Direct Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.	☒	☐	☐	☐	☐	
R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 268-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; (f) a client rights poster in a residential setting except in a foster home or where prohibited by Settings Final Rule; and (g) department code of conduct poster.	☒	☐	☐	☐	☐	
Program Physical Facilities and Safety	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.	☒	☐	☐	☐	☐	
R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.	☒	☐	☐	☐	☐	
R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.	☒	☐	☐	☐	☐	
R501-1-8(4) The licensee shall maintain a first aid kit	☒	☐	☐	☐	☐	
Residential Program Additional Facilities and Safety Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-9(1) A residential licensee shall ensure: (a) designated space is available for records, administrative work, & confidential phone calls for clients; (c) live-in staff have dedicated bedrooms & bathrooms separate from client use; (d) each bedroom designated for a client is comparable to other similarly utilized bedrooms with similar access, location, space, furnishings, and furnishings; (e) clients are not locked in bedrooms; (f) a mirror or safety mirror is secured to each bathroom wall at a convenient height; (g) each bathroom is placed to allow access to each client without disturbing any other client during sleeping hours; (h) each bath or shower allows for individual privacy; (i) each client is supplied with hygiene supplies; (j) each sleeping area has a source of natural light and is ventilated by mechanical means or is equipped with a window that opens; (k) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (m) there are separate containers for soiled & clean laundry, if the program provides common laundry for towels, bedding or clothing; (o) equipment and supplies for washing & drying laundry are provided, if the program permits clients to do their own laundry; (p) there is at least 60 sq ft per person in a multiple occupancy bedroom and 80 sq ft in a single occupant bedroom.	☒	☐	☐	☐	☐	

R501-1-9(3) The licensee utilizing seclusion rooms shall ensure the following: (a) seclusion rooms measure a minimum of 75 sq ft and have a minimum ceiling height of 7 ft with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (b) a seclusion room shall have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (c) a seclusion room may not have locking capability and may not be located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; and (d) a bedroom may not be utilized as a seclusion room and a seclusion room may not be utilized as a bedroom.	☒	☐	☐	☐	☐	
Food Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-10(2) A licensee that provides meals shall: (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; and (f) provide adequate dining space for clients that is maintained in a clean and safe condition.	☒	☐	☐	☐	☐	
Program Client Record Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-11(2) The licensee shall document a plan detailing how each program staff and client file is maintained and remains available to the office and other agencies legally authorized to access the files for seven years regardless of whether the program remains licensed.	☒	☐	☐	☐	☐	
Rule Compliance, Penalties, Agency Action Reviews, and Appeals	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-8(11) Any owner identified in a license or certificate revocation action may not be approved for a license or certification of any other program or facility overseen by OL for five years from the date the revocation was made effective.	☒	☐	☐	☐	☐	

General Provisions - Inspection Checklist						
(Revised Jan 2026)						
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New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☒	☐	☐	☐	☐	
R380-600-3(3) An applicant or a provider shall permit the office to have immediate, unrestricted access to: (a) any unaltered on and off-site program or facility and client records; (b) each client who independently consents to speak to OL staff; (c) each site subject to licensing or certification; and (d) each staff member.	☒	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☒	☐	☐	☐	☐	
R380-600-3(15) The license or certificate holder shall adhere to any individualized parameter on a program or facility license or certificate to promote the health, safety, and welfare of any client. Parameters may include: (a) adequate square footage to determine capacity; (b) an admission or placement restriction; or (c) an age restriction.	☒	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☒	☐	☐	☐	☐	
R380-600-3(25) The provider shall post their current license or certificate, except in a foster home, on the premises in a place readily visible and accessible to the public.	☒	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☒	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☒	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☒	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☒	☐	☐	☐	☐	
Variances	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-6(5) The provider shall sign the approved variance and comply with the terms of the written variance, including any conditions or modifications contained within the approved written variance.	☒	☐	☐	☐	☐	
Inspection and Investigation Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-7(16) When a critical incident occurs under the direct responsibility and supervision of the program or facility, the provider shall: (a) submit a report of the critical incident to OL in a format required by OL within one business day of the critical incident occurrence; (b) additionally ensure any allegation of an incident of abuse, neglect, or exploitation of a client is reported to DCFS for a minor client or APS for an adult client and law enforcement within 24 hours; (c) notify the parent or legal guardian of each involved client within a 24-hour period from the time of the incident; (d) if the critical incident involves any client in the custody of the department or under contract with the department, notify the involved department division immediately; and (e) collect, maintain, and submit original witness statements and supporting documentation, including video footage if available, regarding each critical incident to OL upon request.	☒	☐	☐	☐	☐	
Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.	☒	☐	☐	☐	☐	
R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.	☒	☐	☐	☐	☐	
Residential Programs Additional Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-5(1) A licensee that manages, stores, or administers client medications shall develop and ensure compliance with the following medication management safe practices: (a) inform staff and clients of program and client responsibility for medication including storage and administration of medications on-site and, as applicable, when staff and clients are offsite in program related activities; (b) if applicable, inform staff and clients of the medication self-administration process; (c) if storing and administering medications, train staff to administer medication and the process to be followed; (d) how staff record medication dosages according to prescriptions; (e) how staff monitor for and record effects and side effects of medications; and (f) how staff log doses and record and report medication errors.	☒	☐	☐	☐	☐	
R501-1-5(2) The licensee shall ensure the care, vaccination, licensure, and maintenance of any animals on-site to include: (a) assessment of pet allergies for any clients interacting with animals in the program; (b) maintenance of required examinations, registrations, and vaccinations; and (c) supervision of clients in the presence of animals.	☒	☐	☐	☐	☐	
R501-1-5(4) The licensee shall ensure that a ratio of one staff to one client during transports is only utilized when the program has conducted a safety assessment that indicates that client and staff safety is reasonably assured.	☒	☐	☐	☐	☐	
Program Administrative and Direct Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.	☒	☐	☐	☐	☐	
R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 26B-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; (f) a client rights poster in a residential setting except in a foster home or where prohibited by Settings Final Rule; and (g) department code of conduct poster.	☒	☐	☐	☐	☐	Gave updated client right for files, they did have posted current.
R501-1-6(3) The licensee shall maintain compliance with or documentation of an exemption from any of the following requirements: (a) a food handler permit for any person preparing meals for any other person; (b) capacity determinations that include each staff and client on premises and may not exceed the capacity limits placed by local authorities; (c) fire clearance, if conducted separately from a business license; (d) licensure and registration of any vehicles used to transport clients.	☒	☐	☐	☐	☐	
R501-1-6(5) The licensee shall maintain and make the following available to the department upon request: (b) general liability insurance; (c) professional liability insurance; (d) vehicle insurance; (e) fire insurance; and (f) additional insurance as required to cover each program activity.	☒	☐	☐	☐	☐	

R501-1-6(6) The licensee shall ensure: (b) current staff and client lists are available at each licensed site; (c) the organizational and governance structure of the program is available to the department upon request and includes: (i) line of authority and responsibility; (ii) a job description, including each duty and qualification for each job title; and (iii) notification to the office of any program changes as described in Section R380-600-3; (d) a quality improvement plan that is implemented and followed and that incorporates, at a minimum, client and staff grievances, feedback, trends in licensing noncompliance and incident reports; (e) the licensee provides an interpreter or refers each client to appropriate resources as necessary to communicate with the client; (f) the licensee maintains an opioid overdose reversal kit on-site with on duty staff trained in its use if the licensee is serving, or is likely to serve, a client with a substance use disorder.	☒	☐	☐	☐	☐	
R501-1-6(7) A licensee serving education entitled children shall comply with Section 26B-2-116 regarding coordination of educational services to include completion of youth education forms at initial and renewal licensure.	☒	☐	☐	☐	☐	
R501-1-6(8) A licensee offering school on-site shall: (a) maintain the established staff to client ratio with behavioral intervention trained staff in the school setting; (b) ensure recognition in good standing by an educational accreditation organization such as the State Board of Education or the National School Accreditation Board; (c) ensure each client is taught at their appropriate grade level.	☒	☐	☐	☐	☐	
R501-1-6(9) The licensee shall ensure clinical and medical staff are licensed or certified in good standing and any unlicensed staff are appropriately supervised	☒	☐	☐	☐	☐	
Residential Program Additional Administration and Direct Services Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-7(1) A residential program licensee shall additionally: (a) ensure each staff shift list remains current and available to the office upon request; (b) ensure that each shift documents any illness, injury or critical incident and passes it on to the next shift and administration; (c) ensure at least two on-duty staff are present at all times; (d) ensure access to a medical clinic or a medical professional familiar with the program and population served; and (e) provide a separate space for clients who are showing symptoms of an infectious disease.	☒	☐	☐	☐	☐	
R501-1-7(3) A congregate care program licensee may allow an individual turning 18 to remain in the program if: (a) the individual remains in the custody of a state entity or the individual was admitted and continuously resided in the program for at least 30 days before the individual's 18th birthday; (b) the licensee has a documented need for the individual to remain in the program; (c) the licensee maintains responsibility for discharge to an appropriate setting when clinically appropriate and no later than the day an individual reaches 19 years of age; (d) the licensee outlines a plan for the protection of younger clients by supervising and separating 18-year-old individuals from youth who are more than two years younger; and (e) the individual signs a consent document outlining: (i) the individual is consenting to remain in the program voluntarily and understands the individual is not required to remain against their will; (ii) that any criminal offenses committed may result in being charged as an adult; and (iii) that if the individual is involved in any critical incidents posing a risk to the health and safety of other program residents they may be discharged from the program.	☒	☐	☐	☐	☐	
R501-1-7(4) A congregate care program licensee shall ensure weekly confidential communication with family and shall ensure that: (a) the frequency or form of the confidential communication requirement is only modified if the program submits a modification request that demonstrates the following to the office: (i) the program operates in an area of limited or unreliable phone accessibility or coverage; (ii) there is significant risk of harm or danger to client safety by providing youth with unsupervised telephone access; (iii) the licensee offers an alternative that satisfies the requirement of weekly confidential two-way communication; or (vi) extenuating circumstances exist outside the individual treatment plans that are prohibitive to offering voice to voice communication; (b) a parent or guardian authorizes in writing an alternate means of confidential communication when voice to voice is unavailable; and (c) the licensee offers voice to voice confidential communication as soon as it can be safely offered.	☒	☐	☐	☐	☐	
R501-1-7(6)(a) A residential program licensee, excluding a residential treatment program, may allow for client independence and responsibility for their own supplies, food, laundry, or transportation by outlining in writing resources and responsibility for the provision of these items. (b) Each residential program licensee shall assist clients on a limited basis if they are temporarily unable to provide the items or services listed in Subsection R501-1-7(6)(a) for themselves.	☒	☐	☐	☐	☐	
Program Physical Facilities and Safety	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.	☒	☐	☐	☐	☐	Intake office and restroom was in compliance.
R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.	☒	☐	☐	☐	☐	
R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.	☒	☐	☐	☐	☐	
R501-1-8(4) The licensee shall maintain a first aid kit	☒	☐	☐	☐	☐	
Residential Program Additional Facilities and Safety Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-9(1) A residential licensee shall ensure: (a) designated space is available for records, administrative work, & confidential phone calls for clients; (c) live-in staff have dedicated bedrooms & bathrooms separate from client use; (d) each bedroom designated for a client is comparable to other similarly utilized bedrooms with similar access, location, space, finishings, and furnishings; (e) clients are not locked in bedrooms; (f) a mirror or safety mirror is secured to each bathroom wall at a convenient height; (g) each bathroom is placed to allow access to each client without disturbing any other client during sleeping hours; (h) each bath or shower allows for individual privacy; (i) each client is supplied with hygiene supplies; (j) each sleeping area has a source of natural light and is ventilated by mechanical means or is equipped with a window that opens; (k) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (l) each client is allowed to decorate & personalize their bedroom, while maintaining respect for other residents and property; (m) there are separate containers for soiled & clean laundry, if the program provides common laundry for towels, bedding or clothing; (n) bedding & towels are laundered weekly & after each client is discharged; (o) equipment and supplies for washing & drying laundry are provided, if the program permits clients to do their own laundry; (p) there is at least 60 sq ft per person in a multiple occupancy bedroom and 80 sq ft in a single occupant bedroom.	☐	☐	☒	☐	☐	There is an administrative building that is utilized for (a), (b)-(p) do not apply to this provider as they do not have clients reside in a building. Necessary privacy is accommodated in the field.
R501-1-9(2) A residential program licensee serving individuals with disabilities shall house no more than two clients in each bedroom.	☐	☐	☒	☐	☐	Licensee does not utilize bedrooms.
R501-1-9(3) The licensee utilizing seclusion rooms shall ensure the following: (a) seclusion rooms measure a minimum of 75 sq ft and have a minimum ceiling height of 7 ft with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (b) a seclusion room shall have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (c) a seclusion room may not have locking capability and may not be located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; and (d) a bedroom may not be utilized as a seclusion room and a seclusion room may not be utilized as a bedroom.	☐	☐	☒	☐	☐	Licensee does not utilize seclusion rooms.
R501-1-9(5) The licensee shall train staff and ensure that the use of any alternate sleeping arrangements other than the client's assigned bedroom complies with Subsection R501-1-4(7) and: (a) preserves client dignity and confidentiality; (b) shall be done on an individualized, time delimited basis; (c) may not be utilized due to staffing shortages or for staff convenience; and (d) may not be used as behavior management or consequence.	☒	☐	☐	☐	☐	

Food Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-10(2) A licensee that provides meals shall: (a) ensure that meals are not used as incentive or punishment; (b) provide nutritional counseling to staff and clients; (c) designate staff responsible for food service who: (i) maintain a current list of each client with special nutritional needs; and (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; (d) except in a day treatment program serving clients for less than ten hours a day, or outpatient programs serving clients for less than six consecutive hours a day, provide a variety of three nutritious meals a day that are: (i) served from dietitian or nutritionist approved menus; or (ii) for programs serving individuals experiencing homelessness, serve meals as required by USDA standard homeless settings; (f) provide adequate dining space for clients that is maintained in a clean and safe condition.	☒	☐	☐	☐	☐	
R501-1-10(3) A licensee that allows self-serve meals shall ensure that self-serve kitchen users are supervised, directed, and trained by a staff that has a food handler's permit or is trained by Serv-Safe, USDA, or a comparable program.	☒	☐	☐	☐	☐	
R501-1-10(4) A licensee that serves parents and their children may allow a consenting adult client to maintain full responsibility for their, and their child's, special dietary needs, if consent is maintained in writing in the client record.	☒	☐	☐	☐	☐	
R501-1-10(5) A licensee that offers meals for clients shall ensure there is documented training confirming staff are trained to and adhere to the following safe practices: (a) how to identify and accommodate clients with special dietary needs; and (b) allowances for nutritious snacks to be available during restricted hours if the program restricts access to food and kitchen equipment.	☒	☐	☐	☐	☐	
R501-1-10(6) If meals are prepared by clients, the licensee shall inform staff and clients in writing of the following: (a) rules and privileges of kitchen use; (b) menu planning and procedures; (c) sharing self-prepared food; (d) nutrition and sanitation requirements; (e) schedule of responsibilities; and (f) shopping and storage responsibilities.	☒	☐	☐	☐	☐	
Program Client Record Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-11(1) The licensee shall maintain client information to include the following: (a) client name, address, email address, phone number, date of birth and identified gender; (b) emergency contact names, including legal guardian where applicable, and at minimum, the emergency contact's physical address, current email address or current phone numbers; (c) a program serving substance use disorder clients shall maintain compliance with an initial and annual client tuberculosis screening results in each client record; (d) any information that could affect health safety or well-being of the client including each medication, allergy, chronic condition or communicable disease; (e) intake screening and assessment; (f) discharge documentation; (g) treatment or service plan; (h) progress notes and services provided with date and signature of staff completing each entry; (i) individualized assessment for restriction of access to on-site items that could be used as weapons, for self-directed violence, or as an intoxicant; (j) any referral arrangements made by the program; (k) client or guardian signed consent or court order of commitment to services in lieu of signed consent for each treatment and non-clinical service; (l) summary of attendance and absences in treatment services; (m) any grievance or complaint made by or against the client and actions taken by the program; (n) each crisis intervention or critical incident report involving the client; and (o) any signed agreement and consent form.	☒	☐	☐	☐	☐	
Program Intake and Discharge Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-12(1) The licensee shall complete an intake screening before accepting a client into the program that includes at least: (a) verification that the client meets the eligibility requirements of the program; (b) verification that the client does not meet any of the exclusionary criteria that the program identified in policy as unable to serve; (c) description of presenting needs; and (d) suicide risk screening.	☒	☐	☐	☐	☐	

R501-1-12(2) A licensee serving substance use disorder clients may not admit anyone who is unresponsive or unable to consent to care because the individual is experiencing convulsions, in shock, delirium tremens, in a coma, or unconscious.	☒	☐	☐	☐	☒	
R501-1-12(3) A licensee serving incarcerated or court-mandated justice involved clients shall: (a) conduct a criminogenic risk assessment; (c) separate high and low criminogenic risk populations.	☐	☐	☒	☐	☐	Licensee does not serve court-mandated clients.
R501-1-12(4) The licensee shall ensure that , the client, parent, or guardian signs and receives copies of the following agreements to be maintained as client records: (a) determination of eligibility; (b) fee agreement outlining costs of services including program, client, parent, or guardian responsibility for payment; and (c) signed consent for treatment that outlines: (i) rules of the program; (ii) expectations of clients, parents, and guardians; (iii) services to be provided; (iv) Medicaid number, insurance information, and identification of any other entities that are billed for the client's services; (v) client rights; and (vi) licensing contact information.	☐	☐	☐	☐	☒	Licensee had Ombuds information, however, not licensing contact information.
R501-1-12(5) The licensee shall ensure that a discharge plan identifies resources available to a client and includes: (a) reason for discharge or transfer; (b) aftercare plan; (c) summary of services provided; and (d) progress evaluation.	☒	☐	☐	☐	☐	
Residential Additional Program Intake and Discharge Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-13(1) A residential program licensee shall ensure an intake assessment is completed following an approved intake screening, no later than seven days from the admission date, and that the assessment considers and contains: (a) gender identity and individualized assessment for bedroom and bathroom assignments; (b) cultural background; (c) dominant language and mode of communication; (d) family history and dynamics; (e) current and past health and medical history; (f) social, psychological, developmental, vocational, and, as appropriate, educational factors; (h) authorization to serve and obtain emergency care.	☒	☐	☐	☐	☐	
R501-1-13(2) A residential program licensee may not serve youth from out of state without a disruption plan, and as applicable, Interstate Compact Placement of Children (ICPC).	☒	☐	☐	☐	☐	
R501-1-13(3) A congregate care program licensee shall ensure that each congregate care disruption plan complies with the following: (a) the program retains jurisdiction and responsibility for the youth while the youth remains in Utah; and (b) the program completes an individualized disruption plan at the time of intake for each out of state client to include: (i) who is responsible for the child's return if placement at the facility disrupts; (ii) current emergency contact information to include the name, address, phone and email address of the parent or responsible person; (iii) a signed statement from parent or responsible person outlining the plan for the youth in the event of an unplanned disruption in care; and (iv) a plan for safe transportation either to the state of origin, the responsible person as identified in Subsection R501-1-13(3)(b)(i) or to another licensed congregate care program or higher level of care, as needed.	☒	☐	☐	☐	☐	
R501-1-13(5) A congregate care program licensee shall report private placements to the office by completing the congregate care out of state placement survey on the office website no later than the fifth business day of each month.	☒	☐	☐	☐	☐	
Program Clinical Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-1-14(1) A licensee that offers clinical treatment shall: (a) assign a clinical director to ensure that assessment, treatment, and service planning practices are: (i) regularly reviewed and updated; (ii) individualized; and (iii) designed to involve the participation of each client or each client's parent or guardian; (b) ensure each person working directly with a client is informed of the client's individual treatment needs and advised of the best approach to working with that client; (c) ensure client treatment plans are developed and signed by a licensed clinical professional within 30 days of admission; (d) ensure discharge goals are identified in the initial treatment plan and treatment goals are structured around the identified discharge goals and objectives; (e) ensure that each client identified for treatment receives individual treatment at least weekly; and (f) ensure any missing individual weekly treatment is justified, approved, and documented by the clinical director.	☒	☐	☐	☐	☐	
R501-1-14(2)(a) A residential program licensee shall ensure that in addition to the required weekly individual therapy, frequency and need for family and group therapy and other clinical services are addressed in the individual's treatment plan. (b) A non-residential program licensee who offers clinical treatment may alter the weekly therapy requirement as designated in the individual's treatment plan.	☒	☐	☐	☐	☐	
R501-1-14(4) A licensee who offers group counseling, family counseling, skills development, or other treatment shall offer and document these treatment services as prescribed in the treatment plan.	☒	☐	☐	☐	☐	
Program Staffing	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-15(1) The licensee shall ensure adequate staffing to safely supervise the current population, including adding more staff than required by the usual staffing ratio as needed to manage behaviors, dynamics, and individual client treatment and supervision needs.	☒	☐	☐	☐	☐	
R501-1-15(2) The licensee shall identify a manager or qualified designee who is immediately available when the program is in operation or there is a qualified and trained substitute when the manager is absent or unavailable.	☒	☐	☐	☐	☐	
R501-1-15(3) A licensee that offers clinical services shall employ or consult with licensed professional staff that include an individual who is familiar with the program and the needs of each client.	☒	☐	☐	☐	☐	
R501-1-15(4) The licensee shall ensure that before allowing a direct care staff to work unsupervised they have an approved background clearance except as excluded in Section R501-14-17;	☒	☐	☐	☐	☐	
R501-1-15(5) A licensee who serves clients with substance use disorder shall ensure each staff is screened for tuberculosis.	☐	☐	☒	☐	☐	Licensee does not screen for tuberculosis.
R501-1-15(6) A licensee who serves a client with substance use disorder may not offer, entice, refer, or recommend medical cannabis as treatment for substance use disorder.	☒	☐	☐	☐	☐	
R501-1-15(7) A licensee who manages, stores, or administers client medication shall identify a medical professional to oversee the medication management, medication oversight, and staff training regarding medication management and administration.	☒	☐	☐	☐	☐	
R501-1-15(8) The licensee shall ensure that each person involved with the prescription, administration, or dispensing of controlled substances maintains appropriate medical or pharmacy licenses and DEA registration numbers	☒	☐	☐	☐	☐	
R501-1-15(9) The licensee shall create and maintain personnel information for each staff member, contracted employee, and volunteer.	☒	☐	☐	☐	☐	
R501-1-15(10) The licensee shall ensure that personnel information includes: (a) any applicable qualification, experience, certification, or license; (b) any approved and current office background clearance, except as excluded in Rule R501-14; (c) a provider code of conduct that is signed by the staff member, contracted employee, or volunteer; (d) any pre-service and annual training records with the date completed, topic, and the individual's signed acknowledgment of training completion; (e) any grievances or complaints made by or against the individual and actions taken by the program; & (f) each crisis intervention or critical incident report involving the individual.	☒	☐	☐	☐	☐	
R501-1-15(11) The licensee shall ensure that at least one CPR and First Aid-certified staff member is available when staff and clients are present unless a currently licensed healthcare professional is present.	☒	☐	☐	☐	☐	
Personnel Training Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-1-16(1) The licensee shall ensure that each staff receives pre-serving training on the following topics before being left unsupervised and within 30-days of hire: (a) program policies, procedures and safe practices as outlined in Section R501-1-5; (b) program emergency preparedness, response, and recovery plan, including at least: (i) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; and (ii) instructions to staff regarding how to report and respond to significant criminal activity and significant medical emergencies; (c) CPR and First Aid; (d) client eligibility, emphasizing the behaviors and circumstances the program can safely manage; (e) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (f) client rights; (g) supervision and ratios; (h) as applicable, medications management, storing, and administration; (i) as applicable, food handling as outlined in Subsection R501-1-10(3); (j) background checks; (k) prevention, signs and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (l) provider code of conduct as outlined in Rule 380-80; (m) non-discrimination policy in accordance with Section 26B-2-109 that includes a prohibition of abuse, discrimination, and harassment based on sex, gender identity, or sexual orientation; (n) staff and client grievance procedures; (o) crisis intervention; (p) appropriate use of restraint and seclusion; (q) de-escalation techniques; (r) appropriate searches; (s) appropriate and inappropriate behaviors of clients; (t) appropriate and inappropriate staff responses to client behaviors; and (u) if applicable, staff response to a client leaving a program without permission.	☒	☐	☐	☐	☐	
R501-1-16(2) The licensee shall ensure each staff completes the following training topics each year, based on the program's license date: (a) program policies, procedures and safe practices as outlined in Section R501-1-4; (b) general provisions and applicable categorical licensing rule; (c) client eligibility, as outlined in Subsection R501-1-6(1)(e), emphasizing the behaviors and circumstances the program can safely manage; (d) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (e) provider code of conduct as outlined in Rule R380-80; (f) program plan for the prevention or control of infectious and communicable disease to include coordination with and following any guidance of the state or local health authorities, Center for Disease Control, and the department; (g) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; (h) program rules regarding firearms that does not conflict with constitutional or statutory rights regarding concealed weapons permits as described in Title 53, Chapter 5, Part 7, Concealed Firearms Act; (i) smoking rules in accordance with Title 26B, Chapter 7, Part 5, Regulation of Smoking, Tobacco Products, and Nicotine Products; (j) how to manage clients who screen with elevated suicide risk levels; (k) general incident reporting; (l) prevention, signs, and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (m) CPR and first aid; (n) if storing and administering medications, training required to administer medication and the process to be followed; (o) training to identify and address in a residential or congregate care program: (i) clients who pose a risk of violence; (ii) what constitutes contraband, possession of contraband, and how the program ensures restriction of client access to contraband and dangerous weapons or materials; (iii) clients who are at risk for suicide; (iv) managing clients with mental health concerns; and (v) identifying the signs and symptoms of clients presenting under the influence of substances or alcohol; (p) if the licensee manages funds for client allowances, training to document each expense; and (q) appropriate use of any alternate sleeping arrangements in a residential or congregate care program.	☒	☐	☐	☐	☐	

Outdoor Youth Program - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-3(1) Each outdoor youth program shall provide an educational component as determined by the Utah State Board of Education for clients 18 years of age or younger who have been removed from their educational opportunities for more than one month. The administrators of the program shall meet and document cooperation with the State Board of Education.	☒	☐	☐	☐	☐	
Program Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-4(4) . Each client has clothing and equipment to protect them from the environment. This equipment is never be removed, denied, or made unavailable.	☒	☐	☐	☐	☐	
R501-8-4(5) . During an expedition, if a client refuses or cannot hike or to carry the client's equipment, the group ceases hiking. The program establishes, documents, and resolves the reasons for the client's refusal or inability to continue before hiking continues.	☒	☐	☐	☐	☐	
R501-8-4(6) . Deprivation of essential equipment or items is not used as a consequence.	☒	☐	☐	☐	☐	
R501-8-4(7) . The program conducts an individual assessment of each client's recommended backpack weight. Each backpack weight guideline may not exceed 20 % of the client's body weight. If a client is required to carry other items, the total weight carried must not exceed 30 % of the client's body weight unless individually documented with parental permission to exceed this ratio.	☒	☐	☐	☐	☐	
R501-8-4(8) . The program provides clients with clean clothing at least weekly and provides a means for each client to bathe or otherwise clean the client's body at least twice weekly.	☒	☐	☐	☐	☐	
R501-8-4(9) . Hiking does not exceed the physical capability of the weakest member of the group. Hiking is prohibited at temperatures above 90 degrees F. or at temperatures below ten degrees F.	☒	☐	☐	☐	☐	
R501-8-4(10) . A field staff in each group carries a means to accurately measure and display the current temperature.	☒	☐	☐	☐	☐	
R501-8-4(11) . Each expedition plan including map routes, anticipated schedules, and times is carried by the field staff and recorded in the field office.	☒	☐	☐	☐	☐	
R501-8-4(12) . A field staff in each group maintains a signed daily log or dictates a recorded log to be transcribed and signed immediately following termination of the activity to contain the following information: (a) each critical incident; (b) prescription compliance; (c) each medical concern; (d) each behavioral concern or refusal to hike and how the concern is addressed; (e) each unusual occurrence; and (f) each log entry that is recorded is in an un-editable format and remains available to the office upon request.	☒	☐	☐	☐	☐	
R501-8-4(13) . Each program staff is required to carry an accurate, reliable time piece accurately reflecting the time of day and for documentation purposes in log notes and incident reports.	☒	☐	☐	☐	☐	
R501-8-4(14) . Program administration trains each staff regarding the standards of this section and regularly monitors and ensures compliance.	☒	☐	☐	☐	☐	
Staff, Interns, and Volunteers	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-8-5(1). The outdoor youth program has a governing body and an executive director who has responsibility and authority over the policies and activities of the program and coordinates office and support services and training. The executive director has the following qualifications: (a) be at least 25 years of age; (b) have a bachelor's degree or equal training and experience in a related field; (c) have at least two years of outdoor youth program administrative experience; (d) have at least 30 credit hours education in recreational therapy or related experience or one-year outdoor youth program field experience; (e) demonstrate knowledge and understanding of relevant licensing rules; and (f) have completed each required staff training.	☒	☐	☐	☐	☐	
R501-8-5(2). The outdoor youth program has a direct care field director who has primary responsibility for coordinating field operations, managing field staff, operating the field office, and supervising emergency response procedures.	☒	☐	☐	☐	☐	
R501-8-5(3). A field director or a qualified designee must: (a) be trained as a direct care staff in accordance with Section R501-1-14; (b) be at least 25 years of age; (c) have a bachelor's degree or equal training and experience in a related field; (d) have at least two years of outdoor youth program field experience; (e) visit in the field at least two days a week with no more than five days between visits; and (f) document each field visit, including: (i) the condition of each client; (ii) interactions with clients and staff; (iii) incidents and interventions to be reported to each client's guardian and the office; (iv) each report of compliance with Subsection 62A-2-123(6) regarding weekly confidential communication with family; and (v) staff compliance with each policy and rule.	☒	☐	☐	☐	☐	
R501-8-5(4). The outdoor youth program has field support staff to be responsible for delivering supplies and mail to the field, communication with each client in the field, and first aid support.	☒	☐	☐	☐	☐	
R501-8-5(5). Each outdoor youth program group has direct care senior field staff working directly with the clients and who meet the following qualifications: (a) be trained as a direct care staff in accordance with Section R501-1-14; (b) be at least 21 years of age; (c) have an associate's degree or high school diploma with 30 credit hours of education and training or comparable experience and training in a related field; and (d) have six months outdoor youth program field experience or comparable experience which shall be documented in the individual's personnel file.	☒	☐	☐	☐	☐	
R501-8-5(6). The outdoor youth program has direct care field staff working directly with the clients and meet the following qualifications: (a) be at least 20 years of age; (b) have a high school diploma or equivalent; (c) have 48 field-days of outdoor youth program experience or comparable experience which shall be documented in the individual's personnel file; and (d) exhibit skilled leadership.	☒	☐	☐	☐	☐	
R501-8-5(7). The outdoor youth program has direct care assistant field staff as required to meet or exceed staff to client ratios. Assistant field staff meet the following qualifications: (a) be at least 19 years of age; (b) have a high school diploma or equivalent; (c) have 24 field days of outdoor youth programs experience; and (d) exhibit skilled leadership.	☒	☐	☐	☐	☐	
R501-8-5(8). The outdoor youth program has a licensed physician and mental health professional accessible to each client.	☒	☐	☐	☐	☐	

R501-8-5(9). If the outdoor youth program has interns or volunteers who are learning the program practices while completing educational requirements: (a) Each intern must be at least 19 years of age. (b) Each volunteer must be at least 18 years of age. (c) Staff training must be completed by each incoming staff including interns and volunteers regardless of background experience. (d) Each volunteer and intern must be supervised by the clinical director, program administration, or senior direct care staff. (e) Each intern and volunteer may never directly supervise a client.	☐	☐	☒	☐	☐	Licsee does not utalize volunteers.
Client Supervision and Ratios	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-6(1). Each youth group is directly supervised by at least two direct care staff, one of which is a direct care senior field staff.	☒	☐	☐	☐	☐	
R501-8-6(2). Each field group does not exceed 16 people with a ratio of at least one staff per four clients. Staff counts towards the field group size.	☒	☐	☐	☐	☐	
R501-8-6(3). Each volunteer is counted as a client in figuring staff to client ratios.	☐	☐	☒	☐	☐	Licsee does not utalize volunteers.
R501-8-6(4). Field group size does not exceed the number specified by federal, state, or local agencies in whose jurisdiction the program is operated.	☒	☐	☐	☐	☐	
Staff Training	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-7(1). The outdoor youth program provides at least 80 hours initial staff training.	☒	☐	☐	☐	☐	
R501-8-7(2). Initial staff training is not considered completed until the staff have demonstrated to the field director proficiency in each of the following areas: (a) counseling, teaching and supervisory skills; (b) water, food, and shelter procurement, preparation, and conservation; (c) low impact wilderness expedition and environmental conservation skills and procedures; (d) client management, including containment, control, safety, conflict resolution, and behavior management; (e) instruction in safety procedures and safe equipment use, fuel, fire, life protection, & related tools; (f) instruction in emergency procedure, medical treatment, evacuation, weather, signaling, fire, and dealing with runaway and lost clients; (g) sanitation procedures, water, trash, human waste, food handling; (h) wilderness medicine, including health issues related to acclimation, exposure to the environment, and anaphylaxis; (i) CPR, standard first aid, first aid kit contents and use, and the program's medication management policy and procedure; (j) navigation skills, including map and compass use and contour and celestial navigation; (k) local environmental precautions, including terrain, weather, spiders, ticks, scorpions, snakes, insects, predatory animals, poisonous plants, giardia, frostbite, hypothermia, heat exhaustion, dehydration, responses to adverse situations, and emergency evacuation; (l) leadership and judgment; (m) report writing, including required development and maintenance of logs; and (n) federal, state, and local regulations.	☒	☐	☐	☐	☐	
R501-8-7(3). At least 80 hours of initial staff training is completed, documented, and maintained in each staff personnel file.	☒	☐	☐	☐	☐	
R501-8-7(4). The field director or equally qualified designee documents in each personnel file how the field director or qualified designee determined that each staff has demonstrated proficiency in each of the required topic areas as listed in Subsection (2) of this section.	☒	☐	☐	☐	☐	
R501-8-7(5). Each initial staff training and demonstration of proficiency is completed and documented before the staff may count in the staff client ratio.	☒	☐	☐	☐	☐	

R501-8-7(6). The program provides and documents on-going staff training to improve proficiency in knowledge and skills and to maintain certifications.	☒	☐	☐	☐	☐	
Staff Health Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-8(1). Before engaging in any field activity, each staff adhereS to the following: (a) each field staff, intern, and volunteer haS an annual physical examination and health history signed by a licensed medical professional; (b) a recognized physical stress assessment is completed as part of the physical examination of each staff; (c) the physical examination of each staff is reviewed and maintained by the provider in the staff personnel file; and (d) each program staff, intern, and volunteer submits to drug and alcohol screening upon request.	☒	☐	☐	☐	☐	
Client Admission Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-9(1). Clients are no younger than 13 years of age and no older than 17 years of age and have a current health history report which includes notation of client physical limitations and prescriptive medications.	☒	☐	☐	☐	☐	
R501-8-9(2). The health history report is completed, submitted, and verified by each client's parent or guardian as part of the intake screening or assessment in accordance with Sections R501-1-18 and R501-1-23 and before entry into the field.	☒	☐	☐	☐	☐	
R501-8-9(3). An admissions assessment is conducted by a treatment professional before each client enters into the field and includes the following: (a) a review of each client's social and psychological history with the client's parent or legal guardian before enrollment; and (b) an interview with the client before entrance into the field program.	☒	☐	☐	☐	☐	
R501-8-9(4). Before entry into the field and within 15 days of admission to the program, the following requirements are met: (a) a licensed medical professional must review each client's health history report and conduct a physical examination; and (b) the program provides a physical examination form to a licensed medical professional that clearly states a description of the physical demands and environment of the program, and requires the following information before a client may enter the field: (i) a urinalysis drug screen; (ii) a complete blood count (CBC) unless waived in writing by the client's parent or guardian; (iii) a complete metabolic profile (CMP) unless waived in writing by the client's parent or guardian; (iv) a urinalysis for possible infections; (v) a pregnancy test; (vi) a physical stress assessment; (vii) a determination by the physician if detoxification is indicated for client before entrance into field portion of the program; (viii) any other tests as necessary to assess fitness for the field portion of the program; and (ix) a medical professional shall review current and historical medical data and approve the client to enter the field with recommendations for any medical monitoring.	☒	☐	☐	☐	☐	
R501-8-9(5). A copy of each client's medical forms and approvals are maintained at the field office and another copy is carried by staff members in a waterproof container throughout the field expedition.	☒	☐	☐	☐	☐	
R501-8-9(6). The program clinically reviews each client's psychological history and conducts an additional psychological assessment as clinically necessary before the client's entry into the field.	☒	☐	☐	☐	☐	
R501-8-9(7). Upon admission and for a period of no fewer than three days in the field, direct care field staff closely monitor each client for any health problems that may be a result of hiking or living outdoors.	☒	☐	☐	☐	☐	

Water and Nutritional Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-10(1). At least six quarts of potable water are available per person per day, plus one additional quart per person for each five miles hiked. Although it is not required that the entire amount be hand carried, water is always accessible during hiking.	☒	☐	☐	☐	☐	
R501-8-10(2). In temperatures above 90 degrees F., staff make sure each client's fluid intake is at least three quarts of water per day.	☒	☐	☐	☐	☐	
R501-8-10(3). Each field group in the field always has electrolyte replacement available.	☒	☐	☐	☐	☐	
R501-8-10(4). In temperatures above 80 degrees F., water is available for coating each client's body, and other techniques are available for cooling as needed.	☒	☐	☐	☐	☐	
R501-8-10(5). Potable water is available at each campsite. Water cache location information is verified with field staff before the group leaves camp each day.	☒	☐	☐	☐	☐	
R501-8-10(6). No expedition group depends on aerial drops for water. Aerial water drops are used for emergency situations only.	☒	☐	☐	☐	☐	
R501-8-10(7). Water from natural sources is made safe to drink through boiling, filtering, or disinfection in accordance with the center for disease control guidance.	☐	☐	☒	☐	☐	Licsee does not utalize any water from natural or sources naturally available in the field.
R501-8-10(8). The outdoor youth program has a written menu describing food supplied to the client which provides at least 3,000 calories per day while in the field. There are fresh fruit and vegetables available at least twice a week. Food is never withheld from a client for any reason. If no fire is available, other food of equal caloric value, which does not require cooking, is available. (a) The menu is adjusted to increase minimum dietary needs as energy expenditure, including exercise and climate conditions, dictate. (b) Food is from a balance of the food groups. (c) Forage items do not count toward the determination of caloric intake. (d) Multiple vitamin supplements are offered daily.	☒	☐	☐	☐	☐	
Health Care	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-11(1). The outdoor youth program provides first aid treatment promptly.	☒	☐	☐	☐	☐	
R501-8-11(2). When a client has an illness or physical complaint that does not respond to or cannot be treated by standard first aid, the program immediately arranges for the client to be seen and treated as indicated by a licensed medical professional.	☒	☐	☐	☐	☐	
R501-8-11(3). Each client's physical condition is assessed at least every 14 days by a qualified medical professional. Blood pressure, heart rate, allergies, and general physical condition are checked and documented. Any assessment concerns are documented, and the client is taken to the appropriate medical professional for treatment. There are no consequences issued to a client for requesting to see a healthcare professional or for anything said to a health care professional.	☒	☐	☐	☐	☐	
R501-8-11(4). Each prescription and over the counter medication is kept in the secure possession of designated staff and provided to clients in accordance with labels or prescription directions.	☒	☐	☐	☐	☐	
R501-8-11(5). Staff are trained for medication administration in accordance with Rule R501-1 and communicate with the field director and document reason and plan for any lost or missing prescription medication.	☒	☐	☐	☐	☐	
R501-8-11(6). A foot check is conducted at least twice daily and documented.	☒	☐	☐	☐	☐	
Safety	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-12(1). Each first aid kits includes sufficient supplies for the activity, location, and environment as approved by the program's medical professional. First aid kit supplies are available during each field activity.	☒	☐	☐	☐	☐	

R501-8-12(2). Each outdoor youth program has a support system that meets the following criteria: (a) reliable daily two-way radio communications between groups and with support staff, with additional charged battery packs and a reliable backup system of contact in the event the radio system fails; (b) the support vehicles and field office are equipped with first aid equipment; (c) the support and field staff have access to contact information including telephone numbers, locations, contact personnel, maps, medical forms, and procedures for an emergency evacuation or field incident; and (d) daily morning and evening contacts are completed between field staff, support staff, and the field office and contacts are documented in the field office log daily.	☒	☐	☐	☐	☐	
Field Office	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-13(1). The program maintains a field office.	☒	☐	☐	☐	☐	
R501-8-13(2). Communication systems between the field and the field office are monitored 24 hours a day when clients are in the field.	☒	☐	☐	☐	☐	
R501-8-13(3). Support staff respond immediately to any emergency situation.	☒	☐	☐	☐	☐	
R501-8-13(4). Support staff on duty are within one hour of any field group.	☒	☐	☐	☐	☐	
R501-8-13(5). When staff are not present in the field office a contact telephone number is posted on the field office door and the field director designates responsible on-call staff who continually monitors communications and remains available and able to access all necessary equipment and files within 15 minutes.	☒	☐	☐	☐	☐	
R501-8-13(6). Field office staff adhere to the following: (a) maintain current staff and client records in accordance with Rule R501-1; (b) maintain a master map of each activity area; (c) maintain copies of each expeditionary route with its schedule and itinerary to be immediately available to the office and emergency medical services, law enforcement or search and rescue agencies as needed; (d) maintain a log of daily communications; (e) be responsible for training and orientation, management of field personnel, related files, and records; and (f) be responsible for maintaining communications, inspecting equipment, and overseeing medical incidents.	☒	☐	☐	☐	☐	
Environmental Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-14(1). The outdoor youth program adheres to land use agency requirements including sanitation and low impact camping.	☒	☐	☐	☐	☐	
R501-8-14(2). Each client is continuously supervised in the observance of low impact camping requirements.	☒	☐	☐	☐	☐	
R501-8-14(3). Personal hygiene supplies are of biodegradable materials or packed out and properly disposed of.	☒	☐	☐	☐	☐	
Evaluation	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-15(1). Following the wilderness experience, each client receives a debriefing to include a written summary of the client's participation and the progress the client achieved.	☒	☐	☐	☐	☐	
R501-8-15(2). Each guardian, client, or other involved individuals are provided the opportunity and be encouraged to submit a written evaluation of the wilderness experience, which is retained by the program in the client file record.	☒	☐	☐	☐	☐	
Individual Experiences	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

R501-8-16(1). If an outdoor youth program conducts an individual component for clients as part of the program, the program has and follows written policies and procedures, which include the following: (a) an assessment of each client's ability to safely participate in the experience; (b) a description of the individual component to ensure that each client is not exposed to an unreasonable risk; (c) guardian permission for the youth to participate in the experience; (d) a policy that individual experiences are not required and must be entered voluntarily by the client or only as clinically indicated; (e) a policy of providing preparatory instruction and guidance to the client before an individual experience; (f) an individual assessment of client readiness; (g) a description of the maximum duration of each individual experience; (h) a policy explaining that a solo experience may not be used as a punishment or general practice; (i) a policy that each staff shall be familiar with the area chosen to conduct individual experiences; (j) a supervision plan for each individual event with a frequent check-in to allow each client to rescind voluntary participation and go back to the group; (k) documentation of the duration of each individual event; (l) a plan for managing emergencies; and (m) documentation of how each individual program component is not used as seclusion or in violation of Section 62A-2-123.	☒	☐	☐	☐	☐	
Stationary Campsites	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-8-17(1). If the program offering a stationary camp that does not provide a 24-hour outdoor group living environment may require residential treatment licensure.	☐	☐	☒	☐	☐	Licensee does not use stationary campsites.
R501-8-17(2). If the outdoor youth program maintains a designated building to serve a client, the program s subject to fire, health, and safety standards. (a) A stationary outdoor youth program camp shall be inspected by a certified fire inspector before being occupied and on an annual basis thereafter. A copy of the inspection shall be maintained at the outdoor youth program camp. (b) At least one 2-A-10BC type fire extinguisher shall be in a group of tents within a 75- foot travel distance. (c) Flammable liquids may not be used to start fires, be stored in structures that house clients, or be stored near ignition sources. If generators are used, they will only be refueled by staff when the generator is not running and cool to the touch. (d) A stationary outdoor youth program camp shall be inspected by the local health department before being occupied and on an annual basis thereafter. A copy of the inspection shall be maintained at the site of the camp. (e) Food shall be stored, prepared, and served in a manner that is protected from contamination. (f) Each water supply shall be from a source that is accepted by the local health authority according to Rule R392-300, Recreation Camp Sanitation, at the time of application and for annual renewal of such licenses. (g) Sewage shall be disposed of through a public system, or in absence of a public system, in a manner approved by the local health authority, according to Rule R392-300, Recreation Camp Sanitation.	☐	☐	☒	☐	☐	Licensee does not use stationary campsites.

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Staff Files

Policy

Client Files

Interviews

Walk Through

Variable Risk
High or Extrem

Non-Residential General Provisions - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☐	☐	☐	☐	☐	
R380-600-3(3) An applicant or a provider shall permit the office to have immediate, unrestricted access to: (a) any unaltered on and off-site program or facility and client records; (b) each client who independently consents to speak to OL staff; (c) each site subject to licensing or certification; and (d) each staff member.	☐	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☐	☐	☐	☐	☐	
R380-600-3(15) The license or certificate holder shall adhere to any individualized parameter on a program or facility license or certificate to promote the health, safety, and welfare of any client. Parameters may include: (a) adequate square footage to determine capacity; (b) an admission or placement restriction; or (c) an age restriction.	☐	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☐	☐	☐	☐	☐	
R380-600-3(25) The provider shall post their current license or certificate, except in a foster home, on the premises in a place readily visible and accessible to the public.	☐	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☐	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☐	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☐	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☐	☐	☐	☐	☐	
Variances	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-6(5) The provider shall sign the approved variance and comply with the terms of the written variance, including any conditions or modifications contained within the approved written variance.	☐	☐	☐	☐	☐	
Inspection and Investigation Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-7(16) When a critical incident occurs under the direct responsibility and supervision of the program or facility, the provider shall: (a) submit a report of the critical incident to OL in a format required by OL within one business day of the critical incident occurrence; (b) additionally ensure any allegation of an incident of abuse, neglect, or exploitation of a client is reported to DCF5 for a minor client or APS for an adult client and law enforcement within 24 hours; (c) notify the parent or legal guardian of each involved client within a 24-hour period from the time of the incident; (d) if the critical incident involves any client in the custody of the department or under contract with the department, notify the involved department division immediately; and (e) collect, maintain, and submit original witness statements and supporting documentation, including video footage if available, regarding each critical incident to OL upon request.	☐	☐	☐	☐	☐	
Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

☐ Staff Files Policy Client Files		Interviews		Walk Through	Variable Risk High or Extrem		
	R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.	☐	☐	☐	☐		
	R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.	☐	☐	☐	☐		
Program Administrative and Direct Service Requirements		R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.	☐	☐	☐	☐		
	R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 26B-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; and (g) department code of conduct poster.	☐	☐	☐	☐		
	R501-1-6(3) The licensee shall maintain compliance with or documentation of an exemption from any of the following requirements: (a) a food handler permit for any person preparing meals for any other person; (b) capacity determinations that include each staff and client on premises and may not exceed the capacity limits placed by local authorities; (c) fire clearance, if conducted separately from a business license; (d) licensure and registration of any vehicles used to transport clients.	☐	☐	☐	☐		
	R501-1-6(5) The licensee shall maintain and make the following available to the department upon request: (b) general liability insurance; (c) professional liability insurance; (d) vehicle insurance; (e) fire insurance; and (f) additional insurance as required to cover each program activity.	☐	☐	☐	☐		
	R501-1-6(6) The licensee shall ensure: (b) current staff and client lists are available at each licensed site; (c) the organizational and governance structure of the program is available to the department upon request and includes: (i) line of authority and responsibility; (ii) a job description, including each duty and qualification for each job title; and (iii) notification to the office of any program changes as described in Section R380-600-3; (d) a quality improvement plan that is implemented and followed and that incorporates, at a minimum, client and staff grievances, feedback, trends in licensing noncompliance and incident reports; (e) the licensee provides an interpreter or refers each client to appropriate resources as necessary to communicate with the client; (f) the licensee maintains an opioid overdose reversal kit on-site with on duty staff trained in its use if the licensee is serving, or is likely to serve, a client with a substance use disorder.	☐	☐	☐	☐		
	R501-1-6(9) The licensee shall ensure clinical and medical staff are licensed or certified in good standing and any unlicensed staff are appropriately supervised	☐	☐	☐	☐		
Program Physical Facilities and Safety		R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.	☐	☐	☐	☐	☐	
			R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.	☐	☐	☐	☐	☐	
			R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.	☐	☐	☐	☐	☐	
			R501-1-8(4) The licensee shall maintain a first aid kit	☐	☐	☐	☐	☐	
			Food Service Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-1-10(2) A licensee that provides meals shall: (a) ensure that meals are not used as incentive or punishment; (b) provide nutritional counseling to staff and clients; (c) designate staff responsible for food service who: (i) maintain a current list of each client with special nutritional needs; and (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; (d) except in a day treatment program serving clients for less than ten hours a day, or outpatient programs serving clients for less than six consecutive hours a day, provide a variety of three nutritious meals a day that are: (i) served from dietitian or nutritionist approved menus; or (ii) for programs serving individuals experiencing homelessness, serve meals as required by USDA standard homeless settings; (f) provide adequate dining space for clients that is maintained in a clean and safe condition.	☐	☐	☐	☐	☐	
			R501-1-10(3) A licensee that allows self-serve meals shall ensure that self-serve kitchen users are supervised, directed, and trained by a staff that has a food handler's permit or is trained by Serv-Safe, USDA, or a comparable program.	☐	☐	☐	☐	☐	
			R501-1-10(4) A licensee that serves parents and their children may allow a consenting adult client to maintain full responsibility for their, and their child's, special dietary needs, if consent is maintained in writing in the client record.	☐	☐	☐	☐	☐	
			R501-1-10(5) A licensee that offers meals for clients shall ensure there is documented training confirming staff are trained to and adhere to the following safe practices: (a) how to identify and accommodate clients with special dietary needs; and (b) allowances for nutritious snacks to be available during restricted hours if the program restricts access to food and kitchen equipment.	☐	☐	☐	☐	☐	
			R501-1-10(6) If meals are prepared by clients, the licensee shall inform staff and clients in writing of the following: (a) rules and privileges of kitchen use; (b) menu planning and procedures; (c) sharing self-prepared food; (d) nutrition and sanitation requirements; (e) schedule of responsibilities; and (f) shopping and storage responsibilities.	☐	☐	☐	☐	☐	
			Program Client Record Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-1-11(1) The licensee shall maintain client information to include the following: (a) client name, address, email address, phone number, date of birth and identified gender; (b) emergency contact names, including legal guardian where applicable, and at minimum, the emergency contact's physical address, current email address or current phone numbers; (d) any information that could affect health safety or well-being of the client including each medication, allergy, chronic condition or communicable disease; (e) intake screening and assessment; (f) discharge documentation; (g) treatment or service plan; (h) progress notes and services provided with date and signature of staff completing each entry; (i) individualized assessment for restriction of access to on-site items that could be used as weapons, for self-directed violence, or as an intoxicant; (j) any referral arrangements made by the program; (k) client or guardian signed consent or court order of commitment to services in lieu of signed consent for each treatment and non-clinical service; (l) summary of attendance and absences in treatment services; (m) any grievance or complaint made by or against the client and actions taken by the program; (n) each crisis intervention or critical incident report involving the client; and (o) any signed agreement and consent form.	☐	☐	☐	☐	☐	
			Program Intake and Discharge Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-1-12(1) The licensee shall complete an intake screening before accepting a client into the program that includes at least: (a) verification that the client meets the eligibility requirements of the program; (b) verification that the client does not meet any of the exclusionary criteria that the program identified in policy as unable to serve; (c) description of presenting needs; and (d) suicide risk screening.	☐	☐	☐	☐	☐	
			R501-1-12(2) A licensee serving substance use disorder clients may not admit anyone who is unresponsive or unable to consent to care because the individual is experiencing convulsions, in shock, delirium tremens, in a coma, or unconscious.	☐	☐	☐	☐	☐	
			R501-1-12(3) A licensee serving incarcerated or court-mandated justice involved clients shall: (a) conduct a criminogenic risk assessment; (c) separate high and low criminogenic risk populations.	☐	☐	☐	☐	☐	
			R501-1-12(4) The licensee shall ensure that , the client, parent, or guardian signs and receives copies of the following agreements to be maintained as client records: (a) determination of eligibility; (b) fee agreement outlining costs of services including program, client, parent, or guardian responsibility for payment; and (c) signed consent for treatment that outlines: (i) rules of the program; (ii) expectations of clients, parents, and guardians; (iii) services to be provided; (iv) Medicaid number, insurance information, and identification of any other entities that are billed for the client's services; (v) client rights; and (vi) licensing contact information.	☐	☐	☐	☐	☐	
			R501-1-12(5) The licensee shall ensure that a discharge plan identifies resources available to a client and includes: (a) reason for discharge or transfer; (b) aftercare plan; (c) summary of services provided; and (d) progress evaluation.	☐	☐	☐	☐	☐	
			Program Clinical Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-1-14(1) A licensee that offers clinical treatment shall: (a) assign a clinical director to ensure that assessment, treatment, and service planning practices are: (i) regularly reviewed and updated; (ii) individualized; and (iii) designed to involve the participation of each client or each client's parent or guardian; (b) ensure each person working directly with a client is informed of the client's individual treatment needs and advised of the best approach to working with that client; (c) ensure client treatment plans are developed and signed by a licensed clinical professional within 30 days of admission; (d) ensure discharge goals are identified in the initial treatment plan and treatment goals are structured around the identified discharge goals and objectives; (e) ensure that each client identified for treatment receives individual treatment at least weekly; and (f) ensure any missing individual weekly treatment is justified, approved, and documented by the clinical director.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem		
	R501-1-14(2)(b) A non-residential program licensee who offers clinical treatment may alter the weekly therapy requirement as designated in the individual's treatment plan.			☐	☐	☐	☐		
	R501-1-14(4) A licensee who offers group counseling, family counseling, skills development, or other treatment shall offer and document these treatment services as prescribed in the treatment plan.			☐	☐	☐	☐		
	Program Staffing			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-1-15(1) The licensee shall ensure adequate staffing to safely supervise the current population, including adding more staff than required by the usual staffing ratio as needed to manage behaviors, dynamics, and individual client treatment and supervision needs.			☐	☐	☐	☐	☐	
	R501-1-15(2) The licensee shall identify a manager or qualified designee who is immediately available when the program is in operation or there is a qualified and trained substitute when the manager is absent or unavailable.			☐	☐	☐	☐	☐	
	R501-1-15(3) A licensee that offers clinical services shall employ or consult with licensed professional staff that include an individual who is familiar with the program and the needs of each client.			☐	☐	☐	☐	☐	
	R501-1-15(4) The licensee shall ensure that before allowing a direct care staff to work unsupervised they have an approved background clearance except as excluded in Section R501-14-17;			☐	☐	☐	☐	☐	
	R501-1-15(6) A licensee who serves a client with substance use disorder may not offer, entice, refer, or recommend medical cannabis as treatment for substance use disorder.			☐	☐	☐	☐	☐	
	R501-1-15(7) A licensee who manages, stores, or administers client medication shall identify a medical professional to oversee the medication management, medication oversight, and staff training regarding medication management and administration.			☐	☐	☐	☐	☐	
	R501-1-15(8) The licensee shall ensure that each person involved with the prescription, administration, or dispensing of controlled substances maintains appropriate medical or pharmacy licenses and DEA registration numbers			☐	☐	☐	☐	☐	
	R501-1-15(9) The licensee shall create and maintain personnel information for each staff member, contracted employee, and volunteer.			☐	☐	☐	☐	☐	
	R501-1-15(10) The licensee shall ensure that personnel information includes: (a) any applicable qualification, experience, certification, or license; (b) any approved and current office background clearance, except as excluded in Rule R501-14; (c) a provider code of conduct that is signed by the staff member, contracted employee, or volunteer; (d) any pre-service and annual training records with the date completed, topic, and the individual's signed acknowledgment of training completion; (e) any grievances or complaints made by or against the individual and actions taken by the program; & (f) each crisis intervention or critical incident report involving the individual.			☐	☐	☐	☐	☐	
	R501-1-15(11) The licensee shall ensure that at least one CPR and First Aid-certified staff member is available when staff and clients are present unless a currently licensed healthcare professional is present.			☐	☐	☐	☐	☐	
	Personnel Training Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-1-16(1) The licensee shall ensure that each staff receives pre-serving training on the following topics before being left unsupervised and within 30-days of hire: (a) program policies, procedures and safe practices as outlined in Section R501-1-5; (b) program emergency preparedness, response, and recovery plan, including at least: (i) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; and (ii) instructions to staff regarding how to report and respond to significant criminal activity and significant medical emergencies; (c) CPR and First Aid; (d) client eligibility, emphasizing the behaviors and circumstances the program can safely manage; (e) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (f) client rights; (g) supervision and ratios; (h) as applicable, medications management, storing, and administration; (i) as applicable, food handling as outlined in Subsection R501-1-10(3); (j) background checks; (k) prevention, signs and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (l) provider code of conduct as outlined in Rule 380-80; (m) non-discrimination policy in accordance with Section 26B-2-109 that includes a prohibition of abuse, discrimination, and harassment based on sex, gender identity, or sexual orientation; (n) staff and client grievance procedures; (o) crisis intervention; (p) appropriate use of restraint and seclusion; (q) de-escalation techniques; (r) appropriate searches; (s) appropriate and inappropriate behaviors of clients; (t) appropriate and inappropriate staff responses to client behaviors; and (u) if applicable, staff response to a client leaving a program without permission.			☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
	R501-1-16(2) The licensee shall ensure each staff completes the following training topics each year, based on the program's license date: (a) program policies, procedures and safe practices as outlined in Section R501-1-4; (b) general provisions and applicable categorical licensing rule; (c) client eligibility, as outlined in Subsection R501-1-6(1)(e), emphasizing the behaviors and circumstances the program can safely manage; (d) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (e) provider code of conduct as outlined in Rule R380-80; (f) program plan for the prevention or control of infectious and communicable disease to include coordination with and following any guidance of the state or local health authorities, Center for Disease Control, and the department; (g) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; (h) program rules regarding firearms that does not conflict with constitutional or statutory rights regarding concealed weapons permits as described in Title 53, Chapter 5, Part 7, Concealed Firearms Act; (i) smoking rules in accordance with Title 26B, Chapter 7, Part 5, Regulation of Smoking, Tobacco Products, and Nicotine Products; (j) how to manage clients who screen with elevated suicide risk levels; (k) general incident reporting; (l) prevention, signs, and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (m) CPR and first aid; (n) if storing and administering medications, training required to administer medication and the process to be followed.			☐	☐	☐	☐	☐	

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Staff Files

Policy

Client Files

Interviews

Walk Through

Variable Risk
High or Extrem

Congregate Care Residential Treatment Programs - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
26B-7-117(3) A person that is licensed by the department to provide residential treatment for a substance use disorder shall include as part of the person's admissions materials a question asking whether the individual seeking treatment has ever received services from a syringe exchange program.	☐	☐	☐	☐	☐	
R501-19-4(2) Any qualifying residential treatment provider: (a) may not deny any client services solely based on a client receiving medication assisted treatment; and (b) shall ensure any client continues the medication assisted treatment while receiving services from the provider.	☐	☐	☐	☐	☐	
R501-19-4(3) Each residential treatment provider shall ensure its policies include client privacy accommodation in each bedroom space while ensuring client health and safety	☐	☐	☐	☐	☐	
R501-19-4(4) Each residential treatment provider serving a child shall: (a) provide direct supervision that meets supervision and ratio requirements described in this section; (b) ensure two direct care staff are always on duty; (c) maintain a staff-to-client ratio of one staff to every four clients except: (i) as otherwise required by a Department of Health and Human Services (department) contract; or (ii) to reduce ratios to one staff to every 16 clients during client sleeping hours; (d) only decrease the number of staff as described in this section if: (i) each client is appropriately supervised to ensure health and safety at the ratio; and (ii) each direct care staff remains awake while on duty. (e) increase the staff-to-client ratio as necessary to ensure the health and safety of the current client population. (f) only allow direct care staff to perform direct supervision with line of sight check-ins every 15 minutes; (g) ensure that any direct care staff member assigned to a client's one-on-one supervision is not counted at the same time in the staffing ratio for any other client, except in an emergency situation; (h) only utilize on-site video surveillance to directly supervise a client in time out or seclusion or as an enhancement to minimum supervision ratio requirements; (i) conduct and document physical check-ins every 15-minutes when a client is being monitored by video; and (j) only use video surveillance in a bedroom: (i) with client, parent, or guardian permission; (ii) when there is a documented need; (iii) when the provider monitors any camera or physically checks in at intervals of 15 minutes or less; (iv) when video surveillance complies with R539-3 for serving an individual with disabilities.	☐	☐	☐	☐	☐	
R501-19-4(5) Each residential treatment provider serving a child may provide step-down privileges including authorized departures from the program and unsupervised time, if the provider: (a) maintains a staff-to-client ratio of one direct care staff to every four clients; (b) documents in the client record and communicates to each of the client's direct care staff: (i) the individualized justification for the step-down privileges; and (ii) which privileges are authorized by a clinical professional; (c) obtains written parental or guardian consent before allowing step-down privileges; and (d) provides a policy to each client and parent or guardian that includes: (i) a description of what constitutes authorized departure and unsupervised time; (ii) a description of how each step-down privilege, including authorized departure or unsupervised time, is achieved or rescinded; (iii) a statement that the provider will immediately communicate to each client parent or guardian and direct care staff when any step-down privilege has been rescinded; and (iv) a statement that no step-down client is allowed to perform any direct care staff duty.	☐	☐	☐	☐	☐	
R501-19-4(6) Each residential treatment provider shall make any necessary accommodation to allow a child to continue that child's education with a curriculum approved by the State Board of Education.	☐	☐	☐	☐	☐	
R501-19-4(7) Each residential treatment provider that offers education shall use a curriculum that is recognized by an educational accreditation organization, including the State Board of Education or the National School Accreditation Board.	☐	☐	☐	☐	☐	
R501-19-4(8) (a) In addition to the behavior management policy and training requirements listed in Rule R501-1, each residential treatment provider serving youth shall ensure each direct care staff member is trained through a nationally or regionally recognized curriculum and can recognize the difference between a restraint and an emergency safety intervention. (b) An emergency safety intervention may not be used as a behavioral compliance method. (c) The provider shall only use an emergency safety intervention to ensure immediate safety of a client or staff member. (d) An emergency safety intervention is subject to each requirement of a restraint for reporting, debriefing, clinical reviews, and training. (e) An emergency safety intervention may exceed the limitations of any restraint listed in Rule R501-1 with documented justification explaining why a regular restraint or other less intrusive intervention was not used	☐	☐	☐	☐	☐	
R501-19-4(10) Each residential treatment provider providing services to a substance use disorder client shall: (a) only admit a substance use disorder client with a level of care that falls within American Society of Addiction Medicine levels 3.1 through 3.5; and (b) obtain any required license before providing any service to a substance use disorder client outside of the residential setting with a level of care described in Subsection (10)(a), unless otherwise outlined in categorical rule.	☐	☐	☐	☐	☐	
R501-19-4(11) Each residential treatment provider that allows a client to participate in food preparation shall ensure the client is trained in safe food handling practices and the provider justifies the client's participation in writing.	☐	☐	☐	☐	☐	
R501-19-4(12) Each residential treatment provider shall provide individual, group, and family counseling or other treatment, including skills development, at least weekly or as outlined in the individual's treatment plan.	☐	☐	☐	☐	☐	

	R501-19-4(13) A clinical professional shall oversee any therapeutic services conducted in the therapeutic environment including: (i) life skill development; (ii) psychoeducation; and (iii) social coaching.	☐	☐	☐	☐	☐	
	R501-19-4(14) Each residential treatment provider shall document the time and date of each service provided to each client and include the signature of the individual providing the service.	☐	☐	☐	☐	☐	
	R501-19-4(15) Each residential treatment provider shall provide indoor space for free time and informal client activities.	☐	☐	☐	☐	☐	
	Requirements for Intermediate Secure Treatment	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-19-5(1)(a) Each intermediate secure treatment provider shall clearly define in policy the responsibilities of the manager described in Section R501-1-15. (b) The provider shall ensure the manager described in Subsection R501-1-15(2): (i) is at least 25 years old; (ii) has a bachelor's degree or equivalent training in a human service-related field; and (iii) has at least three years management experience in a residential or secure treatment setting.	☐	☐	☐	☐	☐	
	R501-19-5(2)(a) Subsection R501-19-4(4)(c) does not apply to an intermediate secure treatment provider serving youth. (b) An intermediate secure treatment provider serving youth shall maintain a staff-to-client ratio of one staff to every five clients.	☐	☐	☐	☐	☐	
	R501-19-5(3) Each intermediate secure treatment provider shall ensure that each direct care staff working in an intermediate secure treatment program: (a) is trained to work with a child with behavioral or mental health needs; and (b) works under the supervision of a licensed clinical professional.	☐	☐	☐	☐	☐	
	R501-19-5(4) Each intermediate secure treatment provider shall ensure each direct care staff completes 30 hours of additional training annually regarding: (a) client record and incident documentation; (b) client rules; (c) human relations and communication skills; (d) maintaining staff, client, and visitor safety in a secure setting; (e) problem-solving and guidance ; (f) the special needs of children and families; and (g) universal precautions for blood-borne pathogens.	☐	☐	☐	☐	☐	
	R501-19-5(5) Each intermediate secure treatment provider shall incorporate the use of fixtures and furnishings that help limit self-harm and suicide including: (a) non-exposed fire sprinkler heads; (b) plexiglass or safety glass; (c) pressure release robe hooks; (d) recessed lighting; and (e) sealed light fixtures.	☐	☐	☐	☐	☐	
	Specialized Services Required to Serve Clients Under the Division of Services for People with Disabilities	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-19-6(1) Each residential treatment provider serving a Division of Services for People with Disabilities (DSPD) client shall: (a) apply for any unearned income benefits for which the client is eligible, in conjunction with the support coordinator for DSPD and each client's parent or guardian; (b) develop and adhere to policies and procedures governing the daily operation and activity available and applicable to each client or visitor; (c) ensure the facility is located within a reasonable distance from a: (i) church (ii) recreation or other community facilities and (iii) school; (d) maintain a record of income and client service fees; (e) maintain a record of each fund deposited with the residential facility for client use; (f) maintain a list of each deposit and withdrawal; (g) maintain a receipt signed by the client and professional staff for any purchase over \$20; (h) maintain a record of each client's petty cash fund; (i) present each client with an individual plan that addresses appropriate day treatment; (j) share a monthly activity schedule with each client; and (k) specify, in policy, the amount of time any non-client individual may stay as an overnight guest.	☐	☐	☐	☐	☐	
	R501-19-6(2) If there is a conflict between this rule and the settings final rule, as defined in Rule R501-1, the settings final rule shall prevail.	☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extreme	
Congregate Care Residential Support Programs - Inspection Checklist <i>(Revised Jan 2026)</i>						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-3(2) The licensee that offers treatment shall obtain the appropriate categorical department license for that treatment.	☐	☐	☐	☐	☐	
R501-22-3(3) The licensee serving an individual experiencing homelessness in a setting with a contracted service provider shall identify each key decision maker and service provider that is associated with the license application and accountable for compliance with licensing rules within the licensed setting.	☐	☐	☐	☐	☐	
R501-22-3(4). Residential support may not require treatment as a condition of admission.	☐	☐	☐	☐	☐	
R501-22-3(5) The licensee shall provide evidence of ongoing coordination with the local health authorities regarding managing communicable diseases within the licensed setting.	☐	☐	☐	☐	☐	
R501-22-3(6) The licensee shall inform staff regarding: (a) various types of communicable diseases; (b) recognizing signs and symptoms of communicable diseases; and (c) steps to take when a potential disease is identified or an outbreak occurs.	☐	☐	☐	☐	☐	
Staffing	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-4(1) The licensee is not required to provide 24-hour supervision unless that program is an emergency homeless shelter or a domestic violence shelter serving adults.	☐	☐	☐	☐	☐	
R501-22-4(2) The licensee shall establish safe practices that identify each situation requiring medical attention and how the program will meet the client's medical needs.	☐	☐	☐	☐	☐	
R501-22-4(3) The licensee shall conduct eligible background clearance and document required training completion for each student or volunteer. (a) a volunteer or student who provides care without a paid staff present in any emergency homeless shelter or domestic violence shelter shall have direct communication access to designated staff and an eligible background screening before any unsupervised client access. (b) The licensee shall inform each volunteer or student verbally and in writing of program objectives and the scope of service.	☐	☐	☐	☐	☐	
Physical Facility	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-5(1) Except as otherwise provided in this section, each licensee has at least one bathroom for every ten clients.	☐	☐	☐	☐	☐	
R501-22-5(2) A domestic violence shelter and emergency homeless shelter licensee may allow family members to share a bathroom. Where a bathroom is shared by more than one family or by children over the age of eight, either the child's parent or program staff ensures that client privacy is maintained.	☐	☐	☐	☐	☐	
R501-22-5(3) Each emergency homeless shelter may exceed the bathroom ratio set forth in Subsection R501-22-5(1) if: (a) each bathroom ratio is approved by either the local authority that determines capacity or the department; (b) each bathroom ratio is specifically designated in adult-only nightly shelter settings; (c) each bathroom is inspected, cleaned, and re-stocked daily and as needed; (d) the licensee ensures individual privacy in bathing and toileting; (e) each individual with disabilities has access to at least one locking bathroom or stall; and (f) the licensee accommodates each parent's needs for changing, toileting, and bathing their children.	☐	☐	☐	☐	☐	
R501-22-5(4) The licensee shall develop safe practices to identify how to manage emergency overflow when capacity has been reached during dangerous weather conditions.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-22-5(5) The licensee shall develop safe practices that allow and encourage each client to have clean linen at least weekly.	☐	☐	☐	☐	☐	
			R501-22-5(6) The emergency homeless licensee may have portable beds, cots, or mats to accommodate fluctuating client volume.	☐	☐	☐	☐	☐	
			R501-22-5(7) Except as outlined in Subsection R501-22-5(13), the licensee shall provide clean bedding that is laundered at least weekly or as needed for each client.	☐	☐	☐	☐	☐	
			R501-22-5(8) A family may share bedroom space.	☐	☐	☐	☐	☐	
			R501-22-5(9) The licensee shall comply with the following bedroom standards for domestic violence shelters, family support centers, temporary homeless youth shelters, emergency homeless family shelters, and children's shelters: (a) there is at least 40 sq feet per client in a multiple occupant bedroom, not counting storage space or one crib for children under two years old if the crib does not inhibit access to and from the room; (b) roll away and hide-a-beds are only used when the 40 sq foot space requirement is maintained; and (c) when a bedroom is shared by more than one family, program staff make arrangements to ensure client privacy.	☐	☐	☐	☐	☐	
			R501-22-5(10) The temporary youth shelter licensee shall ensure that children in a temporary youth shelter with their own children have at least 40 square feet per person, excluding storage space, in a separately enclosed bedroom that houses only children that have their own children.	☐	☐	☐	☐	☐	
			R501-22-5(11) The licensee shall comply with the following bedroom standards for emergency homeless shelters, temporary homeless youth shelters, and receiving centers: (a) dormitory style bedrooms meet the square footage and capacity determinations made by the local fire authority; (b) capacity determinations include any staff present in the facility; (c) if the local fire authority does not identify capacity, licensing sq footage requirements apply; and (d) there is a safe practice to identify how to manage overflow when capacity has been reached.	☐	☐	☐	☐	☐	
			R501-22-5(12) Each licensee shall outline safe practices regarding: (a) rules and guidelines for each family or mixed gender sharing the same dormitory space or bedroom, including each individualized bedroom assignment; (b) securing personal belongings; (c) responsibility for each client supervising the client's own children; (d) conflict resolution; (e) nuisance and disruptive behavior; (f) housekeeping responsibilities; (g) daily schedules; (h) prohibited items; and (i) search policy.	☐	☐	☐	☐	☐	
			R501-22-5(14) Each licensee that requires a client to provide the client's own laundry supplies and locate a laundromat for laundering shall have a safe practice to assist each client on a limited basis when the client cannot provide the client's laundry supplies and locate a laundromat.	☐	☐	☐	☐	☐	
			Specialized Services for Programs Serving Client's With Substance Use Disorders	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-6(1) Each licensee may not admit anyone who is currently experiencing convulsions, shock, delirium tremens, unconsciousness, or is in a coma.	☐	☐	☐	☐	☐	
			R501-22-6(2) Each licensee serving clients with substance use disorder provides evidence of ongoing coordination with the local health authorities regarding managing communicable diseases within the licensed setting.	☐	☐	☐	☐	☐	
			R501-22-6(3) The licensee shall screen staff and clients for risk of tuberculosis.	☐	☐	☐	☐	☐	
			R501-22-6(4) A licensed substance abuse treatment program shall complete the National Survey of Substance Abuse Treatment annually.	☐	☐	☐	☐	☐	
			Specialized Services for Programs Serving Children	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-22-7(1) A licensee serving only child populations is considered congregate care as defined in Section 26B-2-101 and shall adhere to each requirement of Sections 26B-2-120, 26B-2-123, and 26B-2-124 for background clearances, policy development, and behavior management practices.	☐	☐	☐	☐	☐	
			R501-22-7(2) Congregate care rules under Title R501 apply to youth programs that retain clients past the age of 18 to complete treatment or education.	☐	☐	☐	☐	☐	
			R501-22-7(3) The licensee shall provide clean and safe age appropriate toys for children.	☐	☐	☐	☐	☐	
			R501-22-7(4) The licensee shall provide an outdoor play area enclosed with a five-foot safety fence or enclosure as otherwise required by local ordinances.	☐	☐	☐	☐	☐	
			R501-22-7(5) Only a custodial parent, legal guardian, or person designated in writing may remove any child from the program.	☐	☐	☐	☐	☐	
			R501-22-7(6) The licensee shall provide adequate staff to supervise children or be available to monitor parents supervising their own children.	☐	☐	☐	☐	☐	
			R501-22-7(7) The licensee shall comply as required with Title 80, Chapter 2, Part 9, the Interstate Compact on Placement of Children (ICPC), including ensuring the disruption plan is followed when an out of state minor presents at a shelter as a result of a failed ICPC placement in a Utah residential setting.	☐	☐	☐	☐	☐	
			Specialized Services for Domestic Violence Shelters	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-8(1) The licensee shall document that shelter rules, reason for termination and rights to confidentiality are provided to each client, verbally and in writing.	☐	☐	☐	☐	☐	
			R501-22-8(2) Each parent is responsible for supervising their own child while at the shelter. If a parent is required to be away from the shelter or involved in shelter activities without their child, the parent arranges for appropriate child care services.	☐	☐	☐	☐	☐	
			R501-22-8(3) The licensee shall ensure that each domestic violence shelter action plan documents and includes: (a) a review with each victim regarding danger and lethality and the level of the victim's risk of safety assessment; (b) a review of the victim's safety plan with each victim; (c) a review of the procedure for a protective order and a referral for the victim to the appropriate agency or clerk of the court authorized to issue the protective order; and (d) a review of supportive services for each client, including medical care, self-sufficiency, day care, legal assistance, financial assistance, and housing assistance.	☐	☐	☐	☐	☐	
			R501-22-8(4) The licensee shall assist with connecting the client to identified resources.	☐	☐	☐	☐	☐	
			R501-22-8(5) The licensee shall make and document a referral when indicated in the client record for victim treatment, psychiatric consultation, drug and alcohol treatment, or other allied service.	☐	☐	☐	☐	☐	
			R501-22-8(6) The licensee shall ensure that shelter staff completing an action plan are supervised by an experienced and trained domestic violence provider.	☐	☐	☐	☐	☐	
			Specialized Services for Temporary Homeless Youth Shelters	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-9(1) Each licensee shall provide a staff ratio of at least one direct care staff for every ten children.	☐	☐	☐	☐	☐	
			R501-22-9(2) Except as outlined in Subsection R501-22-9(4), the licensee may only admit individuals under the age of 18.	☐	☐	☐	☐	☐	
			R501-22-9(3) The licensee may admit a child with the child's own biological child.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-22-9(4) The licensee may provide shelter to an individual that is older than 18 but younger than 21 under the following conditions: (a) each individual is placed in age and gender appropriate sleeping quarters away from the minor population; (b) each individual remains in the program voluntarily and is made aware of program rules and the repercussions of criminal behavior as an adult; (c) a ratio of at least one staff to every ten clients is maintained; (d) children and individuals are assessed by a facility staff member that is a mental health therapist, as described in Section 58-60-102, to determine whether the individual is at imminent risk of harming themselves or others; and (e) individuals that are assessed as at imminent risk of harm to self or others are referred to a program qualified to serve them.	☐	☐	☐	☐	☐	
			R501-22-9(5) The licensee shall document and maintain individualized assessments of risk of harm and justification for each client admitted in the youth setting.	☐	☐	☐	☐	☐	
			R501-22-9(6) The licensee shall comply with Section 80-5-601 regarding mandatory reporting requirements for harboring a runaway.	☐	☐	☐	☐	☐	
			R501-22-9(7) The licensee shall comply with Section 26B-2-116 to coordinate educational requirements for each individual.	☐	☐	☐	☐	☐	
			R501-22-9(8) The licensee shall coordinate and transition each client to a more appropriate setting when the client cannot remain in the youth setting.	☐	☐	☐	☐	☐	
			Specialized Services for Emergency Homeless Shelters	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-10(1) An emergency shelter licensee shall prioritize the safety of those needing services and emphasize transitioning into a more permanent housing setting.	☐	☐	☐	☐	☐	
			R501-22-10(2) An emergency homeless shelter licensee shall ensure that no less than two direct care staff are always present and available and maintain a ratio of no fewer than one staff present for every 40 clients during weekday daytime hours.	☐	☐	☐	☐	☐	
			R501-22-10(3) An emergency homeless shelter may operate above staffing ratios during dangerous weather conditions, on weekends, and during sleeping hours if: (a) the program has a documented chain of command for on-call availability; (b) the program has a surveillance camera system; (c) the program has an emergency radio onsite and each staff on-duty are trained regarding how and when it is to be used; or (d) the program identifies and can rely upon other means of back up support in case of emergency.	☐	☐	☐	☐	☐	
			R501-22-10(4) In accordance with Subsections 35A-16-703(1), (2), and (3), an emergency homeless shelter licensee operating in a county where a code blue alert is in effect may increase capacity by 35% when: (a) fire code and building code capacities permit the increase; (b) procedures are implemented for expediting intake; and (c) procedures are implemented for only denying entry if the building capacity is at maximum or the individual poses a risk to the population.	☐	☐	☐	☐	☐	
			R501-22-10(5) Each emergency homeless shelter shall require each adult resident to sign an agreement form at admission that outlines the following: (a) visitors are allowed on premises to assist with housing, food stamps, assessments, religious, social and other client-specific needs; (b) participation in any meetings or groups with these visitors is voluntary; (c) each client signature on the form and voluntary participation in the visitation constitutes the client's invitation to the visitors in the department-licensed setting; and (d) each client must sign the agreement before participation in any voluntary services offered onsite.	☐	☐	☐	☐	☐	
			R501-22-10(6) Visitors as outlined in Subsection R501-22-10(5) do not include staff in the homeless shelter setting.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem		
	R501-22-10(7) The emergency homeless shelter licensee shall maintain the following information regarding each client or have documented reasons why the information is not obtainable: (a) name; (b) date of birth; (c) race; (d) ethnicity; (e) gender; (f) veteran status; (g) disabling condition; (h) start date; (i) exit date; (j) destination; (k) relationship to head of household; (l) service location; (m) prior living situation; (n) case management log and service plan, where applicable; (o) information that could affect health, safety, or well-being of the client, including medication needs; (p) documentation that is updated at 90-day intervals to include each service and contact; and (q) service plans that emphasize self-sufficiency and identify and refer to applicable sources.								
	R501-22-10(8) The shelter maintains the documentation listed in Subsection (7) for re-opening the client file, if the client returns up to 30 days past the last shelter stay, with the exception of single night stays.								
	Specialized Services for Programs Serving Clients of the Division of Services for People with Disabilities			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-22-11(1) In accordance with the federal Home and Community-Based Services (HCBS) Settings Final Rule, a program serving clients on the HCBS Waiver shall complete and adhere to the residential attestation agreement form and self-assessment survey for each licensed site.								
	R501-22-11(2) The licensee shall maintain current copies of the residential attestation agreement form and self-assessment survey forms in program documentation.								
	R501-22-11(3) In the event of a conflict between this rule and the Settings Final Rule the Settings Final Rule shall prevail.								
	R501-22-11(4) The office shall report any noncompliance of the Settings Final Rule to the DHHS Division of Continuous Quality and Improvement for contract compliance consideration.								
	Specialized Services for Receiving Centers			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-22-12(1) Each receiving center may be licensed under multiple license types to assess and triage immediate client needs.								
	R501-22-12(2) A receiving center licensee may offer short-term residential support that is intended to mitigate the initial identified problem, stabilize each client, and return each client to the community as quickly and safely as possible.								
	R501-22-12(3) A receiving center licensee shall outline safe practices in consumer agreements regarding how each population will be separated and maintained and the circumstances when interactions between populations are permitted.								
	R501-22-12(4) A receiving center licensee shall include individualized clinical documentation outlining the ongoing need and anticipated time frame for discharge for each instance that a client's stay lasts longer than 30 days.								
	R501-22-12(5) A receiving center licensee shall ensure that placement in a receiving center is a voluntary alternative that the client may choose instead of a more restrictive placement.								
	R501-22-12(6) A receiving center may not mandate treatment as a condition to residence.								

Recovery Residence Services - Inspection Checklist
(Revised Jan 2026)

R = Reviewed.
UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

R501-18-4. Legal Requirements.	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-18-4(1) The provider shall comply with Rule R501-14 for background screenings by either: (a) participating in the background clearances for all staff;	☐	☐	☐	☐	☐	
R501-18-4(2) A provider that offers clinical treatment services shall obtain a residential treatment license or applicable separate outpatient or day treatment license. (a) Clinical treatment may not be a mandatory condition of residence. (b) Clinical treatment may not be offered within the recovery residence.	☐	☐	☐	☐	☐	
R501-18-4(3) A provider that offers social detoxification services shall obtain a separate social detoxification license before offering any social detoxification services.	☐	☐	☐	☐	☐	
R501-18-4(4) A provider may not provide housing or services to a client in active withdrawal without a social detoxification license issued by the department.	☐	☐	☐	☐	☐	
R501-18-4(5) A recovery residence program shall only serve adults.	☐	☐	☐	☐	☐	
R501-18-5. Administration.	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-18-5(1) The provider shall ensure that clients receive supportive services from a person associated with the licensee or from a licensed professional.	☐	☐	☐	☐	☐	
R501-18-5(2) The provider may not offer, entice, refer, or recommend medical cannabis for residents in a recovery residence or as treatment for substance use disorder, as it is not considered a qualifying condition.	☐	☐	☐	☐	☐	
R501-18-6. Staffing.	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-18-6(1) The provider shall contract with, or otherwise provide as needed, referral information for client access to a: (a) physician; (b) psychiatrist; (c) mental health therapist (LCMHT); or (d) substance use disorder counselor (SUDC).	☐	☐	☐	☐	☐	
R501-18-6(2) The provider shall identify a director who shall have: (a) Utah licensure, in good standing, as a SUDC, licensed clinical social worker or equivalent; or (b) two years experience in one of the following: (i) administration of a recovery residence; (ii) substance use disorder treatment education; or (iii) recovery or support services education.	☐	☐	☐	☐	☐	
R501-18-6(3) The director's responsibilities that may not be delegated include: (a) monitoring each aspect of the program and operation of the facility; (b) policy and procedure development, implementation, compliance, and oversight; (c) clearly defining responsibilities of the director, manager, and staff of the program; (d) supervision and training of staff; and (e) overseeing client activities.	☐	☐	☐	☐	☐	
R501-18-6(4) The director may manage directly or employ a manager to work under the supervision of the director. (a) The director shall perform the manager's duties when the manager is on scheduled or unscheduled leave unless the manager designates a substitute manager. (b) The manager or substitute manager may not be a currently enrolled client.	☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews	Walk Through	High or Extrem			
	R501-18-6(5) In addition to the staff training requirements of Rule R501-1, the director shall maintain documentation of 40 hours of training covering the following topics for the director, manager and any direct care staff: (a) recovery services in substance use disorder settings; (b) peer support; (c) emergency overdose reversal; (d) recognition and response to substance-related activities; and (e) current certification in First Aid and CPR.		☐	☐	☐	☐		
	R501-18-6(6) An employee may not work unsupervised with clients until training is completed and documented.		☐	☐	☐	☐		
	R501-18-6(7) The director or manager shall conduct daily on-site visits to ensure client safety and support clients. (a) The director or manager shall document visits per-site, not per-client. (b) The director or manager shall use site visits to assess and document: (i) general safety; (ii) general cleanliness; (iii) verification that only admitted residents reside or stay overnight at the residence; (iv) no presence of alcohol or substances of abuse unless lawfully prescribed; and (v) that medications are in locked storage.		☐	☐	☐	☐		
	R501-18-6(8) The director or manager shall have documented face-to-face or telephone daily contact with each admitted client.		☐	☐	☐	☐		
	R501-18-6(9) The recovery residence director shall ensure there is always administrative on-call availability and remain able to respond to the recovery residence staff and the Office of Licensing (OL) immediately by phone, or at the residence in-person within one hour.		☐	☐	☐	☐		
	R501-18-6(10) The provider shall ensure a director, manager, or substitute manager is on-site seven days per week to assess safety and support clients.		☐	☐	☐	☐		
	R501-18-6(11) The provider shall schedule and document daily visits.		☐	☐	☐	☐		
R501-18-7. Direct Service.			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-18-7(1) In addition to client record requirements of Rule R501-1-21, the provider shall ensure that each recovery residence client record contains: (a) intake documentation indicating that the client meets the admission criteria, that includes: (i) the client is not currently using or withdrawing from alcohol or substances of abuse; and (ii) the client is not presenting w/a current clinical assessment contraindicating this level of care; (b) any client medications; (c) any client allergies; (d) any client chronic conditions; (e) any client communicable diseases; (f) individual recovery plan that includes: (i) documentation of each service provided by the program, including a disclosure that no clinical treatment services occur on-site at the recovery residence; and (ii) documentation of each referred supportive service, not directly associated with the recovery residence site; (g) a signed written lease agreement for the recovery residence, if required; and (h) a signed agreement indicating that the client was notified in writing before admission regarding: (i) program and client responsibilities related to transportation to and location of off-site services; (ii) program and client responsibilities related to the provision of toiletries, bedding and linens, laundry, and other household items; (iii) program and client responsibilities related to shopping, provision of food and prep of meals; (iv) fee disclosures including Medicaid number, insurance information and identification of any other entities who may be billed for the client's services; and (v) rules of the program.		☐	☐	☐	☐	☐	
	R501-18-7(2) The recovery plan shall contain the signature and title of the program representative that prepared the plan and the signature of the client.		☐	☐	☐	☐	☐	
R501-18-8. Building and Grounds.			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-18-8 In addition to the physical facility requirements of Rule R501-1, the provider shall: (1) maintain a client to toilet ratio of 1:10; (2) maintain a client to tub or shower ratio of 1:8; (3) adhere to the following if a fire clearance is not required from the local fire authority: (a) a bedroom on the ground floor shall have a minimum of one window that may be used to evacuate the room in case of fire; and (b) a bedroom that is not on the ground floor shall have a minimum of two exits, at least one of which shall exit directly to outside the building that may be used to evacuate the room in case of fire; (4) provide either equipment or reasonable access to equipment for washing and drying of linens and clothing; (5) maintain an environment free from non-prescribed substances and alcohol; (6) ensure client mail addressed to the program site is opened by the client in the presence of program staff to ensure that no contraband enters the program; and (7) ensure any unlawful items found on-site are turned over to law enforcement and a critical incident report made to OL.		☐	☐	☐	☐	☐	

R501-18-9. Medical Standards.	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-18-9(1)(a) The provider may not admit anyone who is currently in an intoxicated state, withdrawing from alcohol or drugs, or otherwise unable to understand terms and consent to reside in the recovery residence. (b) Any qualifying recovery residence provider: (i) may not deny any client services solely based on a client receiving medication assisted treatment; and (ii) shall ensure any client continues the medication assisted treatment while receiving services from the provider	☐	☐	☐	☐	☐	
R501-18-9(2) A provider shall ensure any client overseeing the management that client's medications keeps any prescription and non-prescription medication in individually accessed locked storage that is not accessible to other clients.	☐	☐	☐	☐	☐	
R501-18-9(3)(a) The provider shall ensure that a non-expired opioid reversal kit is safely maintained and available onsite. (b) The provider shall ensure staff and clients are trained on the proper use of the opioid reversal kit.	☐	☐	☐	☐	☐	
R501-18-9(4) Medical cannabis is permitted in a recovery residence if: (a) an individual has a medical cannabis card issued by the department; (b) the cannabis is obtained from a department-licensed pharmacy with identifying information on the label; and (c) the cardholder is instructed and adheres to the requirement to ensure medication is not shared and always remains locked when not in active use.	☐	☐	☐	☐	☐	

Adult Residential General Provisions - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
New and Renewal Licensing Procedures	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-3(1) Until a license or certificate is approved by OL, an applicant or provider may not: (a) accept any fee; (b) enter into any agreement to provide a client service; or (c) provide any client service.	☐	☐	☐	☐	☐	
R380-600-3(3) An applicant or a provider shall permit the office to have immediate, unrestricted access to: (a) any unaltered on and off-site program or facility and client records; (b) each client who independently consents to speak to OL staff; (c) each site subject to licensing or certification; and (d) each staff member.	☐	☐	☐	☐	☐	
R380-600-3(4) A provider may not permit a staff or client to threaten, verbally or physically abuse, or use violence of any kind while interacting with a representative of the department.	☐	☐	☐	☐	☐	
R380-600-3(15) The license or certificate holder shall adhere to any individualized parameter on a program or facility license or certificate to promote the health, safety, and welfare of any client. Parameters may include: (a) adequate square footage to determine capacity; (b) an admission or placement restriction; or (c) an age restriction.	☐	☐	☐	☐	☐	
R380-600-3(18) Unless previously approved by OL to provide services before receiving a license or certificate for special circumstances, a provider must submit an application, any required fee, and obtain a new or a renewed license or certificate before providing any service that requires a license or certificate.	☐	☐	☐	☐	☐	
R380-600-3(25) The provider shall post their current license or certificate, except in a foster home, on the premises in a place readily visible and accessible to the public.	☐	☐	☐	☐	☐	
R501-14-3(1) A provider representative shall ensure that an applicant for an initial background check completes the required application fields and disclosure statements to authorize OBP's continual monitoring of the applicant's fingerprints and applicable state registries.	☐	☐	☐	☐	☐	
R501-14-4(3)(a) The provider representative shall keep the program's roster and employee information current in DACS. (b) The provider representative shall check the roster at least monthly to verify employee information and the employment of employees due for a renewal review.	☐	☐	☐	☐	☐	
R501-14-5(2)(a) The provider representative shall submit a background check application for each applicant for an initial background check no later than two weeks from the date the applicant becomes associated with the licensee, certification, or contract. (b)(i) The provider representative shall ensure an applicant is directly supervised until OBP issues a conditional or eligible clearance determination. (ii) The provider representative shall document how the applicant remains supervised for the entirety of the applicant's supervised employment term before receiving a clearance determination.	☐	☐	☐	☐	☐	
R501-14-5(5)(a) The provider representative may not allow an applicant whose background check application is denied to have any supervised or unsupervised direct access to clients unless: (i) OBP approves a subsequent application; or (ii) the denial is overturned in an administrative hearing or by the OBP director. (b) The provider representative shall ensure an applicant initiating an appeal of a denied application works under direct supervision until OBP issues a determination regarding the appeal.	☐	☐	☐	☐	☐	
Variances	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-6(5) The provider shall sign the approved variance and comply with the terms of the written variance, including any conditions or modifications contained within the approved written variance.	☐	☐	☐	☐	☐	
Inspection and Investigation Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R380-600-7(16) When a critical incident occurs under the direct responsibility and supervision of the program or facility, the provider shall: (a) submit a report of the critical incident to OL in a format required by OL within one business day of the critical incident occurrence; (b) additionally ensure any allegation of an incident of abuse, neglect, or exploitation of a client is reported to DCFS for a minor client or APS for an adult client and law enforcement within 24 hours; (c) notify the parent or legal guardian of each involved client within a 24-hour period from the time of the incident; (d) if the critical incident involves any client in the custody of the department or under contract with the department, notify the involved department division immediately; and (e) collect, maintain, and submit original witness statements and supporting documentation, including video footage if available, regarding each critical incident to OL upon request.	☐	☐	☐	☐	☐	
Program Policies, Procedures, and Safe Practices	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem	
R501-1-4(2) The licensee shall develop, implement, and comply with safe practices that: (a) ensure client health and safety; (b) ensure the needs of the client population served are met; (c) ensure that none of the program practices conflict with any administrative rule or statute before implementation; and (d) inform staff of how to manage any unique circumstances regarding the specific site's physical facility, supervision, community safety, and mixing populations.			☐	☐	☐	☐	☐	
R501-1-4(3) The licensee shall submit any change to an office approved policy or curriculum to the office for approval before implementing the proposed change.			☐	☐	☐	☐	☐	
Residential Programs Additional Safe Practices			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-5(1) A licensee that manages, stores, or administers client medications shall develop and ensure compliance with the following medication management safe practices: (a) inform staff and clients of program and client responsibility for medication including storage and administration of medications on-site and, as applicable, when staff and clients are offsite in program related activities; (b) if applicable, inform staff and clients of the medication self-administration process; (c) if storing and administering medications, train staff to administer medication and the process to be followed; (d) how staff record medication dosages according to prescriptions; (e) how staff monitor for and record effects and side effects of medications; and (f) how staff log doses and record and report medication errors.			☐	☐	☐	☐	☐	
R501-1-5(2) The licensee shall ensure the care, vaccination, licensure, and maintenance of any animals on-site to include: (a) assessment of pet allergies for any clients interacting with animals in the program; (b) maintenance of required examinations, registrations, and vaccinations; and (c) supervision of clients in the presence of animals.			☐	☐	☐	☐	☐	
R501-1-5(4) The licensee shall ensure that a ratio of one staff to one client during transports is only utilized when the program has conducted a safety assessment that indicates that client and staff safety is reasonably assured.			☐	☐	☐	☐	☐	
Program Administrative and Direct Service Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-6(1) The licensee shall clearly identify services to the office, public, potential client, parent, or guardian regarding: (a) current and accurate contact information; (b) the complaint reporting and resolution process; (c) a description of each service provided; (d) each program requirement and expectation; (e) eligibility criteria outlining behavior, diagnosis, situation, population, and age that can be safely served, including: (i) an outline of the behaviors and presenting issues that would be reason for discharge or exclusion from the program; and (ii) a statement that the program may not take placement of a child whose needs exceed the scope or ability of the program to reasonably manage; (f) each cost, fee, and expense for a service and refund policy; and (g) identification of each non-clinical, extracurricular, or supplemental service offered or referred.			☐	☐	☐	☐	☐	
R501-1-6(2) The licensee shall post the following in conspicuous places where each visitor, staff, and client may view: (a) abuse reporting laws as described in Sections 80-2-609 and 26B-6-205; (b) civil rights notice; (c) Americans with Disabilities Act notice; (e) any office notice of agency action; (f) a client rights poster in a residential setting except in a foster home or where prohibited by Settings Final Rule; and (g) department code of conduct poster.			☐	☐	☐	☐	☐	
R501-1-6(3) The licensee shall maintain compliance with or documentation of an exemption from any of the following requirements: (a) a food handler permit for any person preparing meals for any other person; (b) capacity determinations that include each staff and client on premises and may not exceed the capacity limits placed by local authorities; (c) fire clearance, if conducted separately from a business license; (d) licensure and registration of any vehicles used to transport clients.			☐	☐	☐	☐	☐	
R501-1-6(5) The licensee shall maintain and make the following available to the department upon request: (b) general liability insurance; (c) professional liability insurance; (d) vehicle insurance; (e) fire insurance; and (f) additional insurance as required to cover each program activity.			☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem
R501-1-6(6) The licensee shall ensure: (b) current staff and client lists are available at each licensed site; (c) the organizational and governance structure of the program is available to the department upon request and includes: (i) line of authority and responsibility; (ii) a job description, including each duty and qualification for each job title; and (iii) notification to the office of any program changes as described in Section R380-600-3; (d) a quality improvement plan that is implemented and followed and that incorporates, at a minimum, client and staff grievances, feedback, trends in licensing noncompliance and incident reports; (e) the licensee provides an interpreter or refers each client to appropriate resources as necessary to communicate with the client; (f) the licensee maintains an opioid overdose reversal kit on-site with on duty staff trained in its use if the licensee is serving, or is likely to serve, a client with a substance use disorder.							
R501-1-6(9) The licensee shall ensure clinical and medical staff are licensed or certified in good standing and any unlicensed staff are appropriately supervised							
Residential Program Additional Administration and Direct Services Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes	
R501-1-7(1) A residential program licensee shall additionally: (a) ensure each staff shift list remains current and available to the office upon request; (b) ensure that each shift documents any illness, injury or critical incident and passes it on to the next shift and administration; (d) ensure access to a medical clinic or a medical professional familiar with the program and population served; and (e) provide a separate space for clients who are showing symptoms of an infectious disease.							
R501-1-7(6)(a) A residential program licensee, excluding a residential treatment program, may allow for client independence and responsibility for their own supplies, food, laundry, or transportation by outlining in writing resources and responsibility for the provision of these items. (b) Each residential program licensee shall assist clients on a limited basis if they are temporarily unable to provide the items or services listed in Subsection R501-1-7(6)(a) for themselves.							
Program Physical Facilities and Safety	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes	
R501-1-8(1) The licensee shall ensure: (a) the appearance & cleanliness of the building/grounds are maintained & free from health/fire hazards; (b) any appliances, plumbing, electrical, HVAC, and furnishings are maintained in operating order and in a clean and safe condition; (c) fire drills in non-outpatient programs are conducted at least quarterly and documented, including feedback regarding response time and process; (d) a phone that can be used to call 911 is always available on-site when clients are present; (e) bathroom facilities for staff and clients allow for individual privacy and afford reasonable accommodation based on gender identity; (f) each bathroom is properly equipped with toilet paper, paper towels or a dryer, and soap; (g) each bathroom is ventilated by mechanical means or equipped with a window that opens; (h) non-prescription medication, if stored on-site, is stored in original manufacturer's packaging together with the manufacturer's directions and warnings; and (i) prescription medication, if stored on-site, is stored in original pharmacy packaging or individual pharmacy bubble pack together with the pharmacy label, directions, and warnings.							
R501-1-8(2) The licensee shall accommodate a client with physical disabilities as needed or appropriately refer to comparable services.							
R501-1-8(3) The licensee shall maintain medication and potentially hazardous items on-site lawfully, responsibly, and with consideration of the safety and risk level of the population served to include locked storage for each medication and hazardous chemical that is not in active use.							
R501-1-8(4) The licensee shall maintain a first aid kit							
Residential Program Additional Facilities and Safety Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes	

Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem	
R501-1-9(1) A residential licensee shall ensure: (a) designated space is available for records, administrative work, & confidential phone calls for clients; (c) live-in staff have dedicated bedrooms & bathrooms separate from client use; (d) each bedroom designated for a client is comparable to other similarly utilized bedrooms with similar access, location, space, furnishings, and furnishings; (e) clients are not locked in bedrooms; (f) a mirror or safety mirror is secured to each bathroom wall at a convenient height; (g) each bathroom is placed to allow access to each client without disturbing any other client during sleeping hours; (h) each bath or shower allows for individual privacy; (i) each client is supplied with hygiene supplies; (j) each sleeping area has a source of natural light and is ventilated by mechanical means or is equipped with a window that opens; (k) each client has a similar solid type of bed or sleeping equipment to any other client in the program; (l) each client is allowed to decorate & personalize their bedroom, while maintaining respect for other residents and property; (m) there are separate containers for soiled & clean laundry, if the program provides common laundry for towels, bedding or clothing; (n) bedding & towels are laundered weekly & after each client is discharged; (o) equipment and supplies for washing & drying laundry are provided, if the program permits clients to do their own laundry; (p) there is at least 60 sq ft per person in a multiple occupancy bedroom and 80 sq ft in a single occupant bedroom.			☐	☐	☐	☐	☐	
R501-1-9(2) A residential program licensee serving individuals with disabilities shall house no more than two clients in each bedroom.			☐	☐	☐	☐	☐	
R501-1-9(3) The licensee utilizing seclusion rooms shall ensure the following: (a) seclusion rooms measure a minimum of 75 sq ft and have a minimum ceiling height of 7 ft with no equipment, hardware or furnishings that obstruct staff's view of the client or present a hazard; (b) a seclusion room shall have either natural or mechanical ventilation with break resistant windows and either a break resistant two-way mirror or camera that allows for observation of the entire room; (c) a seclusion room may not have locking capability and may not be located in closets, bathrooms, unfurnished areas or other areas not designated as part of residential living space; and (d) a bedroom may not be utilized as a seclusion room and a seclusion room may not be utilized as a bedroom.			☐	☐	☐	☐	☐	
R501-1-9(5) The licensee shall train staff and ensure that the use of any alternate sleeping arrangements other than the client's assigned bedroom complies with Subsection R501-1-4(7) and: (a) preserves client dignity and confidentiality; (b) shall be done on an individualized, time delimited basis; (c) may not be utilized due to staffing shortages or for staff convenience; and (d) may not be used as behavior management or consequence.			☐	☐	☐	☐	☐	
Food Service Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-1-10(2) A licensee that provides meals shall: (a) ensure that meals are not used as incentive or punishment; (b) provide nutritional counseling to staff and clients; (c) designate staff responsible for food service who: (i) maintain a current list of each client with special nutritional needs; and (ii) ensure that each client with special nutritional needs has food storage and a preparation area that is not exposed to any identified allergen or contaminant; (d) except in a day treatment program serving clients for less than ten hours a day, or outpatient programs serving clients for less than six consecutive hours a day, provide a variety of three nutritious meals a day that are: (i) served from dietitian or nutritionist approved menus; or (ii) for programs serving individuals experiencing homelessness, serve meals as required by USDA standard homeless settings; (f) provide adequate dining space for clients that is maintained in a clean and safe condition.			☐	☐	☐	☐	☐	
R501-1-10(3) A licensee that allows self-serve meals shall ensure that self-serve kitchen users are supervised, directed, and trained by a staff that has a food handler's permit or is trained by Serv-Safe, USDA, or a comparable program.			☐	☐	☐	☐	☐	
R501-1-10(4) A licensee that serves parents and their children may allow a consenting adult client to maintain full responsibility for their, and their child's, special dietary needs, if consent is maintained in writing in the client record.			☐	☐	☐	☐	☐	
R501-1-10(5) A licensee that offers meals for clients shall ensure there is documented training confirming staff are trained to and adhere to the following safe practices: (a) how to identify and accommodate clients with special dietary needs; and (b) allowances for nutritious snacks to be available during restricted hours if the program restricts access to food and kitchen equipment.			☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem	
		R501-1-10(6) If meals are prepared by clients, the licensee shall inform staff and clients in writing of the following: (a) rules and privileges of kitchen use; (b) menu planning and procedures; (c) sharing self-prepared food; (d) nutrition and sanitation requirements; (e) schedule of responsibilities; and (f) shopping and storage responsibilities.	☐	☐	☐	☐	☐	
Program Client Record Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
		R501-1-11(1) The licensee shall maintain client information to include the following: (a) client name, address, email address, phone number, date of birth and identified gender; (b) emergency contact names, including legal guardian where applicable, and at minimum, the emergency contact's physical address, current email address or current phone numbers; (c) a program serving substance use disorder clients shall maintain compliance with an initial and annual client tuberculosis screening results in each client record; (d) any information that could affect health safety or well-being of the client including each medication, allergy, chronic condition or communicable disease; (e) intake screening and assessment; (f) discharge documentation; (g) treatment or service plan; (h) progress notes and services provided with date and signature of staff completing each entry; (i) individualized assessment for restriction of access to on-site items that could be used as weapons, for self-directed violence, or as an intoxicant; (j) any referral arrangements made by the program; (k) client or guardian signed consent or court order of commitment to services in lieu of signed consent for each treatment and non-clinical service; (l) summary of attendance and absences in treatment services; (m) any grievance or complaint made by or against the client and actions taken by the program; (n) each crisis intervention or critical incident report involving the client; and (o) any signed agreement and consent form.	☐	☐	☐	☐	☐	
Program Intake and Discharge Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
		R501-1-12(1) The licensee shall complete an intake screening before accepting a client into the program that includes at least: (a) verification that the client meets the eligibility requirements of the program; (b) verification that the client does not meet any of the exclusionary criteria that the program identified in policy as unable to serve; (c) description of presenting needs; and (d) suicide risk screening.	☐	☐	☐	☐	☐	
		R501-1-12(2) A licensee serving substance use disorder clients may not admit anyone who is unresponsive or unable to consent to care because the individual is experiencing convulsions, in shock, delirium tremens, in a coma, or unconscious.	☐	☐	☐	☐	☐	
		R501-1-12(3) A licensee serving incarcerated or court-mandated justice involved clients shall: (a) conduct a criminogenic risk assessment; (c) separate high and low criminogenic risk populations.	☐	☐	☐	☐	☐	
		R501-1-12(4) The licensee shall ensure that , the client, parent, or guardian signs and receives copies of the following agreements to be maintained as client records: (a) determination of eligibility; (b) fee agreement outlining costs of services including program, client, parent, or guardian responsibility for payment; and (c) signed consent for treatment that outlines: (i) rules of the program; (ii) expectations of clients, parents, and guardians; (iii) services to be provided; (iv) Medicaid number, insurance information, and identification of any other entities that are billed for the client's services; (v) client rights; and (vi) licensing contact information.	☐	☐	☐	☐	☐	
		R501-1-12(5) The licensee shall ensure that a discharge plan identifies resources available to a client and includes: (a) reason for discharge or transfer; (b) aftercare plan; (c) summary of services provided; and (d) progress evaluation.	☐	☐	☐	☐	☐	
Residential Additional Program Intake and Discharge Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem
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Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem
	R501-1-15(10) The licensee shall ensure that personnel information includes: (a) any applicable qualification, experience, certification, or license; (b) any approved and current office background clearance, except as excluded in Rule R501-1-4; (c) a provider code of conduct that is signed by the staff member, contracted employee, or volunteer; (d) any pre-service and annual training records with the date completed, topic, and the individual's signed acknowledgment of training completion; (e) any grievances or complaints made by or against the individual and actions taken by the program; & (f) each crisis intervention or critical incident report involving the individual.		☐	☐	☐	☐	
	R501-1-15(11) The licensee shall ensure that at least one CPR and First Aid-certified staff member is available when staff and clients are present unless a currently licensed healthcare professional is present.		☐	☐	☐	☐	
	Personnel Training Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-1-16(1) The licensee shall ensure that each staff receives pre-serving training on the following topics before being left unsupervised and within 30-days of hire: (a) program policies, procedures and safe practices as outlined in Section R501-1-5; (b) program emergency preparedness, response, and recovery plan, including at least: (i) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; and (ii) instructions to staff regarding how to report and respond to significant criminal activity and significant medical emergencies; (c) CPR and First Aid; (d) client eligibility, emphasizing the behaviors and circumstances the program can safely manage; (e) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (f) client rights; (g) supervision and ratios; (h) as applicable, medications management, storing, and administration; (i) as applicable, food handling as outlined in Subsection R501-1-10(3); (j) background checks; (k) prevention, signs and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (l) provider code of conduct as outlined in Rule 380-80; (m) non-discrimination policy in accordance with Section 26B-2-109 that includes a prohibition of abuse, discrimination, and harassment based on sex, gender identity, or sexual orientation; (n) staff and client grievance procedures; (o) crisis intervention; (p) appropriate use of restraint and seclusion; (q) de-escalation techniques; (r) appropriate searches; (s) appropriate and inappropriate behaviors of clients; (t) appropriate and inappropriate staff responses to client behaviors; and (u) if applicable, staff response to a client leaving a program without permission.		☐	☐	☐	☐	
	R501-1-16(2) The licensee shall ensure each staff completes the following training topics each year, based on the program's license date: (a) program policies, procedures and safe practices as outlined in Section R501-1-4; (b) general provisions and applicable categorical licensing rule; (c) client eligibility, as outlined in Subsection R501-1-6(1)(e), emphasizing the behaviors and circumstances the program can safely manage; (d) staff involvement and responsibility in the intake, discharge, and unplanned discharge processes; (e) provider code of conduct as outlined in Rule R380-80; (f) program plan for the prevention or control of infectious and communicable disease to include coordination with and following any guidance of the state or local health authorities, Center for Disease Control, and the department; (g) emergency procedures to instruct staff how to address incident reporting, continuity of care, transport, relocation, and client health and safety during natural disasters, extreme weather events, fire, utility or structural failures, or other unexpected disruptions to the program service; (h) program rules regarding firearms that does not conflict with constitutional or statutory rights regarding concealed weapons permits as described in Title 53, Chapter 5, Part 7, Concealed Firearms Act; (i) smoking rules in accordance with Title 26B, Chapter 7, Part 5, Regulation of Smoking, Tobacco Products, and Nicotine Products; (j) how to manage clients who screen with elevated suicide risk levels; (k) general incident reporting; (l) prevention, signs, and symptoms of abuse and neglect, including sexual abuse, and legal reporting requirements; (m) CPR and first aid; (n) if storing and administering medications, training required to administer medication and the process to be followed; (o) training to identify and address in a residential or congregate care program: (i) clients who pose a risk of violence; (ii) what constitutes contraband, possession of contraband, and how the program ensures restriction of client access to contraband		☐	☐	☐	☐	

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem
	and dangerous weapons or materials; (iii) clients who are at risk for suicide; (iv) managing clients with mental health concerns; and (v) identifying the signs and symptoms of clients presenting under the influence of substances or alcohol; (p) if the licensee manages funds for client allowances; training to document each expense; and (q) appropriate use of any alternate sleeping arrangements in a residential or congregate care program.							



Staff Files	Policy	Client Files	Interviews	Walk Through	Multi Risk inc High or Extrem	
Day Treatment Programs - Inspection Checklist (Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration and Direct Service	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-20-4(1) In addition to the following rules, each day treatment licensee shall comply with Rules R501-1 and R501-14.	☐	☐	☐	☐	☐	
R501-20-4(2) Non-residential licensees may meet less frequently than weekly only with individualized justification in the client record.	☐	☐	☐	☐	☐	
R501-20-4(3) A licensee shall: (a) ensure that activity plans are prepared to meet individual client needs or link to applicable resources in the community; (b) design and implement a daily activity or service schedule based on client needs and not staff convenience; (c) ensure clients are supervised onsite and encouraged to participate in activities; and (d) ensure all clients are given the same quality of care.	☐	☐	☐	☐	☐	
R501-20-4(4) Except as outlined in Subsections R501-20-6(2) and R501-20-7(3), a licensee shall ensure that the minimum staffing ratio is one direct care staff to ten clients at all times.	☐	☐	☐	☐	☐	
R501-20-4(5) Day treatment or day support services may not be offered within a residential setting unless: (a) each resident is a recipient of the day support services; or (b) the residential setting has a current residential treatment license.	☐	☐	☐	☐	☐	
R501-20-4(6) A licensee who utilizes restraints shall report each incident of restraint resulting in injury beyond basic first aid to the office as a critical incident.	☐	☐	☐	☐	☐	
Physical Facility	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-20-5(1) Except as outlined in Subsection R501-20-6(3), a licensee shall ensure that the facility has a minimum of 50 square feet of floor space per client designated specifically for day treatment services. Hallways, office, storage, kitchens, and bathrooms may not be included in computation.	☐	☐	☐	☐	☐	
R501-20-5(2) A licensee shall ensure that outdoor recreational space and compatible recreational equipment are available when necessary to meet treatment plans.	☐	☐	☐	☐	☐	
Additional Considerations for Professional Licensees Providing ABA Day Treatment Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-20-6(1) An ABA licensee shall additionally adhere to Rule R539-4, if contracted for providing services to DSPD clients.	☐	☐	☐	☐	☐	
R501-20-6(2) A licensee shall ensure that behavior support plans outline individual behaviors and staff responses to them.	☐	☐	☐	☐	☐	
R501-20-6(3) The licensee shall submit a critical incident to the office for: (a) any self-directed violence not identified in the behavior support plan; and (b) any staff responses outside of the behavior support plan.	☐	☐	☐	☐	☐	
Additional Considerations for Licensees Providing Clubhouse Day Treatment Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-20-7(2)(a) A clubhouse licensee accredited by Clubhouse International may apply a staff to client ratio of 1 staff to 15 clients in accordance with national standards. (b) A clubhouse licensee may apply the higher ratio only for specialized activities involving transports or for clients and their guests when: (i) staff and client safety has been assessed; and (ii) there is identified back-up for the staff in case of emergency.	☐	☐	☐	☐	☐	
R501-20-7(3) Square footage calculations in a clubhouse may include hallways, office, storage, kitchens, and bathrooms.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Multi Risk inc High or Extrem		
	R501-20-7(4) A clubhouse licensee may offer clients the option to bring their own food or purchase meals or snacks at a reduced rate.			☐	☐	☐	☐	☐	
	R501-20-7(5) A clubhouse licensee may allow clients cleaning chemicals as part of their work-ordered day. Clubhouse staff shall follow suicide prevention policy and safety protocols when assessing and allowing client access to chemicals.			☐	☐	☐	☐	☐	
	R501-20-7(6) Visitors to the clubhouse may only be exempt from background clearance in accordance with Subsection 26B-2-120(10).			☐	☐	☐	☐	☐	
	Additional Considerations for DSPD Home and Community Based Medicaid Waiver Licensees			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-20-8(2) A licensee serving clients of DSPD shall ensure staff to client ratios are determined by the DSPD worksheet and are individualized based on the person's need.			☐	☐	☐	☐	☐	
	R501-20-8(3) A licensee shall ensure a ratio of one staff to six persons is maintained at all times.			☐	☐	☐	☐	☐	
	R501-20-8(5) A licensee shall identify any community-based supports provided under the day treatment license and ensure that community-based services are provided safely and in consideration of weather, transportation, emergencies and overall client needs for food, medicine and any other assistance necessary for safe participation in the program.			☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews	Walk Through		
Outpatient Treatment Programs - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed.						
UR = Under Review. Under Review indicates evidence of potential noncompliance.						
NA = Not Assessed during this inspection.						
Administration and Direct Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-21-4(1) A provider shall: (a) provide general outpatient treatment: (i) on a weekly basis, or (ii) only with individualized clinical justification, less than weekly; (b) only provide intensive outpatient treatment, if offered, for: (i) between nine and 19 hours weekly for adults; or at least six hours weekly for youth; and (c) ensure any client present in the facility for six or more consecutive hours; (i) receives meals and any required medication; and (ii) is placed in the appropriate group size according to building capacity and a physical environment that provides for the comfort of each clients	☐	☐	☐	☐	☐	
R501-21-4(2) (a) An outpatient treatment provider that provides only telehealth services may apply for a single license for one centralized site to cover any telehealth services offered. (b) An outpatient treatment provider that provides only telehealth services shall ensure that any telehealth services provided to an out of state client are done in accordance with the telehealth laws of the client's state of residence.	☐	☐	☐	☐	☐	
Substance Use Disorder Treatment Programs	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-21-5(1) Each provider shall: (a) develop and implement a plan on how to support opioid overdose reversal; (b) maintain proof of completion of the National Survey of Substance Abuse Treatment Services annually; and (c) ensure medical cannabis is not used as an enticement for a client or offered, referred, or recommended as treatment for substance use disorder.	☐	☐	☐	☐	☐	
R501-21-5(2) A provider providing medication for opioid use disorder (MOUD) shall: (a) maintain a program-wide ratio of one counselor for every 65 clients to provide adequate substance use counseling to each client as clinically necessary; and (b) ensure that, at least once yearly, each client sees a licensed practitioner that may prescribe controlled substances.	☐	☐	☐	☐	☐	
R501-21-5(3) Each MOUD provider that prescribes, administers, or dispenses methadone shall: (a) admit a client to the program only after the completion of a face-to-face visit with a licensed practitioner authorized to prescribe controlled substances who confirms that client's opioid dependence; (b) ensure that a licensed practitioner authorized to prescribe controlled substances approves every subsequent dose increase before the change; (c) require each client admitted to the program to participate in random drug testing performed at least eight times a year for each client in maintenance treatment, in accordance with generally accepted clinical practice and with 42 CFR 8 (2025); and (d) require and document at least one hour of prescribing practitioner time at the program site each month for every ten enrolled MOUD clients.	☐	☐	☐	☐	☐	
R501-21-5(4) Each MOUD program that prescribes, administers or dispenses methadone shall: (a) maintain Substance Abuse and Mental Health Services Administration certification and accreditation as an opioid treatment program; (b) employ: (i) a licensed physician who is an ASAM-certified physician; (ii) a prescribing licensed practitioner who can document specific training in current industry standards regarding methadone treatment for opioid addiction; or (iii) a prescribing licensed practitioner who can document specific training or experience in methadone treatment for opioid addiction; and (c) provide one covered provider, as defined in Section 58-17b-309.7, to dispense or administer medication for every 150 methadone clients dosing on an average of a daily basis.	☐	☐	☐	☐	☐	
R501-21-6(5) A provider may offer services from a mobile unit, as defined in Section 58-17b-309.7, under the provider's physical site license if: (a) the existing licensed site provides MOUD services; (b) the provider maintains policy and procedures in accordance with department policy applicable to a mobile unit and Rule R523-10; and (c) the provider meets registration requirements of 21 CFR 1300, 1301, and 1304 (April 1, 2021), published by the Office of the Federal Register, incorporated by reference in this rule.	☐	☐	☐	☐	☐	
R501-21-5(6) An alcohol and drug education provider may only provide court-ordered education if certified to do so through the OSUMH in accordance with Rule R523-11.	☐	☐	☐	☐	☐	
R501-21-5(7) A licensed substance use disorder counselor (SUDC) in a substance use disorder outpatient treatment program may: (a) collect client information; (b) conduct the screening portion of an assessment; (c) make level of care recommendations; and (d) identify a substance use disorder.	☐	☐	☐	☐	☐	
R501-21-5(8) A SUDC may not diagnose a client.	☐	☐	☐	☐	☐	
R501-21-5(9) A OSUMH-certified alcohol and drug education provider shall: (a) complete and maintain a substance use screening; (i) for each client before providing the education course; and (ii) that may be shared between providers with written client consent; (b) provide a workbook to each client to keep upon completion of the course; (c) ensure at least 16 hours of course education; and (d) provide separate classes for adults and youth.	☐	☐	☐	☐	☐	
R501-21-5(10) A provider offering services to a justice-involved client shall: (a) operate in compliance with Rule R523-3; (b) maintain a validated criminogenic screen or risk assessment for each justice-involved client which is conducted with an accepted tool, including: (i) Level of Service Inventory-Revised (LSI-R); (ii) Risk and Needs Triage (RANT); (iii) Ohio Risk Assessment System (ORAS); or (iv) any other screen that the provider can demonstrate the validation to the OSUMH; (c) separate clients into treatment groups according to each client's assessed level of risk; (d) complete screenings that assess substance abuse and mental health comorbidity; and (e) treat, or refer to another licensed department program that serves justice-involved clients to treat, an array of disorders noted in the screening.	☐	☐	☐	☐	☐	
Domestic Violence Programs	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-21-6(1) A domestic violence (DV) treatment provider shall comply with generally accepted and current practices in DV treatment.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	
	R501-21-6(2) A DV treatment provider shall: (a) maintain and document cooperative working relationships with any DV shelter, treatment program, referring agency, local DV coalition, and custodial parent when the client is a minor; (b) offer at least ten sessions of treatment for any child or victim, not including intake or orientation; (c) if the client is a perpetrator, contact any victim, current partner, or required criminal justice referring agency; and (d) ensure a licensed mental health therapist, as defined in Section 58-60-102, completes a DV treatment evaluation for each client who is a perpetrator that includes individualized recommendations for that client's treatment.			☐	☐	☐	☐
	R501-21-6(3) For groups, a provider shall ensure there is at least one staff for: (a) there is at least one staff for every eight clients in an adult treatment group for up to an hour; (b) every ten clients in an adult treatment group exceeding one hour; (c) every eight children in a child victim or child witness group where each child client is under 12 years old; and (d) every ten children in a child victim or child witness group where each child client is at least 12 years old.			☐	☐	☐	☐
	R501-21-6(4) For client safety, the provider shall: (a) ensure that: (i) when a client enters a DV treatment program, the staff conduct an in-depth, face-to-face interview and intake assessment to determine the client's clinical profile and treatment needs; and (ii) when the client is a perpetrator, the evaluation described in Subsection (2)(d) ; (b) obtain additional information for any client who is a perpetrator from any police incident report, criminal history of the client, prior treatment provider, victim, or victim advocate; (c) when appropriate, obtain additional information for a child client from any parent, prior treatment provider, school, or the Division of Child and Family Services Child Protective Services; (d) when any document listed in Subsections (4)(a) through (c) cannot be obtained, document the reason; and (e) ensure that the intake assessment or evaluation described in Subsection (2)(d) includes: (i) a profile of the frequency, severity, and duration of the DV behavior, including a summary of any psychological violence; (ii) documentation of any homicidal, suicidal ideation and intentions, or abusive behavior toward children; (iii) a clinical diagnosis and a referral for evaluation to determine the need for medication, if indicated; (iv) documentation of safety planning when: (A) the client is an adult victim, child victim, or child witness; and (B) that client has contact with the perpetrator; (v) documentation of a safety plan for any victim who chooses not to become a treatment client; and (vi) documentation that appropriate measures have been taken to protect children from harm.			☐	☐	☐	☐
	R501-21-6(5) For treatment procedures, a provider shall: (a) create an individualized treatment plan addressing relevant treatment issues for each client; (b) refer each client deemed not appropriate for a DV program to the appropriate resource or program, with any reason for referral documented, and notify the referred agency; (c) provide DV counseling concurrently with, or after, other necessary treatment when appropriate; (d) ensure that any client who is a perpetrator, or a co-perpetrator, is not provided conjoint or group therapy in the same session as any victim who has experienced violence from the perpetrator or co-perpetrator until staff has completed a comprehensive assessment to determine that: (i) any violence between the client who is a perpetrator, or a co-perpetrator, and the victim has stopped; and (ii) conjoint therapy is appropriate; (e) require the client who is a perpetrator to complete at least four DV treatment sessions, unless otherwise noted in the evaluation recommendations described in Subsection (2)(d), before the provider implements conjoint therapy; (f) implement written procedure in an efficient and timely manner to facilitate any: (i) entry of a court-ordered defendant into treatment as a client; (ii) notification of client compliance, participation, or completion; (iii) disposition of a non-compliant client; (iv) notification of the recurrence of violence; and (v) notification of factors that may exacerbate a client's potential for violence; (g) comply with the duty to warn; (h) document specialized training in DV assessment and treatment practices for any individual providing treatment service, including: (i) completing 24 hours of Utah Association for Domestic Violence Treatment pre-service training; (ii) completing 16 hours annual training after the initial training in Subsection (5)(h)(i); and (iii) at least one hour per week of clinical supervision for any treatment staff that is not clinically licensed to discuss clinical dynamics of a case.			☐	☐	☐	☐
	R501-21-6(6) The provider shall ensure training is documented and approved by the designated specialist regarding assessment and treatment practices for treating DV victims and perpetrators.			☐	☐	☐	☐
	R501-21-6(7) A provider shall disclose any: (a) current department contracts; or (b) action against the contract to the Office of Licensing.			☐	☐	☐	☐
	R501-21-6(8) A provider shall disclose any current accreditation or action against accredited status to the Office of Licensing.			☐	☐	☐	☐

ADDITIONAL INFORMATION

[illegible]

Social Detoxification Programs - Inspection Checklist
(Revised Jan 2026)

R = Reviewed.
UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

Physical Facility	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-11-5(2) Each provider shall ensure that the physical facility is compliant with the following: (a) 24 hour live-in staff have a separate living space with a private bathroom; (b) a large room may be used as a dormitory style bedroom; (c) a minimum of 50 sq ft per individual, excluding storage space, is provided for a multiple occupant bedroom; (d) a minimum of 70 sq ft per individual, excluding storage space is provided for a single occupant bedroom; (e) there is an escape window for each sleeping room unless there are two ways to exit the room; & (f) each bathroom meets a minimum ratio of one toilet, one sink, and one tub or shower for each eight residents.	☐	☐	☐	☐	☐	
Specialized Services	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-11-6(1) Social detoxification services shall serve clients who require a clinical managed residential withdrawal management level of care only until they can be stabilized and transition to a lower level of care, or medical necessity requires moving the individual to a higher level of care.	☐	☐	☐	☐	☐	
R501-11-6(2) A provider wishing to provide medically monitored inpatient withdrawal management under a social detoxification license may only do so under the following conditions: (a) medical and nursing professionals provide 24-hour medically monitored evaluation and withdrawal management under physician-approved policies and physician-monitored procedures and protocols; (b) justification is documented for how clients served at this level do not require services at the level of either a higher or lower level of care; (c) the individual meets the admission and continued service criteria for medically monitored withdrawal management; and (d) the program meets each requirement for medically monitored withdrawal management.	☐	☐	☐	☐	☐	
R501-11-6(3) The provider shall complete a preliminary screening when an individual presents for service to determine appropriateness for social model detox and shall complete the subsequent intake evaluation within seven days.	☐	☐	☐	☐	☐	
R501-11-6(4) The provider shall require a client to provide recent evidence of a Tuberculosis screening or get tested for Tuberculosis within one week of presenting for service.	☐	☐	☐	☐	☐	
R501-11-6(5) The provider shall ensure: (a) a client who exhibits signs of possible active Tuberculosis is screened by the provider immediately with assistance from the local health department; and (b) any local health department recommendations are followed.	☐	☐	☐	☐	☐	
R501-11-6(6) Once a client has completed the acute detoxification period as demonstrated by reasonable physical and psychological stability, the provider shall conduct an evaluation to determine the treatment referral.	☐	☐	☐	☐	☐	

Adult Day Care - Inspection Checklist
 (Revised Jan 2026)

R = Reviewed.
UR = Under Review. Under Review indicates evidence of potential noncompliance.
NA = Not Assessed during this inspection.

Record Keeping	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-13-2.(1)(a) In addition to record keeping requirements in Rule R501-1, the director shall maintain the following onsite at all times: (b) daily client attendance records; and (c) a current health assessment signed by a physician for each client.	☐	☐	☐	☐	☐	
Activity Plans	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-13-3(1) Each licensee shall prepare daily activity plans that meet individual client and group physical, social, psychological, or emotional, development. These plans may include: (a) community living skills; (b) work activity; (c) recreation; (d) nutrition; (e) personal hygiene; or (f) social skills.	☐	☐	☐	☐	☐	
R501-13-3(2) The licensee shall maintain activity plans on file and orient staff to their use.	☐	☐	☐	☐	☐	
R501-13-3(3) There shall be a daily schedule posted and implemented onsite.	☐	☐	☐	☐	☐	
R501-13-3(4) Each client shall have the opportunity to use 4 of the following activity areas each day: (a) general activities; (b) sedentary activities; (c) specialized activities; (d) rest area; (e) self-care area; (f) appointed outdoor area; (g) kitchen and nutrition area; or (h) reality orientation area.	☐	☐	☐	☐	☐	
Staffing	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-13-4(1) The licensee shall ensure that staff to client ratios meet the following: (a) except as outlined in Subsection R501-13-4(1)(e), eight or less clients require one staff person to provide direct supervision at all times with a second staff person meeting minimum staff requirements immediately available; (b) nine to sixteen clients require two staff providing direct supervision at all times; (c) sixteen or more clients require a staffing ratio of one staff to each eight clients; (d) administrative and maintenance staff shall not be included in the staff to client ratio when nine or more clients are present; (e) in each program where one-half or more of the clients are medically diagnosed with Alzheimer's Disease or related dementia, a staffing ratio of one staff to six clients is required; and (f) staff trainees shall be supervised at all times while with clients.	☐	☐	☐	☐	☐	
R501-13-4(2) The director shall meet one of the following credentials: (a) licensed nurse; (b) licensed social worker; (c) licensed psychologist; (d) licensed or certified recreational or physical therapist; (e) other licensed professionals in related fields who have demonstrated competence in working with functionally impaired adults; or (f) an individual who has received verifiable training to work with functionally impaired adults and is in consultation on an ongoing basis with a licensed or certified professional with director credentials.	☐	☐	☐	☐	☐	
R501-13-4(3) The director shall obtain & document 10 hours of related training on an annual basis.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews	Walk Through	Multi Risk inc High or Extrem			
	R501-13-4(4) Direct care staff shall be 18 years of age or older and able to demonstrate competency in working with functionally impaired adults.			☐	☐	☐			
	R501-13-4(5) In addition to the training requirements of Subsection R501-1-19(2)(d), direct care staff shall receive: (a) eight hours of initial orientation training specific to the program and client needs; and (b) ten hours of work-related training on an annual basis.			☐	☐	☐			
	Physical Environment			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-13-5(1) The licensee shall ensure that a minimum of 50 square feet of indoor floor space per client is designated specifically for adult daycare during operation hours. Hallways, kitchens, offices, storage, and bathrooms may not be included in the computation.			☐	☐	☐	☐	☐	
	R501-13-5(2) The licensee shall ensure that there is at least one bathroom exclusively for clients during operation hours.			☐	☐	☐	☐	☐	
	R501-13-5(3) The licensee shall maintain indoor air temperature at a minimum of 70 degrees Fahrenheit.			☐	☐	☐	☐	☐	
	R501-13-5(4) The licensee shall ensure that clients receive meals or snacks in accordance with the Child and Adult Care Food Program (CACFP) and the following: (a) there is no more than three hours between snack or meal service; (b) there is sufficient food for second servings; and (c) powdered milk is used for cooking only.			☐	☐	☐	☐	☐	
	R501-13-5(5) The licensee shall provide outdoor recreational space on or off site with compatible recreational equipment available to facilitate activity plans.			☐	☐	☐	☐	☐	
	R501-13-5(6) The licensee shall comply with local building code enforcement for disability accessibility.			☐	☐	☐	☐	☐	
	R501-13-5(7) The licensee shall abate and mitigate hazards on the property, including burning, falling, or drowning hazards, through protective hardware, fences, banisters, railings, grates, natural barriers, or other licensor or local fire authority approved methods.			☐	☐	☐	☐	☐	

Age Group	Male (%)	Female (%)
18-24	~10	~10
25-34	~10	~10
35-44	~10	~10
45-54	~10	~10
55-64	~10	~10
65-74	~10	~10
75+	~10	~10



Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extreme	
Residential Support Programs - Inspection Checklist (Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-3(2) The licensee that offers treatment shall obtain the appropriate categorical department license for that treatment.	☐	☐	☐	☐	☐	
R501-22-3(3) The licensee serving an individual experiencing homelessness in a setting with a contracted service provider shall identify each key decision maker and service provider that is associated with the license application and accountable for compliance with licensing rules within the licensed setting.	☐	☐	☐	☐	☐	
R501-22-3(4) Residential support may not require treatment as a condition of admission.	☐	☐	☐	☐	☐	
R501-22-3(5) The licensee shall provide evidence of ongoing coordination with the local health authorities regarding managing communicable diseases within the licensed setting.	☐	☐	☐	☐	☐	
R501-22-3(6) The licensee shall inform staff regarding: (a) various types of communicable diseases; (b) recognizing signs and symptoms of communicable diseases; and (c) steps to take when a potential disease is identified or an outbreak occurs.	☐	☐	☐	☐	☐	
Staffing	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-4(1) The licensee is not required to provide 24-hour supervision unless that program is an emergency homeless shelter or a domestic violence shelter serving adults.	☐	☐	☐	☐	☐	
R501-22-4(2) The licensee shall establish safe practices that identify each situation requiring medical attention and how the program will meet the client's medical needs.	☐	☐	☐	☐	☐	
R501-22-4(3) The licensee shall conduct eligible background clearance and document required training completion for each student or volunteer. (a) a volunteer or student who provides care without a paid staff present in any emergency homeless shelter or domestic violence shelter shall have direct communication access to designated staff and an eligible background screening before any unsupervised client access. (b) The licensee shall inform each volunteer or student verbally and in writing of program objectives and the scope of service.	☐	☐	☐	☐	☐	
Physical Facility	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-22-5(1) Except as otherwise provided in this section, each licensee has at least one bathroom for every ten clients.	☐	☐	☐	☐	☐	
R501-22-5(2) A domestic violence shelter and emergency homeless shelter licensee may allow family members to share a bathroom. Where a bathroom is shared by more than one family or by children over the age of eight, either the child's parent or program staff ensures that client privacy is maintained.	☐	☐	☐	☐	☐	
R501-22-5(3) Each emergency homeless shelter may exceed the bathroom ratio set forth in Subsection R501-22-5(1) if: (a) each bathroom ratio is approved by either the local authority that determines capacity or the department; (b) each bathroom ratio is specifically designated in adult-only nightly shelter settings; (c) each bathroom is inspected, cleaned, and re-stocked daily and as needed; (d) the licensee ensures individual privacy in bathing and toileting; (e) each individual with disabilities has access to at least one locking bathroom or stall; and (f) the licensee accommodates each parent's needs for changing, toileting, and bathing their children.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
			R501-22-5(4) The licensee shall develop safe practices to identify how to manage emergency overflow when capacity has been reached during dangerous weather conditions.	☐	☐	☐	☐	☐	
			R501-22-5(5) The licensee shall develop safe practices that allow and encourage each client to have clean linen at least weekly.	☐	☐	☐	☐	☐	
			R501-22-5(6) The emergency homeless licensee may have portable beds, cots, or mats to accommodate fluctuating client volume.	☐	☐	☐	☐	☐	
			R501-22-5(7) Except as outlined in Subsection R501-22-5(13), the licensee shall provide clean bedding that is laundered at least weekly or as needed for each client.	☐	☐	☐	☐	☐	
			R501-22-5(8) A family may share bedroom space.	☐	☐	☐	☐	☐	
			R501-22-5(9) The licensee shall comply with the following bedroom standards for domestic violence shelters, family support centers, temporary homeless youth shelters, emergency homeless family shelters, and children's shelters: (a) there is at least 40 sq feet per client in a multiple occupant bedroom, not counting storage space or one crib for children under two years old if the crib does not inhibit access to and from the room; (b) roll away and hide-a-beds are only used when the 40 sq foot space requirement is maintained; and (c) when a bedroom is shared by more than one family, program staff make arrangements to ensure client privacy.	☐	☐	☐	☐	☐	
			R501-22-5(11) The licensee shall comply with the following bedroom standards for emergency homeless shelters, temporary homeless youth shelters, and receiving centers: (a) dormitory style bedrooms meet the square footage and capacity determinations made by the local fire authority; (b) capacity determinations include any staff present in the facility; (c) if the local fire authority does not identify capacity, licensing sq footage requirements apply; and (d) there is a safe practice to identify how to manage overflow when capacity has been reached.	☐	☐	☐	☐	☐	
			R501-22-5(12) Each licensee shall outline safe practices regarding: (a) rules and guidelines for each family or mixed gender sharing the same dormitory space or bedroom, including each individualized bedroom assignment; (b) securing personal belongings; (c) responsibility for each client supervising the client's own children; (d) conflict resolution; (e) nuisance and disruptive behavior; (f) housekeeping responsibilities; (g) daily schedules; (h) prohibited items; and (i) search policy.	☐	☐	☐	☐	☐	
			R501-22-5(14) Each licensee that requires a client to provide the client's own laundry supplies and locate a laundromat for laundering shall have a safe practice to assist each client on a limited basis when the client cannot provide the client's laundry supplies and locate a laundromat.	☐	☐	☐	☐	☐	
			Specialized Services for Programs Serving Client's With Substance Use Disorders	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-6(1) Each licensee may not admit anyone who is currently experiencing convulsions, shock, delirium tremens, unconsciousness, or is in a coma.	☐	☐	☐	☐	☐	
			R501-22-6(2) Each licensee serving clients with substance use disorder provides evidence of ongoing coordination with the local health authorities regarding managing communicable diseases within the licensed setting.	☐	☐	☐	☐	☐	
			R501-22-6(3) The licensee shall screen staff and clients for risk of tuberculosis.	☐	☐	☐	☐	☐	
			R501-22-6(4) A licensed substance abuse treatment program shall complete the National Survey of Substance Abuse Treatment annually.	☐	☐	☐	☐	☐	
			Specialized Services for Domestic Violence Shelters	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
			R501-22-8(1) The licensee shall document that shelter rules, reason for termination and rights to confidentiality are provided to each client, verbally and in writing.	☐	☐	☐	☐	☐	
			R501-22-8(2) Each parent is responsible for supervising their own child while at the shelter. If a parent is required to be away from the shelter or involved in shelter activities without their child, the parent arranges for appropriate child care services.	☐	☐	☐	☐	☐	
			R501-22-8(3) The licensee shall ensure that each domestic violence shelter action plan documents and includes: (a) a review with each victim regarding danger and lethality and the level of the victim's risk of safety assessment; (b) a review of the victim's safety plan with each victim; (c) a review of the procedure for a protective order and a referral for the victim to the appropriate agency or clerk of the court authorized to issue the protective order; and (d) a review of supportive services for each client, including medical care, self-sufficiency, day care, legal assistance, financial assistance, and housing assistance.	☐	☐	☐	☐	☐	
			R501-22-8(4) The licensee shall assist with connecting the client to identified resources.	☐	☐	☐	☐	☐	
			R501-22-8(5) The licensee shall make and document a referral when indicated in the client record for victim treatment, psychiatric consultation, drug and alcohol treatment, or other allied service.	☐	☐	☐	☐	☐	
			R501-22-8(6) The licensee shall ensure that shelter staff completing an action plan are supervised by an experienced and trained domestic violence provider.	☐	☐	☐	☐	☐	
			Specialized Services for Emergency Homeless Shelters	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-22-10(1) An emergency shelter licensee shall prioritize the safety of those needing services and emphasize transitioning into a more permanent housing setting.	☐	☐	☐	☐	☐	
			R501-22-10(2) An emergency homeless shelter licensee shall ensure that no less than two direct care staff are always present and available and maintain a ratio of no fewer than one staff present for every 40 clients during weekday daytime hours.	☐	☐	☐	☐	☐	
			R501-22-10(3) An emergency homeless shelter may operate above staffing ratios during dangerous weather conditions, on weekends, and during sleeping hours if: (a) the program has a documented chain of command for on-call availability; (b) the program has a surveillance camera system; (c) the program has an emergency radio onsite and each staff on-duty are trained regarding how and when it is to be used; or (d) the program identifies and can rely upon other means of back up support in case of emergency.	☐	☐	☐	☐	☐	
			R501-22-10(4) In accordance with Subsections 35A-16-703(1), (2), and (3), an emergency homeless shelter licensee operating in a county where a code blue alert is in effect may increase capacity by 35% when: (a) fire code and building code capacities permit the increase; (b) procedures are implemented for expediting intake; and (c) procedures are implemented for only denying entry if the building capacity is at maximum or the individual poses a risk to the population.	☐	☐	☐	☐	☐	
			R501-22-10(5) Each emergency homeless shelter shall require each adult resident to sign an agreement form at admission that outlines the following: (a) visitors are allowed on premises to assist with housing, food stamps, assessments, religious, social and other client-specific needs; (b) participation in any meetings or groups with these visitors is voluntary; (c) each client signature on the form and voluntary participation in the visitation constitutes the client's invitation to the visitors in the department-licensed setting; and (d) each client must sign the agreement before participation in any voluntary services offered onsite.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem		
	R501-22-10(7) The emergency homeless shelter licensee shall maintain the following information regarding each client or have documented reasons why the information is not obtainable: (a) name; (b) date of birth; (c) race; (d) ethnicity; (e) gender; (f) veteran status; (g) disabling condition; (h) start date; (i) exit date; (j) destination; (k) relationship to head of household; (l) service location; (m) prior living situation; (n) case management log and service plan, where applicable; (o) information that could affect health, safety, or well-being of the client, including medication needs; (p) documentation that is updated at 90-day intervals to include each service and contact; and (q) service plans that emphasize self-sufficiency and identify and refer to applicable sources.			☐	☐	☐	☐	☐	
	R501-22-10(8) The shelter maintains the documentation listed in Subsection (7) for re-opening the client file, if the client returns up to 30 days past the last shelter stay, with the exception of single night stays.			☐	☐	☐	☐	☐	
	Specialized Services for Programs Serving Clients of the Division of Services for People with Disabilities			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-22-11(1) In accordance with the federal Home and Community-Based Services (HCBS) Settings Final Rule, a program serving clients on the HCBS Waiver shall complete and adhere to the residential attestation agreement form and self-assessment survey for each licensed site.			☐	☐	☐	☐	☐	
	R501-22-11(2) The licensee shall maintain current copies of the residential attestation agreement form and self-assessment survey forms in program documentation.			☐	☐	☐	☐	☐	
	R501-22-11(3) In the event of a conflict between this rule and the Settings Final Rule the Settings Final Rule shall prevail.			☐	☐	☐	☐	☐	
	Specialized Services for Receiving Centers			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-22-12(1) Each receiving center may be licensed under multiple license types to assess and triage immediate client needs.			☐	☐	☐	☐	☐	
	R501-22-12(2) A receiving center licensee may offer short-term residential support that is intended to mitigate the initial identified problem, stabilize each client, and return each client to the community as quickly and safely as possible.			☐	☐	☐	☐	☐	
	R501-22-12(3) A receiving center licensee shall outline safe practices in consumer agreements regarding how each population will be separated and maintained and the circumstances when interactions between populations are permitted.			☐	☐	☐	☐	☐	
	R501-22-12(4) A receiving center licensee shall include individualized clinical documentation outlining the ongoing need and anticipated time frame for discharge for each instance that a client's stay lasts longer than 30 days.			☐	☐	☐	☐	☐	
	R501-22-12(5) A receiving center licensee shall ensure that placement in a receiving center is a voluntary alternative that the client may choose instead of a more restrictive placement.			☐	☐	☐	☐	☐	
	R501-22-12(6) A receiving center may not mandate treatment as a condition to residence.			☐	☐	☐	☐	☐	



Staff Files

Policy

Client Files

Interviews

Walk Through

Variable Risk High
or Extreme**Child Placing Foster Checklist****(Revised Jan 2026)****R = Reviewed.****UR = Under Review. Under Review indicates evidence of potential noncompliance.****NA = Not Assessed during this inspection.**

Initial Application, Renewal, and Reapplication Process	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-12-3(1) An individual or legally married couple age 18 or over may apply for licensure or certification to be a foster parent.	☐	☐	☐	☐	☐	
R501-12-3(2) (a) The foster parent applicant for licensure shall apply by completing a form in the online provider portal. (b) The foster parent applicant for certification shall complete the initial application template form found on the OL website and submit that application form to the child-placing foster care agency from which they wish to receive certification.	☐	☐	☐	☐	☐	
R501-12-4(3) OL or agency may consider poverty guidelines when evaluating the dependence of a foster parent on foster payments for their own expenses.	☐	☐	☐	☐	☐	
R501-12-3(4) OL or agency may require supporting documentation of household income and expenses to verify the foster parent or foster parent applicant is financially stable and will not be dependent on foster care reimbursement.	☐	☐	☐	☐	☐	
R501-12-3(5) The foster parent applicant shall provide verification of successful completion of DCFS or agency approved pre-service training within the past 24 months.	☐	☐	☐	☐	☐	
R501-12-3(6) The foster parent applicant shall provide verification of current cardiopulmonary resuscitation (CPR) and first aid training. Accepted training includes; (a) Family and Friends CPR, provided through the American Heart Association; (b) Heartsaver, provided through the American Heart Association; and (c) courses provided through the American Red Cross.	☐	☐	☐	☐	☐	
R501-12-3(7) The foster parent applicant shall authorize a licensed health care professional to complete a physical exam within the previous 12 months and send a signed medical reference report directly to OL or the agency. A medical reference report shall assess the current ability of the individual to be a foster parent.	☐	☐	☐	☐	☐	
R501-12-3(8) (a)(i) If required by OL or the agency to assess mental health status, the foster parent or foster parent applicant shall complete a professional mental health examination. (ii) OL or the agency shall determine the type of professional mental health examination required based on the nature of the presenting concerns. (b) OL or the agency administration shall collaborate with a clinical professional to make the determination of need and type of examination required. (c) The foster parent or foster parent applicant shall authorize the release of examination information to OL or the agency, including a signed report that assesses the ability of the individual to parent a vulnerable foster child full time as a foster parent. (d) The foster parent or foster parent applicant shall pay for any requested medical or mental health examination. (e) OL or the agency may, in the exercise of their professional assessment, deny, suspend, place conditions on, or revoke an application, certification or license if a medical reference report or other examination reveals reasonable concerns regarding an applicant's ability to provide foster care services, or if the required examination is not completed and provided to OL or the agency.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-12-3(9) (a) Upon initial application, or as requested thereafter, the foster parent applicant shall submit the name, mailing address, email address, and phone number of no more than four individuals as referents, who OL or the agency shall contact to provide a reference letter. (b) If there is more than one individual listed on the foster parent application, the referent may address each individual in the same reference. (c) The foster parent applicant shall select referents who are knowledgeable regarding the ability of the applicant to provide a safe environment and to nurture foster children. (d) The foster parent applicant shall select one referent that is a relative of the applicant and three non-relatives. (e) OL or the agency shall only consider the four original referents submitted. (f) A minimum of three out of the four individuals, including one relative and two non-relatives, shall submit reference letters directly to OL or the agency. Except as provided in Subsection R501-12-15(3), OL or the agency shall require a minimum of three reference letters received that are acceptable to OL or the agency. (g) OL or the agency may, in the exercise of their professional assessment, deny an application if a reference reveals reasonable concerns regarding an applicant's ability to provide foster care services.	☐	☐	☐	☐	☐	
			R501-12-3(10) (a) The foster parent applicant and each person 12 years of age or older residing in the home shall submit a background check application as part of the initial application. (b) A background check application is also required for any new individual over the age of 12 who moves into the home. (c) OL or the agency may not license or certify a foster parent unless the background check of each person 12 years of age or older that resides in the home is deemed eligible by the OBP in compliance with Section 26B-2-120 and Rule R501-14. (d) The foster parent may not permit any person without an eligible background check to have unsupervised direct access to a foster child unless: (i) the person is a provider of incidental care; or (ii) the person's access is driven by child-centered normalcy needs that are guided by reasonable and prudent parenting as described in Section 80-2-308 and is not a foster parent-centered delegation of parental responsibility. (e) The foster parent shall immediately notify OBP if any person in the home is charged with or under investigation for any criminal offense, or allegation of abuse, neglect, or exploitation of any child or vulnerable adult. (f) A pending Child Protective Services, Adult Protective Services or law enforcement investigation of any person in the home may result in a conditional or suspended license until resolved to the satisfaction of OL. (g) In accordance with Section 80-2-1001, OBP shall review and evaluate information from the DCFS Management Information System for licensing and monitoring individuals who reside in the foster home. When, in the professional assessment of OL, a supported or substantiated finding against any individual who resides in the foster home may pose a risk of harm to a foster child, the OL may issue a safety plan, place parameters on the license or issue a notice of agency action to the foster parent or agency.	☐	☐	☐	☐	☐	
			R501-12-3(11) After completing the required background checks, OL or the agency must conduct a home study before any placement is made in the home. (a) If the home study is being conducted for adoptive purposes, an adoption service provider, as defined in Section 78B-6-103, shall complete the home study. (b) If the home study is not being conducted for adoptive purposes, the home study may be conducted by an individual who: (i) is an adoption service provider; or (ii) is employed or contracted to conduct a home study for an agency licensed by OL and who has participated in the recruiting, hiring, training, and supervising of proctor foster homes for at least a year. (c) OL or the agency must complete the home study document on the OL approved home study document template, that is found on the OL website, before an applicant is licensed or certified to take foster placements.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
	R501-12-3(12) (a) The foster parent who wishes to remain licensed or certified to provide foster care services must submit a renewal application before the license or certification expiration. (b) Each applicant requesting license or certification renewal shall complete the renewal application form located on the OL website. (c) OL or the agency may require supporting documentation of household income and expenses to verify the foster parent will not be dependent on foster care reimbursement. (d) OL or the agency shall update the home study in writing annually after a home visit and safety inspection as a means to assess the family's experience over the past year as a foster family to include: (i) any changes to required home study information; (ii) interviews with any members of the home; and (iii) references or other requested information needed to update the home study.			☐	☐	☐	☐	☐	
	R501-12-3(13) A previously licensed or certified foster parent is subject to the same requirements as an initial application, with the following additional requirements: (a) each applicant shall disclose previous foster care licenses and certifications, including those outside of Utah; (b) each previously licensed applicant shall request a written reference from the custodial agency where they last held a foster care license to be sent directly to OL or the agency; (c) each previously certified applicant shall request a written reference letter from the last agency where they were certified, and each agency they have been certified by within the past three years, to be sent directly to the office or agency; and (d) each applicant shall sign a release of information for any agency where the foster parent previously provided certified or licensed foster care.			☐	☐	☐	☐	☐	
	R501-12-3(14)(a) Reapplication of previously licensed or certified applicants may utilize an update of the previous home study if the home study was created by the same agency currently relicensing or recertifying the home. (b) OL or the agency may add an update to the existing home study from another agency if the agency provides it directly and it is completed on an OL approved home study addendum template found on the OL website that addresses and updates general foster parent requirements. The update may reference applicable portions of the original study as an attachment. (c) OL or the agency may request new reference letters or additional information if needed to update the home study. (d) The reference letter requirement is waived if 12 months or less have passed since the lapse of any license or certification. (d) A personal health statement is still required, but a physician's statement is waived if 12 months or less have passed since lapse of any license or certification. (f) Initial training requirements are waived, as long as there is not a change of the licensing or certifying agency if 24 months or less have passed since lapse of any license or certification. (g) A change in agency shall require new initial training.			☐	☐	☐	☐	☐	
	R501-12-3(15)(a) OL or the agency shall base the decision to approve or deny the applicant to provide foster services on the facts, health and safety factors, and the professional assessment of OL or the agency. (b) OL or the agency may not deny a person a foster care license or certification on a basis that violates any applicable federal or state anti-discrimination law. (c) The approval of a license or certification is not a guarantee that a foster child will be placed or retained in the foster parent's home. (d) Except for a kinship parent or a short-term relief care provider, a foster parent may not be licensed or certified to provide foster or respite care services in the same home where they provide child care for five or more children or another licensed or certified department program. (e) To promote health and safety, OL or the agency may issue a license or certification that includes additional restrictions unique to the circumstances of the license. (f) If a license or certification is denied, an applicant may not reapply for a minimum of 90 days from the date of denial.			☐	☐	☐	☐	☐	
	Three-Year License			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extreme		
	R501-12-4(1) (a) This section supersedes Subsections R380-600-3(16), (18), and (19) in accordance with Subsection 26B-2-105(5)(d). (b) This section does not apply to a child-placing foster care agency or foster homes certified under a child-placing foster care license.			☐	☐	☐	☐		
	R501-12-4(2) (a) A foster parent who has remained continually licensed by OL for any two or more consecutive years, with no penalties or notations of noncompliance during that period, shall receive a renewal license that expires three years following the renewal license start date. (b) A foster parent with a three-year license remains subject to: (i) ongoing background checks and approvals and denials, as outlined in Section 26B-2-120; and (ii) the requirement to submit annual update information to ensure OL has accurate information regarding the home and family members. (c) In accordance with Subsection 26B-2-107(3), an announced or unannounced on-site inspection may not be conducted by OL for any foster parent with a three-year license, unless: (i) the office is made aware of any safety concerns in the home requiring an on-site visit from OL; (ii) the licensee has not had any placements for more than 12 consecutive months and seeks to take a new placement; or (iii) as necessary to monitor a material change.			☐	☐	☐	☐		
	R501-12-4(3) If the foster parent with a three-year license receives a new notation of noncompliance, OL shall revert to the annual license requirements that maintains the same expiration day and month for a one-year license.			☐	☐	☐	☐		
	R501-12-4(4) The foster parent may not allow any person without an OBP eligible background check to have unsupervised direct access to a foster child unless: (i) the person is a provider of incidental care; (ii) the person is identified in the foster child's department record as an individual for preserving connection; (iii) the person is a resident living in the home of a short-term relief care provider that maintains supervision of the resident in the presence of a foster child; or (iv) the person's access is driven by child-centered normalcy needs that are guided by reasonable and prudent parenting as described in Section 80-2-308 and is not a foster parent delegation of parental responsibility.			☐	☐	☐	☐		
	R501-12-4(5) The foster parent who selects short-term care for a foster child as a delegation of parental responsibility and not for respite care, incidental care, preserving connection or short-term relief care must ensure the caregiver has an eligible OBP background check and the child's caseworker approves the caregiver in-writing before allowing unsupervised direct access to the foster child.			☐	☐	☐	☐		
	Short-Term Relief Care			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-12-5(1) This section does not apply to a child-placing foster care agency or foster homes certified under their license, unless the agency maintains a department contract for placement of DCFS foster children.			☐	☐	☐	☐	☐	
	R501-12-5(2) The foster parent may use a short-term relief provider if: (a) DCFS approves the immediate family member or relative of the foster parent in-writing for caring for the foster child for less than six consecutive nights; (b) the immediate family member or relative of the foster parent has an eligible OBP background check; and (c) the immediate family member or relative of the foster parent, is not a relative of the foster child, unless approved by DCFS for a licensed kinship foster parent or an individual identified for preserving connection.			☐	☐	☐	☐	☐	
	Foster Parent Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
			R501-12-6(1) The foster parent shall: (a) be in good health and emotionally stable; (b) be able to provide for the physical, social, mental health, and emotional needs of the foster child; (c) be a responsible person who is 18 years of age or older; (d) be able to communicate with the foster child, the department, health care providers and other service providers; (e) have at least one functionally literate applicant in the home able to read medication labels and other critical information; (f) provide documentation of legal residential status in accordance with the Code of Federal Regulations 8 U.S.C. Sec. 1642; (g) have the ability to help the foster child thrive; (h) demonstrate financial responsibility without dependence on the foster care reimbursement for their own expenses, outside of those expenses directly associated with providing foster care services; (i) provide updated medical, social, financial, or other family information when requested by OL or the agency; (j) follow federal, state and local laws and ordinances; (k) demonstrate safe parenting practices by not engaging in conduct that poses a substantial risk of harm to any person or that is illegal or grounds for denying a license under Section 26B-2-703; and (l) cooperate with the custodial agency goals and requirements regarding permanency, reunification, education, health care, and any other services required by the child's treatment plan or involved professionals.	☐	☐	☐	☐	☐	
			R501-12-6(2)(a) A department employee may not be licensed or certified as a foster parent for children in the custody of their respective custodial agency, unless they qualify as a relative to the child in accordance with Subsection 80-2a-101(5). (b) An employee may provide foster services for children in the custody of a different division only with the prior written approval of both divisions' directors in accordance with DHHS conflict of interest policy.	☐	☐	☐	☐	☐	
			R501-12-6(3) The foster parent shall cooperate with the department, the agency, if applicable, courts, and law enforcement officials.	☐	☐	☐	☐	☐	
			R501-12-6(4) The foster parent shall read, acknowledge, and comply with the department code of conduct, as outlined in Rule R380-80. (a) The foster parent may not abuse, neglect, or maltreat a foster child through any act or omission. (b) The foster parent may not encourage or fail to deter the acts or omissions of another that abuse, neglect, or maltreat a foster child.	☐	☐	☐	☐	☐	
			R501-12-6(5) No more than two children under the age of two, including children who are members of the household and foster children, shall reside in a foster home.	☐	☐	☐	☐	☐	
			R501-12-6(6) No more than two non-ambulatory children, including children who are members of the household and foster children, shall reside in a foster home.	☐	☐	☐	☐	☐	
			R501-12-6(7) A foster parent may not have more than the numerical limit of foster children in a foster home in accordance with Section 26B-2-128.	☐	☐	☐	☐	☐	
			R501-12-6(8) The foster parent shall utilize reasonable and prudent judgment in selecting an incidental caregiver for a foster child and incidental care may only be utilized by a department licensed foster parent, not a foster parent certified by a licensed child-placing foster care agency.	☐	☐	☐	☐	☐	
			R501-12-6(9)(a) The foster parent may provide respite care in their home as long as they remain in compliance with licensing rules in regard to each child placed for foster and respite care. (b) The foster parent may provide respite care when the additional foster children exceed their licensed capacity only if: (i) there are no licensing sanctions currently imposed, including corrective action plans or conditional licenses; and (ii) the total number of foster and respite children in a home does not exceed six unless all except one or two of the children are part of a single sibling group.	☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
			R501-12-6(10)(a) Respite care, child care, incidental care, emergency care or other temporary care, for a foster child may be allowed in a licensed or unlicensed setting, with or without background checks if the child's department client record identifies, by name, the individual's role in preserving connection. (b) The department custodial agency shall set parameters and oversee the safety aspects of a setting identified for preserving connection. (c) Unlicensed kinship respite caregivers, identified by the department, remain subject to licensure background check requirements and a custodial agency walk-through of the home for safety approval. (d) A licensed child-placing foster care agency, except a department custodial agency, may not utilize an unlicensed caregiver for care of any foster child, unless specifically outlined in the custodial agency client record and authorized by the child's caseworker.	☐	☐	☐	☐	☐	
			R501-12-6(11) The foster parent or the agency shall report the following major changes or events to OL or the agency within one business day: (a) the death or serious illness of a member of the foster parent's household; (b) change in marital status; (c) loss of employment; (d) change in household composition, including the birth or adoption of a child, or the addition of household members, or tenants; (e) allegations of abuse or neglect of any child or vulnerable adult against any member of the foster parent's household; (f) any material change; and (g) anything defined as a "critical incident" in Rule R380-600.	☐	☐	☐	☐	☐	
			R501-12-6(12) OL or the agency shall evaluate major changes to determine necessary actions that may include an update to the home study, implementation of a safety plan, amendments to the license certification, request for new references or examinations, or agency action.	☐	☐	☐	☐	☐	
			R501-12-6(13)(a) The foster parent shall report any potential change in address in advance to OL of the agency. (b) A license or certification is site-specific. (c) An adjoining dwelling with a separate address that is not accessible from the foster home is not considered part of the foster home site. (d) A foster child may not be moved into a home that is not licensed or certified to provide foster care except as allowed in Subsection R380-600-4(4)(d) for relocation of a license. (e) The foster parent shall reside at the license location when a foster child is placed in their care. (f) In the event of a separation or divorce: (i) OL or agency shall remove the foster parent who no longer resides at the licensed or certified location from the license or certificate and that foster parent shall apply for and meet licensing or certification requirements in the new residence to become licensed or certified at the new location; and (ii) The foster parent remaining in the home shall demonstrate the ability to continue to meet the financial and other foster care licensure requirements and the office or agency shall complete an update to the home study.	☐	☐	☐	☐	☐	
			R501-12-6(14) The foster parent shall offer nutritious, balanced meals that meet each foster child's individual needs.	☐	☐	☐	☐	☐	
			R501-12-6(15)(a) The foster parent with a foster placement in the home shall continually comply with Rule R501-12. (b) The foster parent with no placements in the home must demonstrate ability to comply upon request and ensure compliance before any new placement is made.	☐	☐	☐	☐	☐	
			Physical Aspects of Home	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem
	R501-12-7(1) The provider shall ensure the following regarding the foster home environment: (a) each indoor and outdoor area of the home is maintained to ensure a safe physical environment; (b) the home is free from health and fire hazards; (c) the home has a working smoke detector and a working carbon monoxide detector on each separated level and at least one of each shall be close to sleeping areas; (d) the home has at least one fire extinguisher meeting the rating requirements of 2A:10BC, that is fully charged and readily accessible to the main living area; (e) the home has at least one toilet, sink and tub or shower; and (f) each bathroom has a lock sufficient to maintain the privacy of the occupant.			☐	☐	☐	☐
	R501-12-7(2) The provider shall ensure each bedroom spaces complies with the following: (a) children of the opposite genders do not share a bedroom unless: (i) each child sharing the room is under two years of age; (ii) the department client record identifies gender-specific rationale; or (iii) there is written caseworker approval for the bedroom assignment; (b) the foster parent's bedroom is only shared with a foster child under the age of two years and the foster parent may not bed-share with a foster child; (c) the foster parent's bedroom is not considered in calculating the allowable bedroom space for a foster child; (d) a foster child may not share a bedroom with other adults in the home; (e) a foster child has an individual bed or crib, mattress, and linens that meet the child's needs; (f) a weighted blanket is only used for a foster child if therapeutically recommended in writing or approved in writing by the child's caseworker; (g) there is a minimum of 40 sq ft per child, excluding adjoining bathrooms and storage space; (h) no more than four children are housed in a single bedroom that houses at least one foster child; (i) a bedroom used for a foster child is comparable to other similarly utilized bedrooms in the home, including access, location, space, finishings, and furnishings; (j) a bedroom used by a foster child on the ground floor shall have a minimum of one screened window that opens that may be used to evacuate the room if there is a fire; (k) a bedroom used by a foster child that is not on the ground floor shall have a source of natural light and a minimum of two exits, at least one of which shall exit directly to outside the home that may be used to evacuate the room if there is a fire; and (l) closet or dresser space is provided within the bedroom for the foster child's personal possessions and for a reasonable degree of privacy.			☐	☐	☐	☐
	R501-12-7(3) The provider shall ensure: (a) there is space or access to common areas for recreational activities; (b) there is adequate lighting, ventilation and the home is maintained at a reasonable temperature when occupied by a foster child in consideration of the age and needs of the foster child and other residents; (c) there is a properly operating kitchen with working refrigerator, cooking appliances, adequate supply of safe drinking water and functional indoor plumbing; (d) each hazard on the property is abated and mitigated through the use of protective hardware, fencing, banisters, railings, grate, natural barrier, or other licensor approved method to secure any: (i) fall hazard of 3 feet or greater including a steep grade, cliff, open pit, window well, stairwell, elevated porch, retaining wall; (ii) drowning hazard including a swimming pool, hot tub, water feature, pond and stream; (iii) burn hazard including fireplaces, candles, radiators, water temperature; (iv) unstable heavy item including a television, bookshelf; (v) high voltage booster; and (vi) dangerous traffic condition; (e) the home and its contents are maintained in a clean and safe conditionand food, clothing, supplies, furniture, and equipment are of sufficient quantity, variety, and quality to meet the foster child's needs; (f) the home is free from rodent and insect infestation; (g) there are at least two exits adequately sized for emergency personnel on each accessible floor of the home and a multiple-level home has a functional, automatic fire suppression system, or an escape ladder, stairway, or other exterior egress to ground level accessible from each of the upper levels; (h) the foster parent uses child safety devices appropriate to the needs of the foster child, including safety gates, and electrical outlet covers; (i) the home address is clearly visible and location is accessible; (j) the water and sewage disposal system, other than a public system, is approved by the appropriate authorities; (k) there is trash and recycling disposal;			☐	☐	☐	☐

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
	(k) there is trash and recycling disposal; (l) any swimming pool is secured to prevent unsupervised access and complies with applicable community ordinances; and (m) any hot tub or spa has a locked cover.								
	Safety			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-12-8(1) The foster parent and their guests may not smoke any substance in the foster home or vehicle when a foster child is present or residing in the home and shall ensure that smoking materials are inaccessible to foster children.			☐	☐	☐	☐	☐	
	R501-12-8(2) The foster parent shall provide training to children regarding response to fire warnings and other instructions for life safety upon the initial placement of a foster child and annually thereafter. This includes an evacuation plan that also anticipates the evacuation of a foster child who is non-ambulatory or who has a disability.			☐	☐	☐	☐	☐	
	R501-12-8(3) (a) The foster parent shall have a phone that can make outgoing calls and is recognizable to the 911 system on-site during any time that a foster child is present. (b) The foster parent shall post telephone numbers for emergency assistance, poison control, the emergency evacuation plan and the address of the home in a central location accessible to the foster child.			☐	☐	☐	☐	☐	
	R501-12-8(4) The foster parent shall have a fully supplied first aid kit as recommended by the American Red Cross.			☐	☐	☐	☐	☐	
	R501-12-8(5) The foster parent shall inform OL or the agency if they have or use a firearm or other weapon.			☐	☐	☐	☐	☐	
	R501-12-8(6) The foster parent shall ensure that any ammunition, firearm, or other weapon is inaccessible to foster a child.			☐	☐	☐	☐	☐	
	R501-12-8(7) The foster parent may not provide a weapon to a foster child or permit a foster child to have a weapon except as outlined in Sections 76-10-509.4 through 76-10-509.7. (a) The foster parent does not have the authority of a parent or guardian to provide a dangerous weapon to a minor under Section 76-10-509. (b) The foster parent shall ensure the following regarding any firearm in the foster home: (i) a firearm is only stored together with ammunition in a locked container commercially manufactured for the secure storage of a firearm; (ii) a firearm not stored in a locked container commercially manufactured for the secure storage of firearms is unloaded and securely locked and ammunition for the firearm is kept securely locked in a separate location; (iii) the locked storage for a firearm and ammunition is not accessible through the same key or combination; (iv) the key and combination used to open locked storage for a firearm and ammunition is not accessible to a foster child; and (v) a firearm is stored in a display case only if it is unloaded and made inoperable through the effective use of a trigger lock, bolts removed, or another disabling method.			☐	☐	☐	☐	☐	
	R501-12-8(9) The foster parent who has alcoholic beverages in their home may not consume in excess and shall ensure that the beverages are closely monitored and inaccessible to foster children.			☐	☐	☐	☐	☐	

	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extreme		
	R501-12-8(10) The foster parent shall ensure hazardous materials remain locked when not in active use, and closely monitored while in active use, and shall ensure compliance with the following: (a) hazardous materials are stored in the manufacturer's original packaging together with the manufacturer's directions and warnings, or a container that complies with the manufacturer's directions and warnings and is clearly labeled with the contents, manufacturer's directions and warnings; (b) flammable substances, including gasoline and kerosene, are locked in a ventilated storage area separate from living areas, this requirement does not include substances contained within the storage tanks of equipment, including automobiles, lawnmowers, ATVs, boats and snow blowers; and (c) general, common use, household items are stored responsibly in consideration of the age, behavior, history, and cognitive and physical ability of each foster child in the home, and in consultation with the caseworker and child and family team regarding individual restrictions.			☐	☐	☐	☐	☐	
	R501-12-8(11) The foster parent shall comply with local laws and ordinances regarding the care and number of animals on their property.			☐	☐	☐	☐	☐	
	R501-12-8(12) The foster parent shall ensure that the foster child has the safety equipment, supervision, and training necessary for the foster child to safely participate in an activity that has an inherent risk of bodily harm, injury, or death. (a) These activities include participation in rock climbing, swimming, hunting, target practice, camping, hiking, use of recreational vehicles, and sports. (b) The foster parent shall take every precaution in allowing a foster child to participate in the respective activity as safely as possible to include: (i) wearing Department of Transportation or Snell-approved helmets when riding off-highway vehicles (OHVs); (ii) completing OHV education; (iii) completing personal watercraft or boating education; (iv) wearing Coast Guard-approved lifejackets; and (v) completing hunter's education. (c) The foster parent shall follow any applicable statute pertaining to minors operating OHVs, personal watercraft, boats, and firearms.			☐	☐	☐	☐	☐	
	R501-12-8(13) The foster parent shall comply with any written safety plan or license parameter required by OL or the agency, that establishes additional safety requirements to protect the foster child from hazardous conditions on the foster parent's property. A safety plan may not waive any applicable requirement of Rule R501-12.			☐	☐	☐	☐	☐	
	R501-12-8(14) The foster parent must provide verification of compliance with the department recommended immunization schedules for each individual residing in the home who is not a foster child. The foster parent may only be licensed or certified for placements of foster children who are over the age of 2 months and are currently immunized if vaccination compliance of each resident in the home cannot be verified. (a) The foster parent shall disclose if any individual residing in the home is not in compliance with the department recommended immunization schedules to the child-placing foster care agency before accepting a placement. (b) A newborn infant shall reach the required age and receive their first dose of required vaccinations to be considered appropriately immunized for their age.			☐	☐	☐	☐	☐	
	R501-12-8(15) The foster parent may not accept the placement of a foster child into their home outside any license conditions or parameters.			☐	☐	☐	☐	☐	
Emergency Plans				R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
			R501-12-9(1) The foster parent shall have a written plan of action for emergencies and disaster to include: (a) evacuation with a pre-arranged site for relocation; (b) transportation and relocation of foster children when necessary; (c) supervision of foster children after evacuation or relocation; and (d) notification of appropriate authorities.	☐	☐	☐	☐	☐	
			R501-12-9(2) The foster parent or agency shall immediately report any serious illness, injury, or death of a foster child to the appropriate custodial agency and OL.	☐	☐	☐	☐	☐	
			Infectious Disease	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-12-10 In the event of an infectious or communicable disease outbreak, the foster parent shall follow specific instructions given by the local health department.	☐	☐	☐	☐	☐	
			Medication and Medical Emergencies	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-12-11(1) The foster parent shall ensure the following regarding medications and medical emergencies: (a) prescribed medication is administered according to the written directions of the foster child's health provider; (b) the foster child consumes the medication; (c) any severe or unexpected side effects or reactions are immediately reported to the foster child's health provider; (d) medication is only given to the foster child for whom it was prescribed; (e) medication is not discontinued without the approval of the foster child's health provider; (f) non-prescription medication is administered by the foster parent according to manufacturer's instructions unless otherwise directed by the foster child's health provider; (g) medication is not administered or carried by the foster child, unless approved in writing by the foster child's health provider; (h) medication is not used for behavior management or restraint unless prescribed in writing by the foster child's health provider and after notification to the division or caseworker; (i) medication remains locked at times it is not in immediate, active use; (j) medication in active use is not left unattended and the foster parent may not abuse or misuse prescription or non-prescription drugs or medications; (k) the foster parent may carry a necessary dose of medication for active use, if a foster child requires immediate access to their medication for asthma, allergies, diabetes or other condition requiring urgent administration of the medication; (l) medication remains in the original pharmacy or manufacturer's packaging; (m) the foster parent may not repack medication or divide doses into alternative containers; (n) the foster parent partners with the pharmacy regarding any needed divisions of medication; (o) the foster parent promptly takes a foster child who has a medical emergency, who is sick, or who is injured, for an assessment by a medical practitioner; and (p) the foster parent complies with the treatment orders of the foster child's health provider.	☐	☐	☐	☐	☐	
			R501-12-11(2) The foster parent shall transfer any unused medication to the caseworker or agency when a foster child is no longer placed in the foster parent's home.	☐	☐	☐	☐	☐	
			R501-12-11(3) The foster parent shall have a written plan for medical emergencies, including arrangements for medical transportation, treatment, and care.	☐	☐	☐	☐	☐	
			Transportation	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
			R501-12-12(1) The provider shall ensure a driver of a vehicle carrying a foster child has a valid, current driver's license, valid, current vehicle insurance, and complies with traffic law.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extreme
	R501-12-12(2) The provider shall ensure transport of a foster child is provided in an enclosed, registered vehicle that has functional seatbelts. (a) The provider shall ensure foster children properly utilize seatbelts and other safety equipment, including age and size appropriate car or booster seats. (b) Recreational vehicles, including motorcycles, may not be used for transportation.			☐	☐	☐	☐
	R501-12-12(3) The provider shall ensure emergency contact information, including caseworker and agency information is accessible to any passenger in each vehicle used to transport foster children.			☐	☐	☐	☐
	R501-12-12(4) The foster parent shall equip each vehicle with a first aid kit.			☐	☐	☐	☐
	Behavior Management			R	UR	NA	Corrected During Inspection Technical Assistance Given
	R501-12-13(1) The foster parent shall provide supervision appropriate to the age and needs of each foster child.			☐	☐	☐	☐
	R501-12-13(2) The foster parent may not use, nor permit the use of corporal punishment including: (a) physical, mechanical, or chemical restraint; (b) physical force; (c) infliction of bodily harm or pain; (d) deprivation of meals, rest, or visits with family; and (e) humiliating or frightening methods to discipline, coerce, punish, or retaliate against a foster child.			☐	☐	☐	☐
	R501-12-13(3) The foster parent shall only use behavior management techniques appropriate for the foster child's age, behavior, needs, developmental level, and past experiences.			☐	☐	☐	☐
	R501-12-13(4) The foster parent shall use the least restrictive method of behavior management available to control a situation.			☐	☐	☐	☐
	R501-12-13(5) The foster parent shall only use behavior management techniques that are positive, consistent, and that promote self-control, self-esteem, and independence.			☐	☐	☐	☐
	R501-12-13(6) The foster parent may not use physical work assignments or activities that inflict pain as behavior management technique. A physical work assignment or activity that results in minor sore muscles does not violate this subsection.			☐	☐	☐	☐
	R501-12-13(7) The foster parent may not abuse, threaten, ridicule, intimidate, or degrade a foster child.			☐	☐	☐	☐
	R501-12-13(8) The foster parent may not deny a child medical care, nutrition, hydration, clothing, bedding, sleep, or toilet and bathing facilities.			☐	☐	☐	☐
	R501-12-13(9) (a) Physical restraint of a foster child in the custody of the department is prohibited, unless expressly indicated in the child's department client record and the foster parent is appropriately trained and authorized by the department for its use. (b) Physical restraint of a foster child who is not in department division custody may only be performed by an individual with verified, documented training in accordance with the nonviolent strategies of a state, regional, or nationally recognized behavior management program. (c) Gently hugging, holding, or guiding a foster child is not considered a restraint. (d) The foster parent shall only perform self-defense as long as it is without aggression, retaliation, or unnecessary force and is reported to the caseworker, the agency if applicable, and OL within one business day.			☐	☐	☐	☐
	Foster Child's Rights in Foster Care			R	UR	NA	Corrected During Inspection Technical Assistance Given

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extreme		
	R501-12-14 The foster parent may not violate a foster child's right to: (1) eat nutritious meals with the family; (2) eat the same food as the family, except when the foster child is provided with alternative food ordered by the foster child's physician; (3) participate in family and school activities; (4) privacy, including maintaining the confidentiality of information about the foster child and not retaining copies of the foster child's records once the foster child is no longer placed in the home; (5) be informed of the foster child's responsibilities, including household tasks, privileges, and rules of conduct; (6) be protected from discrimination; (7) be protected from harm or acts of violence, including protection from physical, verbal, sexual, or emotional abuse, neglect, maltreatment, exploitation including source funding, or inhumane treatment; (8) be treated with courtesy and dignity, including reasonable personal privacy and self-expression and not provided temporary items including garbage bags for collecting or transporting belongings; (9) communicate with and visit the foster child's family, attorney, physician, and clergy, except as restricted by court order; (10) have clean clothes and personal hygiene needs met; (11) participate in their own cultural traditions; (12) receive prompt medical care when sick or injured; and (13) be free from media content that is likely harmful considering the foster child's age, behavior, needs, developmental level, and past experiences.			☐	☐	☐	☐		
	Additional Child Placing Agency Considerations			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-12-15(1) The agency shall comply with: (a) this rule; (b) Rule R380-600; (c) Rule R501-14; and (d) Section 80-2-9 regarding the Interstate Compact for the Placement of Children (ICPC) for agencies taking placements from out-of-state.			☐	☐	☐	☐	☐	
	R501-12-15(2) The agency shall ensure certified foster parent compliance with this rule.			☐	☐	☐	☐	☐	
	R501-12-15(3) The agency shall recruit, train, certify, and supervise foster parents.			☐	☐	☐	☐	☐	
	R501-12-15(4) The agency may not certify a home that is licensed or certified or applying to be licensed or certified with any other agency.			☐	☐	☐	☐	☐	
	R501-12-15(5) The agency may not certify agency owners, directors, managers, and members of the governing body to provide foster care services for foster children placed with or by any child placing foster agency.			☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
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			R501-12-15(7) The agency shall: (a) maintain documentation of the initial and annual home studies and any updates and provide to OL upon request; (b) have a written agreement with the foster parents that includes: (i) the expectations & responsibilities of the agency, staff, foster parents and limitations of authority; (ii) the services to be provided to and by the foster parent; (iii) the requirements to provide medical, remedial, treatment, and other specialized services to a foster child; (iv) the financial arrangements for a foster child placed in the home; (v) the authority foster parents can and cannot exercise over a foster child placed in the home; and (vi) actions that require staff or department authorizations; (c) monitor and keep detailed documentation regarding foster parents' compliance with R501-12; (d) document each announced and unannounced visit to the foster home, including an initial safety inspection and a minimum of one unannounced safety inspection annually; (e) document each safety inspection completed by the agency on OL-provided home inspection checklist, or a similar form that contains the OL-provided form contents; (f) coordinate with OL when checklist items are not compliant or other noncompliance is noted to determine how to proceed; (g) document actions on foster parent certifications in the foster parent file to include any request for remediation with assigned time frames, request corrective action plan from the foster parent, or any action to suspend certification or revoke certification; (h) escalate the level of agency action taken toward foster parent certification when there are multiple notations of noncompliance with the same rule; (i) maintain completed checklists & compliance monitoring documentation in each foster parent file; (j) investigate complaints and alleged violations of Rules R501-12, R501-14 and R501-1. The agency shall provide documentation to OL of any investigations into complaints and alleged violations of licensing rules; (k) provide written notification to each foster parent that informs the foster parent of the rights and responsibilities assumed by the foster parent who signs as the responsible adult for a foster child to receive a driver license, as described in Section 53-3-21 and maintain documentation in the foster parent's file, signed and dated by the foster parent, acknowledging receipt of a copy of this written notification; (l) have and comply with written policies and procedures regarding the denial, suspension, and revocation of a foster parent's certification to provide foster care services, that includes written notification of the foster parent's appeal process; (m) provide documentation and immediate notification to OL and the custodial agency of any denial, suspension, revocation or other agency-initiated termination of a foster parent's certification; (n) not grant or permit any variance to Rule R501-12 or any other regulation without the prior written consent of the director or director's designee of OL; (o) provide ongoing supervision of certified foster parents to ensure the quality of care they provide; and (p) participate with each foster child's legal guardian and the foster parent to obtain, coordinate, and supervise care and services necessary to meet the needs of each foster child in their care.	☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme	
	R501-12-15(8) The agency may not take placement of a foster child whose needs exceed the scope or ability of the program to reasonably manage, and the agency shall: (a) outline in policies and procedures the behaviors and presenting issues would be reason for discharge or exclusion from the program; (b) document how the placement of the foster child is appropriate and commensurate with presenting needs and the services that are available to address the child's needs; (c) conduct or coordinate monthly visits to the foster child in the placement or school; (d) maintain responsibility for the child's behavior in the program, school and community; (e) maintain responsibility for transitioning a foster child or 18 to 21 year old into safe and appropriate placement upon discharge from the program or in accordance with ICPC disruption plan if the child is from out-of-state; and (f) ensure in policy and safe practices that sending a child to a homeless shelter, refusal to pick up from detention, or offering one-way plane or bus tickets are not appropriate or responsible program transition actions, unless supported by therapeutic or parental recommendation.			☐	☐	☐	☐	☐	
	R501-12-15(9) The agency shall: (a) provide and receive approval from the school district of certified homes with a youth education coordinating form in compliance with the requirements of Section 26B-2-116; (b) provide accurate and truthful written references for any previously certified home that requests such reference to work with foster children in another licensed agency; (c) maintain copies of completed foster parent initial and renewal applications and accompanying documents, home study document and any subsequent updates, and any other foster parent documentation in a format easily accessible OL review; (d) follow department contract requirements and request guidance from the Division of Continuous Quality and Improvement and OL in the event of conflicting requirements; and (e) if serving individuals involved with the Division of Services for People with Disabilities, ensure compliance with the Home and Community Based Services Settings Final Regulation as identified in 42 CFR 430 and 431 (2024) that shall prevail in the event of a conflict with any rule under Title R501.			☐	☐	☐	☐	☐	
	Additional DCFS Kinship and Specified Home Licensure Considerations			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-12-16(3) Only OL, an agency contracted by the department or an approved DCFS kinship home study specialist may conduct a kinship specific home study.			☐	☐	☐	☐	☐	
	R501-12-16(4) A minimum of two acceptable reference letters sent in accordance with Subsection R501-12-4(5) are required for a kinship or a specific applicant.			☐	☐	☐	☐	☐	
	R501-12-16(5) The foster parent may not accept a placement until the home study, safety inspection, and background checks, are successfully completed, unless the placement is made on an emergency basis as authorized by Section 80-2a-301.			☐	☐	☐	☐	☐	
	R501-12-16(6) OL shall grant a kinship specific probationary license or initial license upon receipt and approval of a completed kinship or specific packet submitted by DCFS. (a) A kinship or specific probationary license expires no later than the last day of the fifth month from the issue date if compliance is not met before that time. (b) The probationary licensee may receive an initial license at any time within the probationary 5 months when compliance with probationary terms is met. (c) A probationary licensee whose probationary terms are not met before the expiration of that license may either expire or extend with OL-approved documented justification.			☐	☐	☐	☐	☐	
	R501-12-16(7) A kinship specific home licensee may not accept placement of any foster child other than the foster child, or relatives to that foster child, as designated on the license or certificate.			☐	☐	☐	☐	☐	

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Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extreme
R501-12-16(8) If a kinship specific licensee desires to provide general foster care services, they shall complete the following: (a) submit written approval from their DCFS kinship support worker to become a general foster parent to OL and the DCFS contracted recruitment and training agency to initiate required training; (b) close the kinship or specific license and submit to the requirements of an initial foster care license to include: (i) complete initial foster care application; and (ii) complete foster care pre-service training series with the exception of session one, if completed within the last 2 years; (c) submit to a home study update interview with their licensor to change child-specific content to general foster parent requirements; and (d) provide any new reference letters as requested.			☐	☐	☐	☐	☐
R501-12-16(9) If DCFS does not support a license change, no further licensing action will be taken, unless the issue is disputed and overturned by the OL director.			☐	☐	☐	☐	☐
R501-12-16(10) In accordance with Section 26B-2-130 and the Indian Child Welfare Act, 25 U.S.C. Sec. 1901 through 1963, DCFS may request reduced requirements for a kinship specific licensee by submitting a variance request outlining the requirements to be varied and how the request does not impact the health and safety of the specific foster child or sibling group. This variance request must be approved in writing by the director of OL before it may become effective.			☐	☐	☐	☐	☐

Therapeutic Schools - Inspection Checklist						
(Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Administration	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-15-4(1) Each therapeutic school provider shall develop, maintain, and follow a current policy and procedure manual, that includes: (a) a requirement that a client may not attend a therapeutic school unless the school [has been presented]receives a certificate of immunization for the client from a licensed physician or an authorized representative of the state or local health department stating that the client has received immunization in accordance with Rule R396-100; (b) a procedure for quarterly evaluation and assessment of the needs of each client; (c) an emergency transportation plan describing how the therapeutic school provider will safely transport each client to the client's legal guardian within 48 hours once the plan has been initiated; (d) a detailed description of the therapeutic school's client admission, exclusion, and expulsion criteria and procedures; (e) a requirement that the therapeutic school provider may not admit or provide services to an individual who: (i) within the past two years, has attempted suicide or made serious self-harm gestures requiring medical or therapeutic treatment; (ii) has a mental health diagnosis of psychosis, schizophrenia, severe depression, mental retardation, or a severe mental illness requiring medical or therapeutic treatment; (iii) is violent, highly combative, or physically or sexually aggressive; (iv) presents substantial security risks; (v) requires medical detoxification; (vi) lacks the ability to engage in a rational decision-making process or exhibits severely impaired judgment; or (vii) has a history of repeated runaway attempts or incidents; and (f) a requirement that the school shall expel a client who exhibits high-risk behavior or conditions, including a client who: (i) attempts suicide or makes serious self-harm gestures requiring medical or therapeutic treatment; (ii) has a psychosis, schizophrenia, severe depression, mental retardation, or a severe mental illness requiring medical or therapeutic treatment; (iii) is violent, highly combative, or physically or sexually aggressive; (iv) presents substantial security risks; (v) requires medical detoxification; (vi) lacks the ability to engage in a rational decision-making process or exhibits severely impaired judgment; (vii) runs away or has attempted to run away more than two times; (viii) uses or attempts to use illegal substances more than two times; or (ix) exhibits any other behavioral or emotional conditions that require more intense supervision and treatment than that permitted in a therapeutic school.	☐	☐	☐	☐	☐	
R501-15-4(2) Each therapeutic school provider's policy and procedure manual shall outline the provider's: (a) academic accreditation or disclosure that the school is not accredited; (b) curriculum; (c) criteria for awarding course credit, and whether credits are transferable; (d) policy on grading, progress assessment, and testing; (e) academic and career counseling; (f) academic activities; (g) graduation requirements; (h) post-graduation planning service; (i) method of providing on-site specialized structure and supervision; (j) methods for providing off-site specialized structure and supervision; (k) policy for services and treatment for managing a client's: (i) disability; (ii) emotional development; (iii) behavioral development; (iv) familial development; and (v) social development; (l) behavior management practice; (m) individual, group, or family counseling service; (n) school rules; (o) food service policy and weekly menu; (p) physical education and recreational activity policy; (q) client rights statement; (r) policy regarding permitted and prohibited items and dangerous weapons that: (i) is developed in consideration of the age and behavioral characteristics of the client population the program serves; and (ii) describes how dangerous weapons are stored and any circumstances when they may be accessible to a client; (s) client grievance policy and appeal process for the grievance policy; and (t) contact information for the office of licensing.	☐	☐	☐	☐	☐	
R501-15-4(3) Each therapeutic school provider shall provide the client manual described in this section to each client and each client's legal guardian before the therapeutic school provider may accept any payment or process any application to provide services.	☐	☐	☐	☐	☐	
Financial Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

	Staff Files	Policy	Client Files	Interviews		Walk Through		Variable Risk High or Extrem	
	R501-15-5(1) Each therapeutic school provider shall provide an itemized accounting of expenditures made on behalf of a client before requiring reimbursement from the client's guardian.			☐	☐	☐	☐	☐	
	R501-15-5(2) Each therapeutic school provider shall maintain an accurate log of each fund deposited and each withdrawal made for the personal use of each client.			☐	☐	☐	☐	☐	
	Staff Requirements			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-15-6(1) Each owner and board member of a therapeutic school shall successfully complete a minimum of eight hours of annual training relating to therapeutic school services.			☐	☐	☐	☐	☐	
	R501-15-6(2) Each therapeutic school provider shall ensure a therapeutic director or equally qualified acting director is immediately available to staff by telephone and able to arrive on-site within one hour after a staff telephone call for assistance.			☐	☐	☐	☐	☐	
	R501-15-6(3) Each therapeutic school provider shall ensure the therapeutic director or qualified acting director: (a) is at least 25 years of age; (b) has a bachelor's degree in social work or a related field, or a minimum of three years of documented training or experience in providing therapeutic school or residential treatment services; and (c) has a minimum of two years of therapeutic school or residential treatment program supervisory experience.			☐	☐	☐	☐	☐	
	R501-15-6(4) Each therapeutic school provider shall ensure there is at least one direct care supervisor or supervisor designee on duty at all times.			☐	☐	☐	☐	☐	
	R501-15-6(5) Each therapeutic school provider shall ensure a supervisor or supervisor designee: (a) has a minimum of six months of experience providing services to children in out-of-home placements; and (b) meets each qualification, including requirements for education, experience, licensing or certification, and current annual continuing education and training directly related to providing: (i) specialized structure and supervision of clients; and (ii) services or treatment related to a client's disability, emotional development, behavioral development, familial development, or social development.			☐	☐	☐	☐	☐	
	R501-15-6(6) Each therapeutic school provider shall maintain a staff manual that includes: (a) specific job descriptions for each staff position; (b) staff qualifications for each staff position, including requirements for education, experience, and licensing or certification; (c) a requirement for continuing education, competency and proficiency, and job-specific training; and (d) the required training for staff who will work with clients with a history of failing to function at home or school.			☐	☐	☐	☐	☐	
	R501-15-6(7) Each therapeutic school provider shall ensure at least two direct care staff are on duty to provide direct supervision to clients at all times.			☐	☐	☐	☐	☐	
	R501-15-6(8)(a) Each therapeutic school provider shall ensure that each client who has earned the privilege of unsupervised time off-site engages in two-way communication with on-duty direct care staff once every four hours. (b) Each therapeutic school provider shall develop and adhere to a policy that outlines steps for staff to take if a client fails to check in with staff once every four hours.			☐	☐	☐	☐	☐	
	R501-15-6(9) Each therapeutic school provider may not include support staff in the minimum staff-to-client ratios.			☐	☐	☐	☐	☐	
	R501-15-6(10)(a) Each therapeutic school provider shall ensure the following minimum waking hour staff-to-client ratios are followed: (i) two direct care, on-duty staff for eight or fewer clients present; (ii) three direct care, on-duty staff for nine to 24 clients present; (iii) four direct care, on-duty staff for 45 to 48 clients present; (iv) five direct care, on-duty staff for 49 to 96 clients present; and (v) six direct care, on-duty staff for more than 97 clients present with a ratio of no less than one direct care, on-duty staff for every 20 clients present. (b) Each therapeutic school provider shall comply with waking hour direct care staff-to-client ratios listed in this subsection unless authorized by the office to modify the required ratios.			☐	☐	☐	☐	☐	
	R501-15-6(11)(a) Each therapeutic school provider shall ensure the following minimum sleeping hour staff-to-client ratios are followed: (i) two direct care, on-duty staff for 48 clients present; and (ii) three direct care, on-duty staff for 49 or more clients present with no less than one direct care, on-duty staff for every 40 clients present. (b) Each therapeutic school provider shall comply with sleeping hour direct care staff-to-client ratios listed in this subsection unless authorized by the office to modify the required ratios.			☐	☐	☐	☐	☐	
	Client Services			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-15-7(1) Each therapeutic school provider shall ensure each service plan includes a quarterly assessment of the effectiveness of the therapeutic school's policy, procedure, and practice in meeting each client's needs.			☐	☐	☐	☐	☐	
	R501-15-7(2) Each therapeutic school provider shall provide each client's legal guardian with a copy of each service plan within two weeks after the service plan is developed or updated.			☐	☐	☐	☐	☐	
	R501-15-7(3) Each therapeutic school provider that must travel more than 30 miles to an emergency room or 24-hour urgent care facility shall ensure a medical practitioner and a licensed mental health therapist are on-call to the program.			☐	☐	☐	☐	☐	

☐	Staff Files	Policy	Client Files	Interviews			Walk Through		Variable Risk High or Extrem	
	R501-15-7(4) Upon admission, each therapeutic school provider shall inform each client of the right to consult with a medical practitioner or a licensed mental health therapist.			☐	☐	☐	☐	☐		
	R501-15-7(5)(a) Each client shall receive an immediate medical assessment by a certified wilderness responder, certified EMT or medical practitioner for the following: (ii) a serious injury; or (iii) upon client request to see a medical practitioner. (b) Each therapeutic school provider shall attach the medical assessment documentation to an incident report.			☐	☐	☐	☐	☐		
	R501-15-7(6) Each therapeutic school provider shall post each monthly schedule of activities in the common area and in the office and retain each activity schedule for at least one year.			☐	☐	☐	☐	☐		
	R501-15-7(7) Each therapeutic school provider's academic curriculum shall receive accreditation by an entity recognized by the Utah State Board of Education, or the school shall present an educational service plan and educational funding plan in accordance with Section 26B-2-116.			☐	☐	☐	☐	☐		
	R501-15-7(8) Each therapeutic school provider shall provide the school's curriculum to each client and the client's legal guardian before accepting any payment or processing any application to provide services.			☐	☐	☐	☐	☐		
	R501-15-7(9) Each therapeutic school provider shall review and update the school's curriculum annually.			☐	☐	☐	☐	☐		
	R501-15-7(10) Each therapeutic school provider shall provide any modifications to the curriculum to each client and the client's legal guardian within two weeks of any curriculum change.			☐	☐	☐	☐	☐		
	R501-15-7(11) Each therapeutic school provider shall monitor and document each client's academic progress and communicate the progress to the client's legal guardian each month.			☐	☐	☐	☐	☐		
	Physical Environment			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes	
	R501-15-8(1) Each therapeutic school provider shall provide indoor common areas for group activities, that may include a gymnasium, recreation area, cafeteria, classrooms, library, and lounge.			☐	☐	☐	☐	☐		
	R501-15-8(2) Each therapeutic school provider shall comply with the following regarding the physical aspects of the facility: (a) each common area space contains at least 30 square feet per client; (b) there are at least three feet between beds and two feet at the end of each bed; (c) each bedroom ceiling is at least seven feet high; (d) each multiple-occupant bedroom contains at least 50 square feet per client, and each single occupant bedroom contains at least 80 square feet per client, excluding storage space; and (e) each client has at least 30 cubic feet of private storage space.			☐	☐	☐	☐	☐		
	R501-15-8(3) Each therapeutic school provider shall provide a school desk or table, light, and chair for each client.			☐	☐	☐	☐	☐		



Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extrem	
Child Placing Adoption Checklist (Revised Jan 2026)						
R = Reviewed. UR = Under Review. Under Review indicates evidence of potential noncompliance. NA = Not Assessed during this inspection.						
Legal Requirements	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-7-3(1) In addition to this rule, all child placing adoption agencies shall comply with R495-876, R501-11, R501-2-1 through R501-2-5, R501-2-8 through R501-2-14, R501-14; R501-22, Title 58, Chapter 60; Title 62A, Chapters 2 and 4a; Section 76-7-203; 78A-6; 78B-6; 78B-13; 78B-15; and all other applicable local, State and Federal laws.	☐	☐	☐	☐	☐	
R501-7-3(2) Child placing adoption agencies that do not arrange housing for birth mothers are exempt from R501-2-5, 10, 11, 12, and 22.	☐	☐	☐	☐	☐	
R501-7-3(3) A child placing adoption agency shall: (a) be legally responsible for the child following relinquishment of the child to the adoption agency until the adoption is finalized, unless a court of competent jurisdiction or applicable law places legal responsibility with another party, in accordance with Section 78B-6-134; (b) comply with the Indian Child Welfare Act; (c) obtain a child placing foster license and comply with R501-12 if providing foster care; (d) obtain a residential support license and comply with R501-22 if providing residential support services to preexisting parent(s); (e) comply with the Interstate Compact on the Placement of Children, in accordance with Section 62A-4a-701 et seq; and (f) ensure that its employees, contractors, volunteers and agents comply with all laws relating to adoption services.	☐	☐	☐	☐	☐	
R501-7-3(4) The Division of Child and Family Services shall additionally comply with R512-40 for recruitment, home study and approval; R512-41 for qualifying and adoptive family and adoptive placement; R512-302 for responsibilities pertaining to out of home caregivers and any other section of 62A-4a and R512 that governs the provision of adoptive services to child welfare clients served by the Division of Child and Family Services. (a) The aforementioned child welfare statute and rule shall supersede this rule when in conflict for child welfare clients served by the Division of Child and Family Services.	☐	☐	☐	☐	☐	
Administrative Ethics and Responsibilities	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			RS01-7-3(1) Child placing adoption agencies shall: (a) identify and strictly adhere to accurate accounting practices, including all fee requirements of this rule; (b) always act in the best interest of a child; (i) best interest determinations are made by considering a number of factors related to the child's circumstances including age and developmental needs and the parent or caregiver's circumstances and capacity to parent the child to adulthood and shall consider the pre-existing parent(s)' wishes when parental rights are voluntarily relinquished; (c) provide services and adhere to ethical practices that support and comply with all client rights and responsibilities; (d) develop and comply with processes that are free from fraud, duress or undue influence and avoid and mitigate conflicts of interest in order to preserve the protections of clients to include: (i) not give preferential treatment to its board members, employees, volunteers, agents, consultants, independent contractors, donors, or their respective families with regard to child placing decisions; (ii) not accepting or soliciting donations from an adoptive family that is under consideration for placement of a child or pending finalization of an adoption; (A) generalized mass solicitation through newsletters or the media shall not constitute a violation under this rule; (iii) not coercing or incentivizing pre-existing parent(s) to make a plan of adoption or to relinquish their parental rights; (iv) not permitting its employees, volunteers, agents, consultants, or independent contractors to provide adoption services to both the pre-existing parent(s) and the adoptive parent(s) unless all parties are made aware of potential conflicts of interest and sign a voluntary consent; (v) inform clients that they are free to select independent attorneys and other non-child placing adoption services; (A) client bears the responsibility to select a competent provider and their choice may affect costs incurred; (vi) not referring any individual to services in which the agency's board members, volunteers, employees, agents, consultants, independent contractors, or their respective families are engaged, without first disclosing potential conflicts of interest and informing said individuals that they are free to select independent adoption service providers; and (vii) require members of the governing body to disclose, in writing, to the chairperson of the governing body and the Office of Licensing, any direct or indirect financial interest in the agency; (e) manage and share information while still preserving confidentiality when required. This includes: (i) documenting information shared with potential adoptive parent(s) regarding unknown pre-existing parent(s), Indian Child Welfare Act, and any known information that could potentially disrupt an adoptive placement; (ii) respond to requests for information from clients and former clients within 30 days and document all requests for information or actual sharing of information to/from birth families, adoptees, adoptive families, and others; (iii) provide non-identifying information in client files that can allowably be shared, and shall comply with previous releases and established policies; (iv) the agency shall refer clients to the Mutual-Consent Voluntary Adoption Registry through Department of Health Vital Records if adult adoptees or birth family members want to reunite; and (v) in more urgent circumstances that could have serious implication to any client or prior client, the agency will utilize prior contact and emergency contact information, as well as engage in simple social media and search engine inquiries to locate and communicate with former clients; (vi) agencies may engage in a fee based more extensive service to search if desired; (vii) the agency may share information with third party search providers only if consent has been given by the affected party; (viii) not misrepresent or withhold any facts or allowable adoptive parent(s) or child/pre-existing parent(s) information relating to its services, involved individuals, or the applicable law; (f) accept and utilize third party assessments, evaluations, references, home studies or pre-placement evaluations only if received directly from the document's author; (g) preserve the confidentiality and content of client files; (h) with respect to adoption services an agency shall refer to or utilize only agencies, entities or individuals that are authorized to provide the service by the laws of this state or the jurisdiction in which that agency, entity or individual performs the service; (i) provide at least 30 days' prior written notice to the Office of Licensing that the agency is: (i) dissolving or ceasing to provide child placing services; or	□	□	□	□	□	

	Staff Files Policy Client Files	Interviews Walk Through Variable Risk High or Extrem				
	(ii) Implementing significant changes in adoption services provided, such as adding or eliminating intercountry adoption. (j) Provide copies of all documents signed by clients directly to those clients upon request.					
	R501-7-3(2) In addition to policy and procedure requirements outlined in R501-2, agencies shall develop and adhere to the following adoption-related policies and procedures: (a) a process regarding how to transfer a relinquishment to another agency in compliance with 78B-6-124 (7); (b) a process to identify a high needs child as defined in 62A-4a-601, and once identified comply with 62A-4a-609 including disclosure and training to adoptive parent(s); (c) a process for the temporary placement of children awaiting adoptive placement for over 30-days; (d) a process and standards for the evaluation and approval or denial of an adoptive home study or pre-placement evaluation; (e) process and standards for the evaluation and approval or denial of applications from prospective adoptive parent(s); (f) a written plan for contact, file maintenance, and record retrieval in the event that the agency ceases to provide child placement adoption services; (i) this plan may involve a secondary licensed or file retention entity; (g) a process for identifying the pre-existing parent(s)' utilization of alternative payment sources including any public assistance that may defray adoptive parent(s) costs; (h) policy identifying what is allowable child/pre-existing parent(s) information to be shared with potential adoptive parent(s), including the development of releases of information as needed; (i) policy identifying what is allowable adoptive parent(s) information to be shared with pre-existing parent(s) including the development of releases of information as needed; (j) process for refunds to include a process for refunding to adoptive parents monies they paid in excess of actual expenses or disclosed agency fees.; and (k) written policy to be provided to the adoptive parent(s) outlining how the match is determined, its relationship to any fees, and how it is managed by the agency.					
	Staffing Requirements	R UR NA Corrected During Inspection Technical Assistance Given	Notes			
	R501-7-5(1) A child placing adoption agency shall have at least one social work supervisor responsible for directly supervising all staff and volunteers who provide adoption services to clients.					
	R501-7-5(2) If an Executive Director is serving as a social work supervisor, they shall not supervise more than four staff and volunteers who provide adoption services to clients.					
	R501-7-5(3) Each social work supervisor shall be licensed in this state as a mental health therapist, shall comply with the Utah Mental Health Professional Practice Act, and shall have at least one year of full time paid professional experience in a licensed child placing adoption agency.					
	R501-7-5(4) A social work supervisor may not supervise more than eight staff and volunteers who provide adoption services to clients.					
	R501-7-5(5) An executive director shall have at least one year of full time paid experience in a licensed child placing adoption agency.					

Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extrem			
R501-7-5(6) All staff that provide services shall receive a minimum of 20 hours of pre-service training, prior to independently providing direct client services, and 12 hours annual in-service training. (a) Training content shall include: (i) agency policy and procedures; (ii) adoption ethics, laws, and rules; (iii) the provision of professional and trauma informed adoption practices; and (iv) any evaluations they will be performing. (b) Staff will be supervised for adherence to training topics.			☐	☐	☐			
Fees and Disclosures			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-7-6(1) All fees, costs and expenses whether actual or estimated must be itemized in accordance with this Rule and Utah Code Ann. 78B-6-140.			☐	☐	☐	☐	☐	
R501-7-6(2) A child placing adoption agency may charge adoptive parent(s) agency fees which include administrative and professional services provided on behalf of the adoptive parent(s), including but not limited to: (a) agency overhead; (b) personnel; (c) background screenings for adoptive parent(s) and staff; (d) training; (e) insurance; (f) legal services for the agency; (g) advertising/recruiting; (h) post-placement visit; (i) agency staff support throughout pregnancy, birth, placement and post placement; (j) home studies, if completed by the agency; and (k) home study updates, if completed by the agency; (l) copies of purchased home studies and updates are to be provided to the subjects of these documents upon request.			☐	☐	☐	☐	☐	
R501-7-6(3) An agency fee may be charged as a flat fee or be itemized and both must clarify what is included or specifically excluded.			☐	☐	☐	☐	☐	
R501-7-6(4) Any fee billed inclusive of an agency fee shall not be billed additionally outside of that agency fee.			☐	☐	☐	☐	☐	

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-7-6(5) An agency may charge and accept payment from the prospective adoptive parent(s) only for reasonable, actual, estimated or outstanding adoption related expenses of the pre-existing parent(s) which are itemized outside of any agency fee. These expenses are limited to the following: (a) additional counseling; (b) adoption related legal fees to utilize an independent attorney for the adoption; (c) maternity expenses limited to pregnancy related clothing, pre-natal vitamins, other non-medical pregnancy related needs; (d) medical and hospital expenses limited to pregnancy and childbirth related medical expenses for the mother/child; and (e) temporary living expenses limited to the duration of the pregnancy and confinement of the pre-existing parent(s) or directly affected person and include only: (i) food; (ii) transportation including bus passes, gasoline, car maintenance, car payments, and taxi/ride share services; (iii) housing; (iv) utilities and telephone; (v) reasonable and minimal incidentals; (vi) sufficient apparel for the weather and circumstances; (vii) daily living household supplies; (viii) travel between the mother's or father's home and the location where the child will be born or placed; (f) any other expense not explicitly outlined in this rule shall be reasonably related to the adoption, incurred for a reasonable amount and not paid for the purpose of inducing a birth parent to place the child for adoption. If such fees are charged or paid, the agency shall notify the Office of Licensing.	☐	☐	☐	☐	☐	
			R501-7-6(6) An agency may charge an adoptive or potential adoptive parent(s) for either the actual adoption related expenses in regard to the pre-existing parent(s) and directly affected individuals or a flat fee estimate of adoption related expenses. Regardless of the fee structure, fees and expenses must be itemized in accordance with this Rule and Utah Code Ann. 78B-6-140. (a) the agency must disclose whether their adoption related expenses charged are actual or estimated and share the agency policy on refunds or re-appropriation prior to charging adoptive parent(s). (b) If the agency charges a flat fee for adoption related expenses, the amount must be stated in the disclosure outlined in (7) of this section and the policies related to refunds, increases or decreases in those fees must be outlined in the disclosure. (c) If the agency charges a fixed amount for adoption related expenses, it must be outlined in the disclosure and capped at that amount. It shall be disclosed whether or not the flat adoption related expenses are or are not refundable in the disclosure. (d) Over collection of adoption related expenses that are not refunded is only permissible with estimated adoption related expenses if: (i) any overage will be used to support the adoption related expenses of another adoption of the adoptive parent(s) that paid the expenses originally or refunded to the adoptive parent(s) upon their request; (ii) any over-collected adoption related expenses shall not be used for the benefit of the agency or anyone associated with the licensee or as a payment to a pre-existing parent.	☐	☐	☐	☐	☐	

Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extrem
R501-7-6(7) A child placing adoption agency shall provide a written disclosure statement of all agency fees, flat fees and adoption related expenses that prospective adoptive parent(s) may incur before the agency accepts any payments, or enters into any agreement with the prospective adoptive parent(s). (a) The written disclosure shall identify and itemize: (i) each fee and the services associated with each fee; and (ii) each adoption-related expense. (b) If providing only estimated expenses provide the average cost for each itemized fee and each adoption-related expense for the preceding two fiscal years, and the maximum amount that may be charged for each fee and adoption related expense. (c) The written disclosure shall identify any fee that is non-refundable. (d) If the agency is charging a flat fee, the disclosure shall contain full acknowledgment by prospective adoptive parents of this fee structure and refund ability of any portion of the flat fee. (e) The written disclosure shall be signed and dated by the prospective adoptive parent(s) and an agency representative and maintained in the adoptive parent(s) file.					
R501-7-6(8) An agency shall not charge prospective adoptive parent(s) for any fees or adoption related expenses that the client obtained independently or were paid for by another entity, including any public assistance.					
R501-7-6(9) An agency shall not charge adoptive parent(s) for any fee that was not included in the written disclosure without providing written agreement and justification approved by the prospective adoptive parent(s), and either the Office of Licensing or the Court.					
R501-7-6(10) An agency shall not directly or indirectly offer, give, or attempt to give money or another thing of value in order to induce or influence pre-existing parent(s) in the adoption process.					
R501-7-6(11) The agency shall retain documentation for any adoption related expense exceeding twenty five dollars, which may include receipts, lease agreements, signed fund transfers to pre-existing parent(s) in reasonable amounts in order to cover basic daily needs such as food and household supplies, and any other pertinent documentation.					
R501-7-6(12) An agency shall not charge the adoptive parent(s) for the temporary living expenses of any person other than the pre-existing parent(s) or directly affected persons					
R501-7-6(13) An agency shall not charge the adoptive parent(s) for any expenses that are post-confinement, with the exception of post-placement counseling if agreed upon.					
R501-7-6(14) A birth mother who decides not to place her child shall not be responsible for reimbursing the costs of any goods or services provided to her by the prospective adoptive parent(s) or the child placing adoption agency during her pregnancy unless they are first convicted of fraud.					
R501-7-6(15) Child placing adoption agencies that provide or pay for pre-existing parent(s)' transportation to the State of Utah shall also ensure that the pre-existing parent(s)' return transportation to their home state is provided, regardless of whether the pre-existing parent(s) decides to relinquish parental rights.					
R501-7-6(16) The agency shall create an affidavit of itemized accounting of the actual fees and adoption related expenses paid by the adoptive parent(s). (a) The agency shall utilize an affidavit form provided by the Office of Licensing or a form inclusive of the Office's form content. (b) The affidavit shall be executed as follows: (i) a copy shall be signed by the adoptive parent(s); (ii) all adoption related expenses shall be itemized and include a declaration that Section 76-7-203 has not been violated; (A) itemized expenses in the affidavit shall align with those verified by pre-existing parents in R501-7-11(3)(n); (iii) the affidavit shall include a declaration of all gifts, property, or other items that have been or will be provided to the pre-existing parent(s), including the source of the gifts, property or other items; (iv) the affidavit shall include a declaration of the state of the residence of the pre-existing parent(s) and the prospective adoptive parent(s); (v) the affidavit shall include a declaration of all public funds used for any medical or hospital costs in connection with the pregnancy, delivery of the child, or care of the child; and (vi) the affidavit shall include the signature of an agency representative with adequate knowledge to verify the contents of the affidavit are accurate and complete.					



Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem		
Services to Pre-existing Parents			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
R501-7-7(2) Child placing adoption agencies other than DCFS shall: (a) offer pre-existing parent(s) all available allowable adoptive parent(s) information unless waived in full or part by the pre-existing parent(s) as early in the matching process or consent to adopt process as reasonable; (b) per 78B-6-119, accept voluntary relinquishments only after offering a minimum of three sessions of adoption related counseling to any person who is considering relinquishing a child for adoption prior to accepting the consent or relinquishment. This counseling shall include at a minimum: (i) parental rights prior to relinquishments; (ii) alternative options for the child and pre-existing parent(s); and (iii) adoption issues including grief/loss; (c) provide complete and accurate information to the pre-existing parent(s) regarding their decision to consent to adopt or relinquish; (d) meet in-person, via video, or via telephone with the pre-existing parent(s) to review the designated adoption orientation form provided by the Office; (i) pre-existing parent(s) will be given the opportunity for questions/clarifications before initialing and signing the document; (ii) a pre-existing parent(s) under the age of 18 shall meet privately with the adoption worker unless they waive the option to meet privately; (e) ensure the written consent to relinquishment includes language acknowledging that the pre-existing parent(s) was afforded adoption related counseling, and that the relinquishment is completely voluntary, permanent and irrevocable under Utah Law once signed; (i) a child placing adoption agency shall wait at least 24 hours after the birth of a child before taking the birth mother's relinquishment of parental rights or legal consent to the adoption of her child, in accordance with Section 78B-6-125 or the laws of the state governing the relinquishment.			☐	☐	☐	☐	☐	
R501-7-7(3) If an agency arranges housing for pre-existing parents, assure that such housing complies with the following minimum standards: (a) housing is in compliance with health, fire, zoning, and other applicable laws and regulations; (b) if the housing meets the definition of Residential Support (R501-22) the agency shall obtain a Residential Support license through the Office of Licensing; (c) housing is clean, well-maintained and adequately furnished; (d) birth mothers shall not share bedrooms with other birth mothers; (e) laundry equipment and supplies shall be available; and (f) adequate nutritious food, or resources to obtain food, is available.			☐	☐	☐	☐	☐	
R501-7-7(7) The agency shall be responsible to encourage and facilitate prenatal and medical care of the birth mother.			☐	☐	☐	☐	☐	
R501-7-7(8) A child placing agency shall inform pre-existing parent(s) of their information that will be shared with adoptive parent(s) including their detailed health history and a genetic and social history in accordance with Section 78B-6-143.			☐	☐	☐	☐	☐	
R501-7-7(9) A child placing adoption agency shall inform pre-existing parent(s) of Utah's Mutual Consent Voluntary Adoption Registry, Section 78B-6-144.			☐	☐	☐	☐	☐	
R501-7-7(10) A child placing adoption agency shall assist the birth and adoptive parent(s) in creating a post-placement contact agreement, including: (a) whether the birth parent wants to disclose their identity to the adoptee or the adoptive family; (b) contact about or from the child or parents, directly or indirectly, in the future and how that will occur; (c) that such agreements are non-binding except in certain public child welfare cases; and (d) Contact agreements shall be updated only when initiated by the previous clients and maintained in case file records.			☐	☐	☐	☐	☐	
Services to Children			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

□	Staff Files	Policy	Client Files	Interviews	Walk Through	Variable Risk High or Extrem
			R501-7-8(1) Assessment. (a) A needs assessment for the child shall be completed to obtain information and identify characteristics which should be given consideration in selecting and preparing a child for an adoptive family and promote appropriate placement for the child. (b) The needs of the child will be determined through this assessment and shall evaluate for high needs or special needs as defined in this chapter. (c) A report(s) regarding all assessment information shall be given to the adoptive parent(s) prior to placement. (d) If the child is an infant that is not defined as special needs or high need, information shall be obtained from the pre-existing parent(s) and any legal guardian to include all allowable child/pre-existing parent(s) information as defined in this chapter. This information should include: (i) If the child is older than six months the same information from Section 2 above, shall be obtained from the birth or legal parent; (ii) additional information shall be obtained using an interdisciplinary approach which may include input from: caseworkers, therapists, pediatricians, teachers, previous caregivers, foster parents, nurses, psychologists, and other consultants. (e) The assessment shall additionally include: (i) information pertaining to changes in caregivers including foster care, separation experiences and description of the child's behaviors; (ii) all evaluations regarding a child's development including: physical, social, emotional, mental health and cognitive; (iii) the child's educational records, and any special educational needs; (iv) talents and interests; and (v) if the child is identified as having special needs or is a high needs child as defined in 62A-4a-601, specific training for prospective adoptive parent(s) is statutorily mandated.	☐	☐	☐
			R501-7-8(2) Recruitment of adoptive families. (a) Child placing adoption agencies shall recruit adoptive families that are able to meet the needs of children the agency serves. (b) If the family states they would be open to a child with special needs or high needs, they will complete training specific to identified needs and in compliance with 62A-4a-609-2.	☐	☐	☐
			R501-7-8(3) Matching. (a) The selection of the adoptive family and the adoptive family's decision to adopt a specific child shall be based on the following: (i) the child's assessment; (ii) adoptive family's ability to meet the identified and potential needs of the child; (iii) the wishes of the pre-existing parent(s) who voluntarily relinquish their rights, the adoptive parent(s), and when applicable, the child, shall be considered.	☐	☐	☐

	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-7-8(4) Placement. (a) A child placing adoption agency shall attempt to place siblings together when appropriate for the children's needs and pre-existing parent(s) wishes. (b) A child shall be placed with the adoptive family at the earliest time possible after being freed for placement or adoption. (c) A child placing adoption agency shall have an individualized written adoptive placement and transition plan that includes the child's current caregivers, the adoptive parent(s), and the child, to facilitate the child's transition into the adoptive family and ensures the family's ability to meet the child's needs. (i) The transition plan shall consider and include as applicable: (A) the child's stated preferences; (B) the child's identified religion; (C) identification of services the family and child may need based on assessment information; (D) statement of who is responsible for identifying services and who is responsible for paying for such services; (E) time frames for transition that consider and accommodate the identified and potential needs of the child in preparing the child for placement; and (F) developmentally appropriate counseling with the child to address to mitigate transition related emotional trauma. (d) If a child placing adoption agency other than DCFS assumes custody of a child and the child is not able to be directly placed in an adoptive placement: (i) the agency may temporarily place the child in a currently home studied adoptive home for up to 30 days; or (ii) if the child needs temporary care for more than 30 days, the agency shall contract with a licensed foster care program or obtain a license to provide foster care services for children in its custody, in accordance with R501-12. (e) A private child placing adoption agency shall obtain a copy of the foster home or facility license prior to placing a child, and shall retain the license in the child's case file. (f) If a child is not placed within 30 days after relinquishment or after determination of availability for adoption by the court, the agency shall document its efforts to screen the child with other child placing agencies and shall list the child with local, regional, and inter-state adoption exchanges.	☐	☐	☐	☐	☐	
			R501-7-8(5) Post Placement Service. (a) The child placing agency shall monitor and support each placement until the adoption is final. (b) An agency social worker shall contact the adoptive family within 2 weeks of the placement to offer support. This does not count towards the pre-finalization visit. (c) Prior to finalization, a minimum of one in-home supervisory visit with both parents and child present shall be made by an agency social worker: (i) to assess that the child and family are adjusting and child is receiving necessary care, nurturance, medical care, and services as needed. (d) The agency shall monitor who has legal and physical responsibility for the child at all times.	☐	☐	☐	☐	☐	
			R501-7-8(6) Disruption. (a) If a disruption occurs, a child placing agency shall provide for the care of the child. (i) The placement shall: (A) be in a currently home studied adoptive home for no longer than 30 days unless it is the identified subsequent adoptive placement; (B) be in a licensed or certified foster home governed by Rule R501-12; or (C) be approved by a judge.	☐	☐	☐	☐	☐	
			Services to Adoptive Parents	R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes



Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem
R501-7-9(2) A child placing adoption agency other than DCFS shall: (a) provide the adoptive parent(s) orientation form to potential adoptive parent(s) who shall sign and initial the form and shall be offered the opportunity to ask clarifying questions prior to match or payment of any fees in excess of \$500.00; (i) adoptive parent(s) will be given the opportunity for questions/clarifications before initialing and signing the document; (b) provide prospective adoptive parent(s) with a written description of their services, fees, policies and procedures; (c) explain the adoption process and the pre-existing parent(s)' rights, including the status of any putative father, to the prospective adoptive parent(s); (j) a copy of the Office provided pre-existing parent(s) adoptive orientation form shall be provided to adoptive parent(s) for information purposes with an acknowledgement that they have discussed and received this information; (d) provide training as outlined in 62A-4a-609 in regards to high needs child, as required; (e) per 62A-4a-607 the agency shall inform each prospective adoptive parent(s) that the state has children available for adoption and that adoption from the Division of Child and Family Services incurs no agency fees and adoption assistance may be available when adopting children in the custody of the state; (f) inform adoptive parent(s) that when a child has a disability, the child may be eligible for SSI benefits and/or federal adoption assistance. The Agency shall refer the potential adoptive parent(s) to coordinate with the Division of People with Disabilities for further disability resources and with Division of Child and Family Services to apply for potential federal adoption assistance; and (g) a child placing adoption agency shall inform prospective adoptive parent(s) of Utah's Mutual Consent Voluntary Adoption Registry, Section 78B-6-144.			☐	☐	☐	☐	☐
R501-7-9(3) A home study completed by an adoption service provider as outlined in 78B-6-128-2(C) for each adoptive family shall include: (a) a recommendation to the court regarding the suitability of the prospective adoptive parent(s) or placement of a child; (b) a description of in-person interviews with the prospective adoptive parent(s), prospective adoptive parent(s)', children, and other individuals living in the home; (c) criminal background and child abuse screening of adoptive applicants and other adults living in the home in accordance with R501-14, and Sections 53-10-108(4) and 78B-6-128; (i) agencies must separately obtain the child abuse registry report through the Division of Child and Family Services in Utah and any out of state comparable entities in order to show compliance with 78B-6-128; (d) written descriptions from at least two non-related and one related references regarding the character and suitability of the prospective adoptive parent(s) for parenting an adoptive child; (e) a medical history and a doctor's report, based upon a doctor's physical examination of each applicant, made within two years prior to the date of the application; (f) description of inspections of the home, to determine whether sufficient space and facilities exist to meet the needs of the child and whether basic health and safety standards are maintained; and (g) description of documented income for each adoptive applicant and a written plan for adoptive applicants who work outside the home addressing how they shall provide security and responsible child care to meet individual child needs.			☐	☐	☐	☐	☐
R501-7-9(4) The adoptive applicants shall be informed, in writing, and within ten business days after the decision is made, as to the acceptance or the reasons for the denial of their home study. (a) The agency shall provide applicants with a written copy of the agency's appeal process, which shall include the right to submit a written appeal and request for reconsideration, upon order of the court in accordance with Section 78B-6-128.			☐	☐	☐	☐	☐
R501-7-9(5) A child placing adoption agency shall select applicants who: (a) are able to provide the continuity of a caring relationship; (b) are informed with regard to a child's ethnic, religious, cultural, and racial heritage; and (c) understand the needs of a child at various developmental stages.			☐	☐	☐	☐	☐
R501-7-9(6) The agency's policies regarding the consideration of religion and marital status in the selection of adoptive families shall be clearly stated in its initial consultation with prospective adoptive parent(s). This disclosure shall also be clearly stated in writing on the adoptive parent(s)' application for services forms.			☐	☐	☐	☐	☐

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extrem		
	R501-7-9(7) The agency shall verify that an applicant's income is sufficient to provide for a child's needs.			☐	☐	☐	☐	☐	
	R501-7-9(8) The agency shall not reject an applicant solely based upon the applicant's choice to work outside the home. Applicants who work outside the home shall provide a written plan describing how they shall provide security and responsible child care to meet the individual child's needs.			☐	☐	☐	☐	☐	
	R501-7-9(9) Except when authorized by court order pursuant to Section 78B-6-128, a child placing adoption agency shall not place a child in an adoptive home until the home study and each adult's criminal and abuse background screenings have been approved.			☐	☐	☐	☐	☐	
	R501-7-9(10) Matching. (a) Disclose all allowable child/pre-existing parent(s) information to the prospective adoptive family. (b) Ensure known special needs are disclosed and referrals and information are provided as necessary to prepare the family to meet the long term needs of the child. (c) A child placing adoption agency shall not make a legal risk placement unless the prospective adoptive parent(s) have first given their written consent, indicating that they have been fully informed of the specific risks involved. (d) Develop the capacities of the parents to meet the ongoing needs of the child according to the child's needs and the transition plan. (e) Matches may only occur once sufficient non-identifying information sharing has occurred to allow for informed decision making by both parties.			☐	☐	☐	☐	☐	
	R501-7-9(11) Placement. (a) A child placing adoption agency shall provide continuing support to the child and the adoptive family after placement and before finalization of the adoption, to include: (i) providing or making referrals to services such as counseling, crisis intervention, respite care, and support groups; and (ii) monitoring the child's adjustment and development. (b) The frequency of home visits, office contacts, telephone calls, and other contacts by the child placing adoption agency shall depend on the needs of the child and the adoptive family and may vary depending whether the child is an infant, an older child, or a child with medical or other challenges, and whether the adoptive parent(s) are faced with unanticipated problems. (c) The first contact after placement shall take place within two weeks of placement. (d) A minimum of one face-to-face supervisory home visit after the initial two week contact shall take place before finalization.			☐	☐	☐	☐	☐	
	R501-7-9(12) Disruption. (a) The agency may remove the child from the adoptive placement due to circumstances that may impair the child's security in the family or jeopardize the child's physical and emotional development, including but not limited to incompatibility; mental illness; seriously incapacitating illness; the death of one of the adoptive parent(s); the separation or divorce of the adoptive parent(s); the abuse, neglect, or rejection of the child; the lack of attachment to the child; or a request by the adopting parents to remove the child. (b) If a child is removed from an adoptive home by a child placing adoption agency, the adoptive parent(s) shall be entitled to appeal the removal decision. (i) The agency shall provide the adoptive parent(s) written notice of their right to appeal and the procedure for appeal.			☐	☐	☐	☐	☐	
	R501-7-9(13) Finalization. (a) A child placing adoption agency shall provide assistance in finalizing the adoption.			☐	☐	☐	☐	☐	
	Intercountry Adoptions			R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes
	R501-7-10(1) All intercountry adoptions are considered high needs per 62A-4a-601 and require compliance with 62A-4a-609.			☐	☐	☐	☐	☐	

□ Staff Files Policy Client Files		Interviews		Walk Through	Variable Risk High or Extrem		
R501-7-10(2) In addition to complying with all other rules, laws and statutes regarding adoption, a child placing adoption agency that is a primary provider of inter-country placement services shall document that it has complied with all applicable laws and regulations of the United States and the child's country of origin, and including: (a) the agency is Hague accredited by a Department of State approved accrediting body; (b) the child is legally freed for adoption in the country of origin; (c) the agency verifies and maintains documentation and agreements regarding the credentials and qualifications of all associates working in their behalf in foreign countries; and (d) information was provided to the adopting parents about naturalization and US citizenship proceedings.		☐	☐	☐	☐		
R501-7-10(3) A child placing adoption agency that provides intercountry adoption services shall: (a) comply with all fee requirements from R501-7-6; (b) establish additional policies and procedures to be provided to the adoptive parent applicant(s) regarding: (i) agency and adoptive parent(s) responsibilities regarding intercountry adoption; (ii) post adopt responsibilities; (iii) identification and disclosure of medical risks in intercountry adoption; (iv) service planning; and (v) establish an official and recorded method of fund transfers to avoid the use of direct cash transactions to pay for adoption services in other countries; (c) additionally include in the written agency fee disclosure required in R501-7-6 the following: (i) itemization of all services and total cost of providing adoption in the child's country of origin and disclosure of whether the fees are paid directly or through the agency to include: (A) foreign country/legal fees; (B) cost of documents required by the agency and by the foreign government as well as costs of apostille or authentication of these documents; (C) required fees paid to USCIS; (D) estimated costs of travel to the foreign country; (E) translation of documents provided to the foreign adoption officials; (F) costs of child care; (G) parent education costs; (H) adopted child passport; (I) USCIS-required medical exam costs; (J) immunization expenses; and (K) any other miscellaneous fees that may apply; (ii) itemization of any mandatory payments to child welfare programs in the country of origin including: (A) any fixed contributions amounts; (B) intended use of payments; and (C) manner in which the transaction will be recorded and accounted for; (d) provide all applicants with written policies governing refunds; (e) notify adoptive applicants within ten business days when information is received that a foreign country is suspending its adoption program; (f) verify and maintain documentation regarding the credentials and qualifications of agents working in their behalf in foreign countries; and (g) in addition to adoptive parent(s) and child file content requirements in R501-7-11, intercountry adoption files shall also include: (i) signed agency agreements and/or contracts; (ii) USCIS approval to proceed with a foreign adoption; (iii) copies of adoption documents required by the adoption officials in the foreign country; (iv) copies of all child information provided by the foreign country; (v) post-adoption reports required by the foreign country; and (vi) copy of the adoption finalization from the foreign country.		☐	☐	☐	☐		
Administrative Documentation		R	UR	NA	Corrected During Inspection	Technical Assistance Given	Notes

☐	Staff Files	Policy	Client Files	Interviews		Walk Through	Variable Risk High or Extreme		
	R501-7-11(2) Adoptive Parent(s) Files shall cross-reference all related files and shall contain: (a) signed and dated application for service including agency disclosure of religion and marital status polices on the application; (b) signed and dated adoptive parent(s) adoptive orientation form as required and provided by DHS Office of Licensing; (c) proof that the content of the pre-existing parent(s) adoption orientation form was provided to adoptive parent(s); (d) proof of compliance with 62A-4a-607 regarding the availability of children in state custody for adoption; (e) itemized written fee disclosure statement as described in Section R501-7-6 signed and dated by prospective adoptive parent(s) and agency representative prior to entering any agreements as outlined in; (f) proof of identification or documented due diligence to determine identity; (g) copies of marriage certificates, divorce papers, custody and visitation orders, proof of US citizenship; (h) proof that all allowable child/pre-existing parent(s) information was shared with adoptive parent(s); (i) voluntary consent agreement acknowledging conflict of interests per R501-7-4 (A); (j) documentation and itemization of all reasonable and actual adoption-related expenses that exceed \$25.00 charged to the adoptive parent(s) as outlined in R501-7-6 to include: (i) written agreement and justification for any expenses charged to the prospective adoptive parent(s) outside the fee disclosure statement; (ii) affidavit signed by adoptive parent(s) and agency representative outlining itemized actual expenditures made on behalf of the pre-existing parent(s) as outlined in fees disclosures section R501-7-6; (k) record of all payments received and disbursements made; (l) home study/pre placement evaluation as outlined in R501-7-9 and 78B-6-128; (i) and including a child abuse registry report obtained from all applicable child welfare agencies per R501-7- 9(3)(c)(i); (m) case notes describing all services provided; (n) physician report for each prospective adoptive parent; (o) background clearances for prospective adoptive parent(s) and all adults over age 18 residing in the home; (p) proof of ability to provide health care for an adopted child; (q) 4 letters of reference; (r) documentation of all requests for information or sharing of information to include: (i) post adopt information exchange with pre-existing parent(s); and (ii) post adopt contact terms with pre-existing parent(s); (s) transition plan for child transition to adoptive placement; (t) written consent to legal risk placement if applicable; (u) documentation of the initial agency contact with the adoptive family within 2 weeks of placement; (v) documentation of one in-home face-to-face supervisory visit prior to finalization post two week visit; (w) original or certified copy of the order of adoption; (x) referral to Mutual Consent Registry; (y) signed declaration of each potential birth father to be filed with the court per 78B-6-110.5; and (z) any other documentation required in order to show compliance with this Rule.			☐	☐	☐	☐	☐	

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-7-11(3) Pre-existing parent(s) files shall cross reference all related files and shall contain: (a) signed and dated application for service to include declaration of birth mother's husband or any alleged father's relationship to the child in accordance with 78B-6-110.5; (b) proof of identification or documented due diligence to determine identity; (c) signed and dated pre-existing parent(s) adoptive orientation form as required and provided by DHS Office of Licensing; (d) declaration, certificate or written statement of putative registry search and disclosure of search results from each state identified by the birth mother in compliance with 78B-6-110.5 Sections 1 and 2; and any communications with potential birth fathers; (e) documentation of any requests for information or sharing of information; (f) genetic and social history, and health history; (g) case notes describing services provided including pre relinquishment counseling; (h) original or certified copies of relinquishment transfer or decree of termination of birth mother and birth father rights per 78B-6-125 (or the state governing relinquishment); (i) proof that non-identifying information was provided re: the adoptive parent(s); (j) proof of compliance with 78B-6-143 and 78B-6-144; (k) copies of marriage certificates, divorce papers, custody and visitation orders, if any; (l) certified copies of death certificates, if any, of pre-existing parent(s); (m) pre-existing parent(s) written agreements or refusals of: (i) waiver of confidentiality; (ii) authorization of release of information; (iii) future third party searcher; (iv) post adopt information exchange with adoptive parent(s); (v) post adopt contact terms; (n) verification that all itemized goods and services billed to the adoptive parent(s) were actually provided to and signed upon receipt to the pre-existing parent(s); (o) documentation of other alternative payment sources, including public assistance; (p) referral to Mutual Consent Registry; and (q) any other documentation required in order to show compliance with this rule.	☐	☐	☐	☐	☐	
			R501-7-11(4) Child Files shall cross reference all related files and shall contain: (a) needs assessments, evaluations, family background study of current and historical physical, psychological, genetic and developmental health information as required in R501-7-8 A and B; (b) individualized assessment determining which adoptive family was selected and why as a means to meet all of the identified wishes and needs of all involved; (c) case notes describing all services provided and referred; (d) copies of any DHS licenses for children placed in outside agency foster care; (e) transition plan for child to adoptive placement; and (f) any other documentation required in order to show compliance with this rule.	☐	☐	☐	☐	☐	

□	Staff Files	Policy	Client Files	Interviews			Walk Through	Variable Risk High or Extrem	
			R501-7-11(5) File maintenance. (a) In the event that any records required in this Rule are not obtained, the child placing adoption agency shall provide documentation of its efforts to obtain those records. (b) All case files shall be retained for a minimum of 100 years from the date the case is closed. (c) If not continuing to operate and incapable of maintaining their own files for 100 years, the agency shall notify the Office of Licensing and post publicly where the records shall be stored; (i) it is permissible for a closed child placing adoption agency to transfer closed adoptive files to another licensed child placing for maintenance as long as the chain of control is clear and transparent to the Office and prior clients and there is good reason to believe that the files will be maintained according to law. (ii) the agency has a written plan involving a secondary entity for contact and file maintenance in the event that the agency changes ownership or ceases to provide child placement adoption services, and notify the Office of Licensing and each client where the records shall be stored; and (iii) enable record retrieval by individuals with a right to access them. (d) All adoption records shall be confidential and shall be maintained in a secure location when not in active use; (i) adoption records shall be accessible only by authorized agency employees or agents; (ii) no information shall be shared with any person w/out the appropriate consent forms, except as required by law. (e) Records regarding the adoptive parents, with the exception of reference letters, are not sealed and information in adoption files can be provided to adoptive parent(s) upon request. (f) A child placing adoption agency shall maintain and provide accurate annual statistics describing the number of applications received the number of children, pre-existing parent(s), and adoptive parent(s) served, and the number of adoptions and disruptions, and the number of children in agency custody.	□	□	□	□	□	

Unannounced Inspection Checklist

(Revised Oct. 2025)

R = Reviewed
UR = Under Review
NA = Not Assessed during this inspection

	R	UR	NA	Corrected During Inspection	Notes
	R	UR	NA	Corrected During Inspection	Notes
	R	UR	NA	Corrected During Inspection	Notes
